Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female

Admit: 3/4/2024
Disch: Disch Time:
FIN: CHH0008011012203
Readmit Risk: 32 - LOW 03/05/24 21:45
Attending: Smuckler DO,Jason M

Orders

Order: **Consult to Case Management (Case Management Consult)**			
Order Date/Time: 3/5/2024 10:21 PST			
Signed Date/Time: 3/5/2024 10:21 PST			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Consults	Activity Type: Case Management Consults
End-state Date/Time: 3/5/2024 10:21 PST		End-state Reason:	
Ordering Physician: Smuckler DO,Jason M		Consulting Physician:	
Entered By: Valina DO,Anisa M on 3/5/2024 10:21 PST			
Order Details: 3/5/24 10:21:00 AM PST, Routine, Reason for Consult: D/C to Home w Home Health, For PT services. Says she has a home PCA and home health set up?			
Order Comment:			

Orders - Admission/Discharge (Curaspan)

Order: **Placement Change**			
Order Date/Time: 3/5/2024 10:25 PST			
Signed Date/Time: 3/5/2024 10:25 PST			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/5/2024 10:25 PST		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/5/2024 10:25 PST			
Order Details: 3/5/24 10:25:59 AM PST, Suggested Placement: Intermediate Care/Step down			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			
Action Type: Order	Action Date/Time: 3/5/2024 10:25 PST	Action Personnel: SYSTEM	
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert	
Order Details: 03/05/24 10:25:59 PST, Suggested Placement: Intermediate Care/Step down			
Review Information: Nurse Review: Electronically Signed, Fishman RN,Lindsey on 3/5/2024 14:48 PST Doctor Cosign: Not Required			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Orders - Admission/Discharge (Curaspan)

Order: **Change Attending Physician**			
Order Date/Time: 3/5/2024 10:25 PST			
Signed Date/Time: 3/5/2024 10:25 PST			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/5/2024 10:35 PST		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/5/2024 10:25 PST			
Order Details: 3/5/24 10:25:59 AM PST, Admitting: Timple DO, Julie M, Attending: Timple DO, Julie M			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/5/2024 10:35 PST	Action Personnel: Hernandez,Stephanie
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/05/24 10:25:59 PST, Admitting: Timple DO, Julie M, Attending: Timple DO, Julie M		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/5/2024 10:25 PST	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/05/24 10:25:59 PST, Admitting: Timple DO, Julie M, Attending: Timple DO, Julie M		
Review Information: Nurse Review: Not Reviewed Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Orders - Admission/Discharge (Curaspan)

Order: **Change Attending Physician**			
Order Date/Time: 3/5/2024 12:20 PST			
Signed Date/Time: 3/5/2024 12:20 PST			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/5/2024 12:26 PST		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/5/2024 12:20 PST			
Order Details: 3/5/24 12:20:58 PM PST, Admitting: Timple DO, Julie M, Attending: Tiwari, Subhikshya MD			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/5/2024 12:26 PST	Action Personnel: Hart,Demi
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/05/24 12:20:58 PST, Admitting: Timple DO, Julie M, Attending: Tiwari, Subhikshya MD		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/5/2024 12:20 PST	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/05/24 12:20:58 PST, Admitting: Timple DO, Julie M, Attending: Tiwari, Subhikshya MD		
Review Information: Nurse Review: Not Reviewed Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Orders - Admission/Discharge (Curaspan)

Order: **Change Attending Physician**			
Order Date/Time: 3/5/2024 21:56 PST			
Signed Date/Time: 3/5/2024 21:56 PST			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/5/2024 22:05 PST		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/5/2024 21:56 PST			
Order Details: 3/5/24 9:56:29 PM PST, Admitting: Timple DO, Julie M, Attending: Smuckler DO, Jason M			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/5/2024 22:05 PST	Action Personnel: Adams,Ivelisse
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/05/24 21:56:29 PST, Admitting: Timple DO, Julie M, Attending: Smuckler DO, Jason M		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/5/2024 21:56 PST	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/05/24 21:56:29 PST, Admitting: Timple DO, Julie M, Attending: Smuckler DO, Jason M		
Review Information: Nurse Review: Electronically Signed, Cypher RN,Tiffany on 3/5/2024 22:02 PST Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Medication Administration Summary

Medications

Administration Date/Time: 3/7/2024 14:53 PST
Medication Name: **tolvaptan**
Ingredients: tolv15Tab 7.5 mg
Admin Details: (Auth) Oral

Administration Date/Time: 3/7/2024 14:32 PST
Medication Name: **insulin aspart (insulin aspart NovoLog correctional scale Low)**
Admin Details: (Not Given) Not Appropriate at this time
MAR Glucose Result: 80

Administration Date/Time: 3/7/2024 13:34 PST
Medication Name: **tolvaptan**
Admin Details: Auth (Verified)
Pharmacist Intervention Time: 1-5 Minutes; Intervention Type Pharmacy: Medication review; Prescriber Response Pharmacy: Corrected prior to contact

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Medication Administration Summary

Medications

Administration Date/Time: 3/7/2024 09:19 PST
Medication Name: **zonisamide (Zonegran)**
Ingredients: zoni100Cap 200 mg 2 Caps
Admin Details: (Auth) Oral

Administration Date/Time: 3/7/2024 09:19 PST
Medication Name: **levETIRAcetam**
Ingredients: leve500Tab 500 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/7/2024 09:19 PST
Medication Name: **heparin**
Ingredients: heparin 5000 units/mL 1 mL Inj 5000 units 1 mL
Admin Details: (Auth) SubCutaneous, Left Lowr Abdom Quad

Administration Date/Time: 3/7/2024 09:19 PST
Medication Name: **famotidine**
Ingredients: famo20Tab 20 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/7/2024 09:19 PST
Medication Name: **buPROPion (buPROPion SR 12 hour extended release)**
Ingredients: buPR150SRTab 150 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/7/2024 06:39 PST
Medication Name: **insulin aspart (insulin aspart NovoLog correctional scale Low)**
Admin Details: (Not Given) Not Appropriate at this time
MAR Glucose Result: 87

Administration Date/Time: 3/7/2024 03:13 PST
Medication Name: **cefepime**
Ingredients: cefe2REC 2 gm 20 mL; D5%50ml 50 mL
Admin Details: (Auth) IV Piggyback, Left Arm

Administration Date/Time: 3/6/2024 20:55 PST
Medication Name: **insulin aspart (insulin aspart NovoLog correctional scale Low)**
Admin Details: (Not Given) Not Appropriate at this time
MAR Glucose Result: 84

Administration Date/Time: 3/6/2024 20:55 PST
Medication Name: **heparin**
Ingredients: heparin 5000 units/mL 1 mL Inj 5000 units 1 mL
Admin Details: (Auth) SubCutaneous, Left Arm

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Medication Administration Summary

Medications

Administration Date/Time: 3/6/2024 20:55 PST
Medication Name: **buPROPion (buPROPion SR 12 hour extended release)**
Ingredients: buPR150SRTab 150 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/6/2024 19:39 PST
Medication Name: **levETIRAcetam**
Ingredients: leve500Tab 500 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/6/2024 16:33 PST
Medication Name: **insulin aspart (insulin aspart NovoLog correctional scale Low)**
Admin Details: (Not Given) Not Appropriate at this time
MAR Glucose Result: 82

Administration Date/Time: 3/6/2024 16:33 PST
Medication Name: **potassium chloride**
Ingredients: pota20pack 30 mEq
Admin Details: (Auth) Oral

Administration Date/Time: 3/6/2024 16:32 PST
Medication Name: **cefepime**
Ingredients: cefe2REC 2 gm 20 mL; D5%50ml 50 mL
Admin Details: (Auth) IV Piggyback, Left Arm

Administration Date/Time: 3/6/2024 12:48 PST
Medication Name: **heparin**
Admin Details: Auth (Verified)
Pharmacist Intervention Time: 6-15 Minutes; Pharmacy Intervention Reduce Cost: Yes; Intervention Type Pharmacy: Anticoagulation Therapy Review; Prescriber Response Pharmacy: Accepted

Administration Date/Time: 3/6/2024 12:24 PST
Medication Name: **insulin aspart (insulin aspart NovoLog correctional scale Low)**
Admin Details: (Not Given) Not Appropriate at this time
MAR Glucose Result: 89

Administration Date/Time: 3/6/2024 11:54 PST
Medication Name: **potassium chloride**
Ingredients: pota20pack 30 mEq
Admin Details: (Auth) Oral

Administration Date/Time: 3/6/2024 09:17 PST
Medication Name: **buPROPion (buPROPion SR 12 hour extended release)**
Ingredients: buPR150SRTab 150 mg 1 Tabs
Admin Details: (Auth) Oral

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Medication Administration Summary

Medications

Administration Date/Time: 3/6/2024 09:06 PST
Medication Name: **zonisamide (Zonegran)**
Ingredients: zoni100Cap 200 mg 2 Caps
Admin Details: (Auth) Oral

Administration Date/Time: 3/6/2024 09:06 PST
Medication Name: **levETIRAcetam**
Ingredients: leve500Tab 500 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/6/2024 09:06 PST
Medication Name: **heparin**
Ingredients: heparin 5000 units/mL 1 mL Inj 5000 units 1 mL
Admin Details: (Auth) SubCutaneous, Right Lwr Abdom Quad

Administration Date/Time: 3/6/2024 09:06 PST
Medication Name: **famotidine**
Ingredients: famo20Tab 20 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/6/2024 06:33 PST
Medication Name: **insulin aspart (insulin aspart NovoLog correctional scale Low)**
Admin Details: (Not Given) Not Appropriate at this time
MAR Glucose Result: 84

Administration Date/Time: 3/6/2024 02:38 PST
Medication Name: **cefepime**
Ingredients: cefe2REC 2 gm 20 mL; D5%50ml 50 mL
Admin Details: (Auth) IV Piggyback, Left Arm

Administration Date/Time: 3/5/2024 23:47 PST
Medication Name: **insulin aspart (insulin aspart NovoLog correctional scale Low)**
Admin Details: (Not Given) Not Appropriate at this time
MAR Glucose Result: 79

Administration Date/Time: 3/5/2024 22:36 PST
Medication Name: **levETIRAcetam**
Ingredients: leve500Tab 500 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/5/2024 22:36 PST
Medication Name: **heparin**
Ingredients: heparin 5000 units/mL 1 mL Inj 5000 units 1 mL
Admin Details: (Auth) SubCutaneous, Left Arm

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Medication Administration Summary

Medications

Administration Date/Time: 3/5/2024 22:36 PST
Medication Name: **buPROPion (buPROPion SR 12 hour extended release)**
Ingredients: buPR150SRTab 150 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/5/2024 17:24 PST
Medication Name: **cefepime**
Ingredients: cefe2REC 2 gm 20 mL; D5%50ml 50 mL
Admin Details: (Auth) IV Piggyback, Right Hand

Administration Date/Time: 3/5/2024 16:30 PST
Medication Name: **insulin aspart (insulin aspart NovoLog correctional scale Low)**
Admin Details: (Not Done) Not Appropriate at this Time
MAR Glucose Result, insulin aspart

Administration Date/Time: 3/5/2024 12:09 PST
Medication Name: **buPROPion (buPROPion SR 12 hour extended release)**
Ingredients: buPR150SRTab 150 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/5/2024 11:44 PST
Medication Name: **insulin aspart (insulin aspart NovoLog correctional scale Low)**
Admin Details: (Not Given) Not Appropriate at this time
MAR Glucose Result: 100

Administration Date/Time: 3/5/2024 11:03 PST
Medication Name: **levETIRAcetam**
Ingredients: leve500Tab 500 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/5/2024 11:03 PST
Medication Name: **heparin**
Ingredients: heparin 5000 units/mL 1 mL Inj 5000 units 1 mL
Admin Details: (Auth) SubCutaneous, Left Lowr Abdom Quad

Administration Date/Time: 3/5/2024 11:03 PST
Medication Name: **famotidine**
Ingredients: famo20Tab 20 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/5/2024 10:00 PST
Medication Name: **zonisamide (Zonegran)**
Admin Details: (Not Done) Equipment/Supplies Unavailable
zonisamide

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Medication Administration Summary

Medications

Administration Date/Time: 3/5/2024 07:52 PST
Medication Name: **insulin aspart (insulin aspart NovoLog correctional scale Low)**
Admin Details: (Not Given) Not Appropriate at this time
MAR Glucose Result: 96

Administration Date/Time: 3/5/2024 03:30 PST
Medication Name: **cefepime**
Ingredients: cefe2REC 2 gm 20 mL; D5%50ml 50 mL
Admin Details: (Auth) IV Piggyback, Right Wrist

Continuous Infusions

Administration Date/Time: 3/6/2024 19:38 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 20 mEq 1000 mL
Admin Details: (Begin Bag) (Auth) 1000 mL, 100 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 06:32 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 06:04 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 06:03 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 20 mEq 1000 mL
Admin Details: (Begin Bag) (Auth) 1000 mL, 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 06:00 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 0 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 05:55 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 20 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 05:01 PST to 3/6/2024 05:59 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 0.992 mEq
Admin Details: (Infuse) (Auth) 49.61 mL, 20 mL/hr, Right Lower Forearm

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Medication Administration Summary

Continuous Infusions

Administration Date/Time: 3/6/2024 04:14 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 04:12 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 20 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 04:00 PST to 3/6/2024 04:56 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 0.309 mEq
Admin Details: (Infuse) (Auth) 15.44 mL, 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 04:00 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 03:00 PST to 3/6/2024 03:59 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 1.5 mEq
Admin Details: (Infuse) (Auth) 75 mL, 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 03:00 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 02:00 PST to 3/6/2024 02:59 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 1.5 mEq
Admin Details: (Infuse) (Auth) 75 mL, 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 02:00 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 01:00 PST to 3/6/2024 01:59 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 1.5 mEq
Admin Details: (Infuse) (Auth) 75 mL, 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 01:00 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Medication Administration Summary

Continuous Infusions

Administration Date/Time: 3/6/2024 00:00 PST to 3/6/2024 00:59 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 1.5 mEq
Admin Details: (Infuse) (Auth) 75 mL, 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/6/2024 00:00 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/5/2024 23:00 PST to 3/5/2024 23:59 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 1.5 mEq
Admin Details: (Infuse) (Auth) 75 mL, 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/5/2024 23:00 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/5/2024 22:00 PST to 3/5/2024 22:59 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 1.5 mEq
Admin Details: (Infuse) (Auth) 75 mL, 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/5/2024 22:00 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/5/2024 21:00 PST to 3/5/2024 21:59 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 1.5 mEq
Admin Details: (Infuse) (Auth) 75 mL, 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/5/2024 21:00 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/5/2024 20:00 PST to 3/5/2024 20:59 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 1.5 mEq
Admin Details: (Infuse) (Auth) 75 mL, 75 mL/hr, Right Lower Forearm

Administration Date/Time: 3/5/2024 20:00 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Admin Details: (Rate Change) (Auth) 75 mL/hr, Right Lower Forearm

Medication Administration Summary

Continuous Infusions

Administration Date/Time: 3/5/2024 14:34 PST
Medication Name: **Sodium Chloride 0.9% with KCl 20 mEq/L intravenous solution 20 mEq**
Ingredients: NS/20KCL1000 20 mEq 1000 mL
Admin Details: (Begin Bag) (Auth) 1000 mL, 75 mL/hr, Right Lower Forearm

Physical Therapy Forms

DOCUMENT TYPE:	PT Daily Documentation - Text
SERVICE DATE/TIME:	3/7/2024 11:00 PST
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Qiu PTA,Carrie (3/7/2024 11:39 PST)
SIGNED INFORMATION:	Qiu PTA,Carrie (3/7/2024 11:39 PST)

PT Daily Documentation Entered On: 3/7/2024 11:39 PST
Performed On: 3/7/2024 11:00 PST by Qiu PTA, Carrie

Review/Treatments Provided
PT Goals : PT Short Term Goals

No qualifying data available
PT Plan : Treatment Frequency: Other: 4-5x/week Performed By: Aikels PT, MPT, Dawn 03/06/2024
Treatment Duration: 1-2 weeks Performed By: Aikels PT, MPT, Dawn 03/06/2024
Planned Treatments: Balance training, Basic activities of daily living, Bed mobility training, Equipment training, Gait training, Manual therapy, Neuromuscular reeducation, Patient education, Therapeutic exercises, Transfer training Performed By: Aikels PT, MPT, Dawn 03/06/2024
Physical Therapy Orders : No active Physical Therapy Orders.
Pain Symptoms : No
Suspected Stroke Patient : No
Post-Acute Discharge Therapy Recommendation PT : Continued intermittent therapy to address functional deficits
PT Therapeutic Exercise : Yes

Qiu PTA, Carrie - 3/7/2024 11:35 PST

Mobility
Mobility Grid
Roll Left : Rehab Minimal assistance
Roll Right : Rehab Minimal assistance
Supine to Sit : Rehab Minimal assistance
Sit to Supine : Rehab Minimal assistance
Transfer Sit to Stand : Rehab Minimal assistance
Transfer Stand to Sit : Rehab Minimal assistance

Qiu PTA, Carrie - 3/7/2024 11:35 PST

Ambulation Level : Min A

Physical Therapy Forms

Device : Front-wheel walker (FWW)
Ambulation Distance : 5 ft

Qiu PTA, Carrie - 3/7/2024 11:35 PST

Therapeutic Exercise

Therapeutic Exercise Grid

	Exercise 1
Exercise :	Lower extremity strengthening
Position :	Unsupported sit
Repetition/Time :	10x1
Completed :	Yes
Comment :	AP, GS, LAQ
	Qiu PTA, Carrie - 3/7/2024 11:35 PST

Education

Teaching Method : Demonstration, Explanation
Barriers to Learning : None evident

Qiu PTA, Carrie - 3/7/2024 11:35 PST

Physical Therapy Education Grid

Ambulation with Roller Walker : Verbalizes understanding, Demonstrates, Needs further teaching
Bed Mobility : Verbalizes understanding, Demonstrates, Needs further teaching
Bed Positioning : Verbalizes understanding, Demonstrates, Needs further teaching
Sit to Stand Transfers : Needs further teaching, Demonstrates, Verbalizes understanding
Exercise Program : Verbalizes understanding, Demonstrates, Needs further teaching

Qiu PTA, Carrie - 3/7/2024 11:35 PST

Additional Information

Additional Information PT : RN consulted and cleared pt. for PT tx. Pt. presents semi supine in bed w/ no c/o pain and agreeable to tx. Pt. on 1.5L NCO2 throughout session. Bed mobility training semi supine<>sit EOB min A w/ no dizziness w/ position change. Transfer training sit<>stand FWW min A w/ no LOB or dizziness w/ upright position. Gait training 5'x1 FWW min A forward, pt. demos step to gait pattern w/ significantly decreased stride length and cadence, pt. demos SOB and labored breathing, no decrease in O2 sat, pt. limited by decreased endurnace for further OOB tx at this time. Assisted pt. BTB and instructed w/ BLE ther ex's as charted above. Pt. left semi supine in bed w/ bed alarm on, call light in reach, positioned for comfort & all needs met.

Barriers to DC: fall risk

Total in time: 40 min

supervised by Krista Rapozo, PT, DPT.

Qiu PTA, Carrie - 3/7/2024 11:39 PST

Environmental Safety Implemented : Adequate room lighting, Bed in low position, Call device within reach, Personal items within reach

Qiu PTA, Carrie - 3/7/2024 11:35 PST

PTA Timed Modalities 2024

Physical Therapy Forms

PTA Timed Min Total : 40
PTA Timed Units Total : 3
PTA THERAPEUTIC ACT 15MIN : 15
ACTIVITY THERAPEUTIC CQ : 1
PTA EXERCISE THERAPEUTIC 15MIN : 10
EXERCISE THERAPEUTIC CQ : 1
PTA GAIT TRAINING 15MIN : 15
GAIT TRAINING CQ : 1

Qiu PTA, Carrie - 3/7/2024 11:35 PST

DOCUMENT TYPE:	PT Examination - Text
SERVICE DATE/TIME:	3/6/2024 08:30 PST
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Aikels PT,MPT,Dawn (3/6/2024 13:53 PST)
SIGNED INFORMATION:	Aikels PT,MPT,Dawn (3/6/2024 13:53 PST)

PT Evaluation/Reevaluation Entered On: 3/6/2024 13:32 PST
Performed On: 3/6/2024 8:30 PST by Aikels PT, MPT, Dawn

General Info
PT Assessment Type : Initial Evaluation
Physical Therapy Orders : PT Evaluation and Treatment - 03/05/24 14:54:00 PST, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, Yes, Yes
Precautions RTF : Active Orders:
» Oxygen Therapy, 03/03/24 23:27:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered
» Oxygen Therapy, 03/03/24 23:23:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 94%, Ordered

Orientation Assessment Rehab : Oriented x 4
Affect/Behavior Rehab : Appropriate, Cooperative
Basic Command Following : Intact
Safety/Judgment : Intact
Pain Symptoms : No
Suspected Stroke Patient : No

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Home Environment
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Fishman RN, Lindsey 03/05/2024
Living Situation : Home with family care
Lives With : Child(ren)
Lives In : Apartment

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Home Environment II

Physical Therapy Forms

Living Environment : Home Environment
Living Situation: Home with family care Performed By: Fishman RN, Lindsey 03/05/2024
Home Equipment Rehab : 4-Wheel walker, Manual wheelchair

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Prior Functional Status Grid
ADL : Assist needed
Mobility : Independent
Instrumental ADL : Dependent
Cognitive-Communication Skills : Independent

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

LE Range/Strength

LE Overall Range of Motion Grid
Left Lower Extremity Passive Range : Within functional limits
Left Lower Extremity Active Range : Within functional limits
Right Lower Extremity Passive Range : Within functional limits
Right Lower Extremity Active Range : Within functional limits

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Left Lower Extremity Strength Grid
Hip Flexion : 3
Hip Extension : 3
Hip Abduction : 3
Hip Adduction : 3
Hip External Rotation : 3
Hip Internal Rotation : 3
Knee Flexion : 3+
Knee Extension : 3+
Ankle Dorsiflexion : 3+
Ankle Plantarflexion : 3+
Ankle Eversion : 3
Ankle Inversion : 3
Great Toe Extension : 3

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Right Lower Extremity Strength Grid
Hip Flexion : 3
Hip Extension : 3
Hip Abduction : 3
Hip Adduction : 3
Hip External Rotation : 3
Hip Internal Rotation : 3
Knee Flexion : 3+
Knee Extension : 3+
Ankle Dorsiflexion : 3+
Ankle Plantarflexion : 3+
Ankle Eversion : 3
Ankle Inversion : 3
Great Toe Extension : 3

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Balance

Kansas University Balance Scale : Yes

Physical Therapy Forms

Sitting Balance Score : 4
Standing Balance Score : 1+

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Mobility

Mobility Grid

Roll Left : Rehab Minimal assistance
Roll Right : Rehab Minimal assistance
Supine to Sit : Rehab Minimal assistance
Sit to Supine : Rehab Minimal assistance
Transfer Sit to Stand : Rehab Minimal assistance
Transfer Stand to Sit : Rehab Minimal assistance

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Ambulation Level : Does not occur

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Assessment

PT Impairments or Limitations : Ambulation deficits, Balance deficits, Bed mobility deficits, Endurance deficits, Equipment training, Strength deficits, Transfer deficits, Transition deficits
Barriers to Safe Discharge PT : Medical diagnosis

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Education

Teaching Method : Demonstration, Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Physical Therapy Education Grid

Bed Mobility : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Bed Positioning : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Sit to Stand Transfers : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Physical Therapy Plan of Care : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Long Term Goals

PT Patient,Caregiver Goal : return home safely

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

PT Long Term Goals Rehab

	Long Term Goal 1	Long Term Goal 2	Long Term Goal 3
Goal :	Ind bed mob	MI transfers using LRAD	MI ambulation x 150' using LRAD
Status :	Initial	Initial	Initial
Timeframe To Complete :	1-2 weeks	1-2 weeks	1-2 weeks
Comments :	LTG=STG		
	Aikels PT, MPT, Dawn -	Aikels PT, MPT, Dawn -	Aikels PT, MPT, Dawn -

Physical Therapy Forms

	3/6/2024 13:33 PST	3/6/2024 13:33 PST	3/6/2024 13:33 PST

Plan
PT Anticipated Treatments, Needs : Balance training, Basic activities of daily living, Bed mobility training, Equipment training, Gait training, Manual therapy, Neuromuscular reeducation, Patient education, Therapeutic exercises, Transfer training
PT Frequency Rehab : Other: 4-5x/week
PT Duration Rehab : 1-2 weeks
PT Intensity Per Day : 30-60 minutes
Rehabilitation Potential : Good
PT Plan/Goals Established w Pt/Caregiver : Yes
Post-Acute Discharge Therapy Recommendation PT : Continued intermittent therapy to address functional deficits

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Additional Information
Additional Information PT : This pt is a 66 yo female who presented to CHH with the primary c/o waekness. Ground level fall experienced and EMS contacted. She was found to be hypotensive on the seen. CT reveals wht matter changes and the pt was assessed as having AKI, facial cellulitis and UTI

PMH: cardiac arrest, TBI, R hemi, HTN, CHF, depression
PLOF: the pt lives with son Down's syndrome son in a 1st floor apt and was MI for ambulation, household distances using 4WW. She has a PCA ~4 hours/day to assist with G/H and higher level household ADLs/IADLs

Treatment recap: After nsg authorized IE and tx, the pt is seen in a semisupine position and ageed to services. She performs mobility per grid when HOB is elevated and with railing requiring min A for sup>sit transfers to EOB. No visible LOB noted at EOB. STS is with min A using FWW. While standing, increased global tremors noted which increase with standing time. She declined prog gt training d/t uncontrollable tremors but was able to take a few side steps toward HOB. She returns BTB with all needs met and CLIR. Intermittent verbal cues required for sequencing and safety during transitional movements including proper hand placement. All vitals monitored and found to be WFL. Nsg notified of the pts function. She was found appropriate for continued skilled intervention to promote improved function and reduce fall risk
Px: fall risk, foley cath
Barriers to DC: fall risk

Treatment time: 40 minutes

Aikels PT, MPT, Dawn - 3/6/2024 13:53 PST

Environmental Safety Implemented : Adequate room lighting, Bed in low position, Call device within reach, Personal items within reach

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

PT Non Timed Modalities 2024
PT Eval Moderate Complexity Total Number Min : 15
PT Total Non Timed : 15

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

PT Timed Modalities 2024
PT Timed Min Total : 25
PT Timed Unit Total : 2
PT THERAPEUTIC ACT 15MIN : 25

Physical Therapy Forms

Activity Therapeutic Num of Units : 2

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Occupational Therapy Forms

DOCUMENT TYPE:	OT Daily Documentation - Text
SERVICE DATE/TIME:	3/7/2024 12:32 PST
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Nguyen OT,Anthony (3/7/2024 14:41 PST)
SIGNED INFORMATION:	Nguyen OT,Anthony (3/7/2024 14:41 PST)

OT Daily Documentation Entered On: 3/7/2024 14:52 PST
Performed On: 3/7/2024 12:32 PST by Nguyen OT, Anthony

Review/Treatments Provided
OT Goals : OT Short Term Goals

No qualifying data available
OT Plan : Treatment Frequency: 5-7 times per week Performed By: Nguyen OT, Anthony 03/06/2024
Treatment Duration: 1-2 weeks Performed By: Nguyen OT, Anthony 03/06/2024
Planned Treatments: Balance training, Basic Activities of Daily Living, Equipment training, Home program, Mobility training, Pain management, Patient education, Safety education, Therapeutic exercises Performed By: Nguyen OT, Anthony 03/06/2024
Occupational Therapy Orders : No active Occupational Therapy orders.
Pain Symptoms : No
Suspected Stroke Patient : No
Post-Acute Discharge Therapy Recommendation OT : Continued intermittent therapy to address functional deficits
OT Therapeutic Activity,Mobility,Balance : Yes

Nguyen OT, Anthony - 3/7/2024 14:41 PST

Therapeutic Activities
Reassess Activities of Daily Living : Yes

Nguyen OT, Anthony - 3/7/2024 14:41 PST

Basic ADL
Upper Extremity Dressing Indep Measure
Grooming : Min A
UE Dressing : Min A
Toileting : Total Assistance

Nguyen OT, Anthony - 3/7/2024 14:41 PST

Mobility
Mobility Grid
Supine to Sit : Rehab Minimal assistance
Sit to Supine : Rehab Minimal assistance
Transfer Sit to Stand : Rehab Minimal assistance
Transfer Stand to Sit : Rehab Minimal assistance

Occupational Therapy Forms

Nguyen OT, Anthony - 3/7/2024 14:41 PST

Ambulation Level OT : Min A
Device utilized OT : Front-wheel walker (FWW)

Nguyen OT, Anthony - 3/7/2024 14:41 PST

Additional Information
Additional Information OT : Room 402

Pt cleared by NSG for OT tx. Pt waited to recieve her neuro medication prior to her tx. Pt min A for supine to EOB. Pt min A for sit to stand with FWW. Pt ambulated ~5ft until she demo increased tremors. Pt had difficulty stepping back to EOB due to tremor and weakness. Bed released to bring next to patient to sit back down. Pt demos increased fatigue and expressed frustrations on how debilitated she is now. Therapeutic listening provided with patient at EOB. Pt declined using FWW to side step to HOB. Pt able to scoot to HOB with CGA. Pt groomed self at EOB with min A. Pt returned semisupine in bed with min A. Pt will cont to benefit from skilled OT for improved functional independence. Time spent: 38 min

DC recommendation: intermittent therapy
Dx: weakness, hyponatremia
Barriers: weakness, poor mobility and self care
Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach

Nguyen OT, Anthony - 3/7/2024 14:41 PST

OT Timed Modalities 2024
OT Timed Min Total : 38
OT Timed Units Total : 3
OT THERAPEUTIC ACT 15MIN : 38
OT THERAPEUTIC ACT Num of Units : 3

Nguyen OT, Anthony - 3/7/2024 14:41 PST

DOCUMENT TYPE:	OT Evaluation - Text
SERVICE DATE/TIME:	3/6/2024 09:01 PST
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Nguyen OT,Anthony (3/6/2024 14:28 PST)
SIGNED INFORMATION:	Nguyen OT,Anthony (3/6/2024 14:28 PST)

OT Evaluation/Reevaluation Entered On: 3/6/2024 14:42 PST
Performed On: 3/6/2024 9:01 PST by Nguyen OT, Anthony

General Information
OT Assessment Type : Initial Evaluation
Occupational Therapy Orders : OT Evaluation and Treatment - 03/05/24 14:54:00 PST, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, Yes, Yes
Precautions RTF : Active Orders:
» Oxygen Therapy, 03/03/24 23:27:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered
» Oxygen Therapy, 03/03/24 23:23:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater

than 94%, Ordered

Orientation Assessment Rehab : Oriented x 4
Affect/Behavior Rehab : Appropriate, Calm, Cooperative
Basic Command Following : Intact
Safety/Judgment : Intact
Pain Symptoms : No
Suspected Stroke Patient : No

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Home Environment
Living Environment : Home Environment
*ADL: Assist needed Performed By: Aikels PT, MPT, Dawn 03/06/2024
*Cognitive-Communication Skills: Independent Performed By: Aikels PT, MPT, Dawn 03/06/2024
*Instrumental ADL: Dependent Performed By: Aikels PT, MPT, Dawn 03/06/2024
*Mobility: Independent Performed By: Aikels PT, MPT, Dawn 03/06/2024
Devices/Equipment at Home: 4-Wheel walker, Manual wheelchair Performed By: Aikels PT, MPT, Dawn 03/06/2024
Lives In: Apartment Performed By: Aikels PT, MPT, Dawn 03/06/2024
Lives With: Child(ren) Performed By: Aikels PT, MPT, Dawn 03/06/2024
Living Situation: Home with family care Performed By: Aikels PT, MPT, Dawn 03/06/2024
Living Situation : Home with family care
Lives With : Child(ren)
Lives In : Apartment
Patient's Responsibilities Rehab : Personal ADL

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Home Environment II
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Fishman RN, Lindsey 03/05/2024
Home Equipment Rehab : 4-Wheel walker, Manual wheelchair

Nguyen OT, Anthony - 3/6/2024 14:28 PST

ADL : Assist needed
Mobility : Independent
Instrumental ADL : Dependent
Cognitive-Communication Skills : Independent

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Basic ADL
Activities of Daily Living Grid
UE Dressing : Min A
Toileting : Total Assistance

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Balance
General Balance : Standing
General Balance Assessment : Fair

Nguyen OT, Anthony - 3/6/2024 14:28 PST

UE Strength/ROM
Upper Extremity Overall ROM Grid
Left Upper Extremity Passive Range : Within functional limits
Left Upper Extremity Active Range : Within functional limits
Right Upper Extremity Passive Range : Within functional limits

Occupational Therapy Forms

Right Upper Extremity Active Range : Within functional limits

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Mobility

Mobility Grid

Supine to Sit : Rehab Contact guard assistance
Sit to Supine : Rehab Contact guard assistance
Transfer Sit to Stand : Rehab Contact guard assistance
Transfer Stand to Sit : Rehab Contact guard assistance

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Ambulation Level OT : Min A
Device utilized OT : Front-wheel walker (FWW)

Nguyen OT, Anthony - 3/6/2024 14:28 PST

OT Assessment

Impairments/Limitations : Balance deficits, Basic activity of daily living deficits, Endurance deficits, Equipment training, Mobility deficits, Strength deficits
Barriers to Safe Discharge : Medical diagnosis

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Education

Teaching Method : Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Occupational Therapy Education Grid

Activity of Daily Living Training : Needs further teaching, Needs practice/supervision
Bed to Chair Transfers : Needs practice/supervision, Needs further teaching
Sit to Stand Transfers : Needs further teaching, Needs practice/supervision
Exercise Program : Needs further teaching, Needs practice/supervision
Home Safety : Needs practice/supervision, Needs further teaching
Plan of Care : Verbalizes understanding
Safety Awareness : Verbalizes understanding

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Long Term Goals

OT Patient/Caregiver Goal : Pt unable to state

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Long Term Goal 5

	Long Term Goal 1	Long Term Goal 2	Long Term Goal 3	Long Term Goal 4
Goal :	Pt to be mod I with transfers	Pt to be mod I with toileting	Pt to groom self in front of sink mod I	Pt to ambulate household distances mod I
Status :	Initial assessment	Initial assessment	Initial assessment	Initial assessment
Timeframe To Complete :	2 weeks	2 weeks	2 weeks	2 weeks
	Nguyen OT, Anthony - 3/6/2024 14:28 PST	Nguyen OT, Anthony - 3/6/2024 14:28 PST	Nguyen OT, Anthony - 3/6/2024 14:28 PST	Nguyen OT, Anthony - 3/6/2024 14:28 PST

Occupational Therapy Forms

Plan
OT Anticipated Treatments, Needs : Balance training, Basic Activities of Daily Living, Equipment training, Home program, Mobility training, Pain management, Patient education, Safety education, Therapeutic exercises
OT Frequency Rehab : 5-7 times per week
OT Duration Rehab : 1-2 weeks
OT Intensity Per Day : Other: as tolerated
Rehabilitation Potential : Excellent
OT Tx Plan/Goals Established w Patient : Yes
OT Evaluation Time : 15 Minutes
OT Evaluation Charge : Yes
Post-Acute Discharge Therapy Recommendation OT : Continued intermittent therapy to address functional deficits

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Additional Information
Additional Information OT : Room 402

Pt cleared by NSG for OT IE. Pt in bed and agreeable to OT IE. Pt is a 66 y/o female admitted for generalized weakness, hyponatremia, AKI, UTI, facial cellulitis. Pt has hx of TBI and mood disorder. Pt lives in 1st floor apt with son where she is mod I with ADL's except bathing and mod I with mobility at home. Pt has a self pay caregiver that comes to assist with bathing at home. Pt CGA for supine to EOB. Pt sat up EOB with SPV for safety. Pt donned clean gown with min A at EOB. Pt CGA for sit to stand with FWW. Pt had bowel accident during stand requiring total A for pericare. Pt side step to HOB with min A. Pt returned semisupine in bed with CGA. Pt educated on role of OT, POC, and recommendations. Pt demos impaired mobility, strength, balance, and self care. Pt will benefit from skilled OT for improved functional independence. Time spent: 38 min

DC recommendation: intermittent therapy
Dx: generalized weakness, hyponatremia, AKI, UTI, facial cellulitis
PMH: TBI, mood disorder, CHF, HTN, hypothyroidism
Barriers: impaired mobility, strength, balance, and self care
Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach

Nguyen OT, Anthony - 3/6/2024 14:28 PST

OT Non Timed Modalities 2024
Eval Moderate Complexity Total Number Min : 15
OT Total NonTimed Mod : 15

Nguyen OT, Anthony - 3/6/2024 14:28 PST

OT Timed Modalities 2024
OT Timed Min Total : 23
OT Timed Units Total : 2
OT THERAPEUTIC ACT 15MIN : 23
OT THERAPEUTIC ACT Num of Units : 2

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Speech Therapy Forms

DOCUMENT TYPE:	Clinical Swallowing - Text
SERVICE DATE/TIME:	3/6/2024 12:17 PST
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Hernandez SLP,Gardenia (3/6/2024 16:12 PST)
SIGNED INFORMATION:	Hernandez SLP,Gardenia (3/6/2024 16:12 PST)

ST Bedside Swallow Eval Entered On: 3/6/2024 15:55 PST
Performed On: 3/6/2024 12:17 PST by Hernandez SLP, Gardenia

General Information
Speech Orders : ST Bedside Swallow Exam - 03/05/24 14:54:00 PST, Bedside Swallow Exam, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, IV Yes, O2 Yes, 16443966655.0
ST Eval and Treat - 03/05/24 14:54:00 PST, Bedside Swallow Exam, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, IV Yes, O2 Yes
Precautions RTF : Active Orders:
» Oxygen Therapy, 03/03/24 23:27:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered
» Oxygen Therapy, 03/03/24 23:23:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 94%, Ordered

History of Swallow Studies : No qualifying data available
Hernandez SLP, Gardenia - 3/6/2024 15:55 PST

Pre-Assessment
Oxygen Therapy : Nasal cannula
Respirations : Other: WFLs
Tracheostomy Information : Oxygen Flow Rate: 2 L/min Performed by: Cruz, Nathan 03/06/24 12:00:00
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Oral-Facial Exam
Facial Movement : Symmetric resting/crying
Dentition : Own teeth
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Oral Structure Grid
Labial Structure : Within functional limits
Mandible : Within functional limits
Lingual Structure : Within functional limits
Hard Palate : Within functional limits
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Oral Function Grid
Labial : Within functional limits
Mandible : Within functional limits
Lingual : Within functional limits
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Volitional Cough : Within functional limits
Secretion Management : Adequate
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Swallowing Exam

Speech Therapy Forms

Patient Position : Upright 90 degrees

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Liquids Swallowing Exam Adult Grid

	Ice Chips
Comment :	please see additional info
	Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Assessment/Recommendations

Clinical Swallowing Impression Grid

Within Functional Limits : Solids, Liquids

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Diet Consistency : Regular
Liquid Viscosity : Thin
Medications : Other: as tolerated – whole w/ liquids or floated in puree
Supervision : Intermittent
Positioning : Upright 90 degrees
Clinical Swallowing Recommendations : Intervention not indicated

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Education

Learner(s) Present for Session : Demonstration, Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Document learning evaluation for Responsible Learner(s)

Swallowing Strategies : Verbalizes understanding, Demonstrates

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Long Term Goals

SLP Patient, Caregiver Goals : N/A

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Additional Information

Additional Information SLP : RM: 402
Minutes: 20

Per RN okay to proceed with swallow evaluation as ordered per MD.

66 y/o female admitted for: generalized asthenia, hyponatremia, AKI, UTI, facial cellulitis, severe sepsis, and dehydration. PMHx positive for: epilepsy NOS, TBI with resultant mild right hemiparesis, CHF, HTN, hypothyroidism, mood disorder NOS, and cardiac arrest per medical record. CXR results from 3/3/24 and head CT results from 3/4/24 were negative for any acute findings. CT Maxillofacial w/o contrast from 3/4/24 noted. Pt on 2L O2 via nasal cannula and on a minced & moist texture/thin liquid diet per MD. Pt was AA&Ox3-4. Impressions as follows.

Pt's vocal quality was strong and clear. Volitional cough was strong and dry. Oral motor structure and function assessment was WFLs. Hyo-laryngeal elevation was palpable. PO trials of thin liquids, purees,

Speech Therapy Forms

regular solids, and mixed consistencies were assessed. Anterior oral containment, mastication, oral manipulation, and A-P transfer were WFLs. Complete oral clearance occurred. No overt clinical s/sx concerning for aspiration were demonstrated. Pt denied any globus sensation.

Overall, at this time pt presents with oropharyngeal swallow that is WFLs. Recommend a regular texture/thin liquid diet. No need for skilled SLP intervention to address swallow. SLP to sign off. Please re-consult SLP service should further concerns arise.

CONCLUSIONS:
OROPHARYNGEAL SWALLOW IS WFLS. NO OVERT CLINICAL S/SX CONCERNING FOR ASPIRATION.

RECOMMENDATIONS:
1. Diet: regular texture/thin liquids
2. Medications: as tolerated - whole w/ liquids or floated in puree
3. Standard aspiration precautions, up at 90 degrees

Environmental Safety Implemented : Bed in low position, Call device within reach

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

ST Non Timed Modalities 2024
ST Eval Oral/Swallow Func Minutes : 20
ST Total Non Timed : 20

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Speech Therapy

DOCUMENT TYPE:	Clinical Swallowing - Text
SERVICE DATE/TIME:	3/6/2024 12:17 PST
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Hernandez SLP,Gardenia (3/6/2024 16:12 PST)
SIGNED INFORMATION:	Hernandez SLP,Gardenia (3/6/2024 16:12 PST)

ST Bedside Swallow Eval Entered On: 3/6/2024 15:55 PST
Performed On: 3/6/2024 12:17 PST by Hernandez SLP, Gardenia

General Information
Speech Orders : ST Bedside Swallow Exam - 03/05/24 14:54:00 PST, Bedside Swallow Exam, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, IV Yes, O2 Yes, 16443966655.0
ST Eval and Treat - 03/05/24 14:54:00 PST, Bedside Swallow Exam, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, IV Yes, O2 Yes
Precautions RTF : Active Orders:
» Oxygen Therapy, 03/03/24 23:27:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered
» Oxygen Therapy, 03/03/24 23:23:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 94%, Ordered

Speech Therapy

History of Swallow Studies : No qualifying data available

Hernandez SLP, Gardenia - 3/6/2024 15:55 PST

Pre-Assessment
Oxygen Therapy : Nasal cannula
Respirations : Other: WFLs
Tracheostomy Information : Oxygen Flow Rate: 2 L/min Performed by: Cruz, Nathan 03/06/24 12:00:00

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Oral-Facial Exam
Facial Movement : Symmetric resting/crying
Dentition : Own teeth

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Oral Structure Grid
Labial Structure : Within functional limits
Mandible : Within functional limits
Lingual Structure : Within functional limits
Hard Palate : Within functional limits

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Oral Function Grid
Labial : Within functional limits
Mandible : Within functional limits
Lingual : Within functional limits

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Volitional Cough : Within functional limits
Secretion Management : Adequate

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Swallowing Exam
Patient Position : Upright 90 degrees

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Liquids Swallowing Exam Adult Grid

	Ice Chips
Comment :	please see additional info
	Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Assessment/Recommendations
Clinical Swallowing Impression Grid
Within Functional Limits : Solids, Liquids

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Diet Consistency : Regular
Liquid Viscosity : Thin
Medications : Other: as tolerated – whole w/ liquids or floated in puree
Supervision : Intermittent
Positioning : Upright 90 degrees
Clinical Swallowing Recommendations : Intervention not indicated

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Speech Therapy

Education
Learner(s) Present for Session : Demonstration, Explanation
Learner(s) Present : Self
Barriers to Learning : None evident
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Document learning evaluation for Responsible Learner(s)
Swallowing Strategies : Verbalizes understanding, Demonstrates
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Long Term Goals
SLP Patient, Caregiver Goals : N/A
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Additional Information
Additional Information SLP : RM: 402
Minutes: 20

Per RN okay to proceed with swallow evaluation as ordered per MD.

66 y/o female admitted for: generalized asthenia, hyponatremia, AKI, UTI, facial cellulitis, severe sepsis, and dehydration. PMHx positive for: epilepsy NOS, TBI with resultant mild right hemiparesis, CHF, HTN, hypothyroidism, mood disorder NOS, and cardiac arrest per medical record. CXR results from 3/3/24 and head CT results from 3/4/24 were negative for any acute findings. CT Maxillofacial w/o contrast from 3/4/24 noted. Pt on 2L O2 via nasal cannula and on a minced & moist texture/thin liquid diet per MD. Pt was AA&Ox3-4. Impressions as follows.

Pt's vocal quality was strong and clear. Volitional cough was strong and dry. Oral motor structure and function assessment was WFLs. Hyo-laryngeal elevation was palpable. PO trials of thin liquids, purees, regular solids, and mixed consistencies were assessed. Anterior oral containment, mastication, oral manipulation, and A-P transfer were WFLs. Complete oral clearance occurred. No overt clinical s/sx concerning for aspiration were demonstrated. Pt denied any globus sensation.

Overall, at this time pt presents with oropharyngeal swallow that is WFLs. Recommend a regular texture/thin liquid diet. No need for skilled SLP intervention to address swallow. SLP to sign off. Please re-consult SLP service should further concerns arise.

CONCLUSIONS:
OROPHARYNGEAL SWALLOW IS WFLS. NO OVERT CLINICAL S/SX CONCERNING FOR ASPIRATION.

RECOMMENDATIONS:
1. Diet: regular texture/thin liquids
2. Medications: as tolerated - whole w/ liquids or floated in puree
3. Standard aspiration precautions, up at 90 degrees

Environmental Safety Implemented : Bed in low position, Call device within reach
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

ST Non Timed Modalities 2024
ST Eval Oral/Swallow Func Minutes : 20
ST Total Non Timed : 20
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Progress Note - Physician

DOCUMENT NAME: Progress Note-Physician
SERVICE DATE/TIME: 3/7/2024 13:29 PST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Piceno MD,Jessie M (3/7/2024 13:32 PST)
SIGN INFORMATION: Piceno MD,Jessie M (3/7/2024 13:32 PST)

NNC Nephro PN - Wolfbrandt

Patient: **WOLFBRANDT, DONNA J** **MRN: CHH7301725** **FIN: CHH0008011012203**
Age: **66 years** Sex: **Female** DOB: **3/25/1957**
Associated Diagnoses: **None**
Author: **Piceno MD, Jessie M**

Subjective

CC: Hyponatremia, AKI

Pt reports feeling better but with sob
RN reports na level remains low

Review of Systems

Constitutional: Weakness.
Respiratory: No shortness of breath.
Cardiovascular: No chest pain.

Objective

VS/Measurements

Inpt. Vital Signs (ST)

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	36.7	(MAR 07 11:51)	36.8	(MAR 06 14:00)	**H** 37.6	(MAR 06 16:36)
Heart Rate Monitored	92	(MAR 07 11:51)	76	(MAR 06 14:00)	92	(MAR 07 11:51)
Resp Rate	17	(MAR 07 11:51)	16	(MAR 06 14:00)	17	(MAR 06 18:00)
SBP	**H** 159	(MAR 07 11:51)	125	(MAR 06 16:36)	**H** 162	(MAR 07 07:45)
DBP	79	(MAR 07 11:51)	66	(MAR 07 00:42)	**H** 86	(MAR 06 16:36)
SpO2	97	(MAR 07 11:51)	96	(MAR 06 14:00)	99	(MAR 06 16:36)
	on 1 L/min		on 2 L/min		on 2 L/min	
	Nasal cannula		Nasal cannula		Nasal cannula	

General: Alert and oriented, No acute distress.
Eye: Extraocular movements are intact, Normal conjunctiva.
HENT: Normocephalic, dry oral mucosa.
Neck: Supple, No jugular venous distention.
Respiratory: Respirations are non-labored, Symmetrical chest wall expansion.
Cardiovascular: Normal rate, Regular rhythm.
Gastrointestinal: Soft, Non-distended.
Integumentary: Warm, Intact.
Neurologic: Alert, Oriented.
Psychiatric: Cooperative, Appropriate mood & affect.

Health Status

Allergies:

Progress Note - Physician

Allergies (1) Active	**Reaction**
No Known Allergies	None Documented

Current medications: Inpatient medications (ST)
Medications (14) Active
SCHEDULED: (13)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (0)
PRN: (1)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H

Problem list:
Active Problems (13)
Alcohol abuse
Anxiety
At risk for falls
At risk of pressure sore
Cardiac arrest
Convulsion
Depression
Hypothyroid
Ineffective airway clearance
MRSA
Obesity
Pneumonia
Ventilator dependence

Assessment
Interpretation of Results
Laboratory: Fishbone Labs ST
MAR 07 03:15

L 128 | **L 97** | 16 **/**
_____ 89

4.3 | 25 | 0.92 ****

MAR 07 03:15

Progress Note - Physician

```
        \  10.8    /
L 3.42  _______  205

        /  L 31.5  \.
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Plan

AKI (acute kidney injury) (N17.9)
- denies hx of ckd, prior cr of 1 on 1/20/23
- due to pre-renal from poor po in setting of diuresis
- improved with ivf per below

Critical hyponatremia - due to hypovolemia in setting of diuresis
- urine studies support hypovolemia
- s/p 1l ns on admission
- na correcting at appropriate rate
- adjusting ivf composition but now discontinued
- tolvaptan 7.5 mg po x 1 as now with early hypervolemia

Hypokalemia (E87.6) - due to poor po and diuretics
- kcl added to ivf, improved
- replace per protocol

CHF EF 30-35%
- diuretics and sglt2i on hold

Dental abscess (K04.7)
Facial cellulitis (L03.211)
Severe sepsis (R65.20)
Weakness (R53.1)

Thank you for consult. Will cont to follow closely.

Electronically Signed By: Piceno, Jessie MD
On: 03.07.2024 13:32 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/7/2024 07:20 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Zaugg DO,Garet J (3/7/2024 08:34 PST)
SIGN INFORMATION:	Zaugg DO,Garet J (3/7/2024 08:34 PST)

Las Vegas Neurology Center Note

Progress Note - Physician

SUBJECTIVE:

Patient seen and examined. No overnight events noted.

Patient in good spirits this morning. Denies facial pain. Denies fevers or chills. Feels as though she has a little more energy. Remains seizure-free, on home AEDs.

Review of systems other than mentioned above was negative upon patient interview.
I reviewed the chart, labs, orders, and all events of the last 48 hours.

OBJECTIVE:

Vital Signs (last 24 hrs)	**Last Charted**	**Minimum**	**Maximum**
Heart Rate	78 (MAR 07 04:35)	76 (MAR 06 14:00)	94 (MAR 06 10:00)
Resp Rate	16 (MAR 07 04:35)	16 (MAR 06 10:00)	17 (MAR 06 18:00)
SBP	132 (MAR 07 04:35)	125 (MAR 06 16:36)	**H** 157 (MAR 06 10:00)
DBP	73 (MAR 07 04:35)	66 (MAR 07 00:42)	**H** 86 (MAR 06 16:36)
SpO2	96 (MAR 07 04:35)	96 (MAR 06 14:00)	99 (MAR 06 16:36)

Medications (14) Active
Scheduled: (13)
buPROPion 150 mg/12 hours SR Tab 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab 500 mg 1 Tabs, Oral, BID
Protocol Protocol, N/A, q72H Interval
Protocol Protocol, N/A, q72H Interval
Protocol Protocol, N/A, q72H Interval
Protocol Protocol, N/A, q72H Interval
Protocol Protocol, N/A, q72H Interval
zonisamide 100 mg Cap 200 mg 2 Caps, Oral, Daily
Continuous: (0)
PRN: (1)
acetaminophen 500 mg Tab 500 mg 1 Tabs, Oral, q6H

Labs (Last four charted values)

WBC	L **3.42** (MAR 07)	L **3.60** (MAR 06)	4.60 (MAR 05)	H **10.94** (MAR 03)
Hgb	10.8 (MAR 07)	10.7 (MAR 06)	11.9 (MAR 05)	12.6 (MAR 03)
Hct	L **31.5** (MAR 07)	L **31.0** (MAR 06)	33.8 (MAR 05)	34.0 (MAR 03)
Plt	205 (MAR 07)	186 (MAR 06)	176 (MAR 05)	202 (MAR 03)
Na	L **128** (MAR 07)	C **125** (MAR 06)	L **126** (MAR 06)	L **126** (MAR 06)
K	4.3 (MAR 07)	4.8 (MAR 06)	4.1 (MAR 06)	L **3.4** (MAR 06)
CO2	25 (MAR 07)	26 (MAR 06)	28 (MAR 06)	27 (MAR 06)
Cl	L **97** (MAR 07)	L **97** (MAR 06)	L **94** (MAR 06)	L **91** (MAR 06)
Cr	0.92 (MAR 07)	0.93 (MAR 06)	0.92 (MAR 06)	1.00 (MAR 06)
BUN	16 (MAR 07)	16 (MAR 06)	18 (MAR 06)	H **20** (MAR 06)
Glucose Random	89 (MAR 07)	93 (MAR 06)	H **109** (MAR 06)	95 (MAR 06)
Mg	2.0 (MAR 07)	2.1 (MAR 06)		
Phos	3.0 (MAR 05)			
Ca	9.2 (MAR 07)	9.2 (MAR 06)	9.0 (MAR 06)	9.0 (MAR 06)
PT	10.9 (MAR 03)			
INR	1.0 (MAR 03)			

Progress Note - Physician

PHYSICAL EXAM:

GEN: Tired, oriented X 3/ No Acute Distress

HEENT: Atraumatic/Normocephalic

Neck: supple, no JVD

NEURO:

Mood/Affect: Appropriate to situation

Attention/Concentration: Intact

Fund of Knowledge: Fair

Speech: Fluent

CN 2-12: Left nasolabial fold blunting due to facial cellulitis

EOMI/PERRLA

Motor Strength: 4/5 R arm, 4-/5 R leg, 4+/5 L arm, 4/5 L leg

Tone/Bulk: Intact

Sensation: Intact to light touch

Reflexes: 2/4 throughout

Finger-nose-finger: No dysmetria

Ambulation: deferred

Babinski: negative bilaterally

Imaging:

ASSESSMENT:
Generalized asthenia in setting of the below, improving
Hyponatremia
AKI
UTI
Facial cellulitis
Epilepsy NOS
History of TBI with resultant mild right hemiparesis
CHF
Hypertension
Hypothyroidism
Mood disorder NOS

Progress Note - Physician

PLAN:
CT brain reassuring
No indication for further neurologic workup, etiology of generalized weakness is known, no new focal deficits
Continue home Keppra, Zonegran
PT/OT when clinically appropriate
BP control
Follow nephrology recs
Sepsis management per IM

Garet Zaugg, D.O.
Board-Certified Neurologist
Las Vegas Neurology Center

Electronically Signed By: Zaugg, Garet DO
On: 03.07.2024 08:34 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/6/2024 18:47 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Piceno MD,Jessie M (3/6/2024 18:49 PST)
SIGN INFORMATION:	Piceno MD,Jessie M (3/6/2024 18:49 PST)

NNC Nephro PN - Wolfbrandt

Patient: **WOLFBRANDT, DONNA J** **MRN: CHH7301725** **FIN: CHH0008011012203**
Age: **66 years** Sex: **Female** DOB: **3/25/1957**
Associated Diagnoses: **None**
Author: **Piceno MD, Jessie M**

Subjective
CC: Hyponatremia, AKI

Pt reports feeling better, diet being advanced
RN reports na levels

Review of Systems
Constitutional: Weakness.
Respiratory: No shortness of breath.
Cardiovascular: No chest pain.

Objective
VS/Measurements
Inpt. Vital Signs (ST)

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	**H** 37.6	(MAR 06 16:36)			**L** 36.3	(MAR 05 20:05) **H** 37.6 (MAR 06 16:36)
Heart Rate Monitored	80	(MAR 06 16:36)	61	(MAR 06 04:31)	92	(MAR 06 10:00)
Resp Rate	16	(MAR 06 16:36)	16	(MAR 05 20:05)	16	(MAR 05 20:05)
SBP	125	(MAR 06 16:36)	123	(MAR 06 00:03)	**H** 157	(MAR 06 10:00)
DBP	**H** 86	(MAR 06 16:36)	**L** 58	(MAR 06 00:03)	**H** 86	(MAR 06 16:36)

Progress Note - Physician

SpO2	99 (MAR 06 16:36)	**L** 93 (MAR 06 04:31)	99 (MAR 06 00:03)
	on 2 L/min	on 2 L/min	on 2 L/min
	Nasal cannula	Nasal cannula	Nasal cannula

General: Alert and oriented, No acute distress.
Eye: Extraocular movements are intact, Normal conjunctiva.
HENT: Normocephalic, dry oral mucosa.
Neck: Supple, No jugular venous distention.
Respiratory: Respirations are non-labored, Symmetrical chest wall expansion.
Cardiovascular: Normal rate, Regular rhythm.
Gastrointestinal: Soft, Non-distended.
Integumentary: Warm, Intact.
Neurologic: Alert, Oriented.
Psychiatric: Cooperative, Appropriate mood & affect.

Health Status

Allergies:

Allergies (1) Active	**Reaction**
No Known Allergies	None Documented

Current medications: Inpatient medications (ST)
Medications (15) Active
SCHEDULED: (13)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq (NS w/K20 20 mEq) 20 mEq 1,000 mL, IV, 100 mL/hr
PRN: (1)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H

Problem list:
Active Problems (13)
Alcohol abuse
Anxiety
At risk for falls
At risk of pressure sore
Cardiac arrest
Convulsion
Depression

Progress Note - Physician

Hypothyroid
Ineffective airway clearance
MRSA
Obesity
Pneumonia
Ventilator dependence

Assessment

Interpretation of Results

Laboratory: Fishbone Labs ST
MAR 06 16:26

L 126 | **L 94** | 18 **/**
_____ **H 109**

4.1 | 28 | 0.92 ****

MAR 06 04:24

**** 10.7 **/**
L 3.60 _____ 186

/ **L 31.0** ****.

Plan

AKI (acute kidney injury) (N17.9)
- denies hx of ckd, prior cr of 1 on 1/20/23
- due to pre-renal from poor po in setting of diuresis
- improving with ivf per below

Critical hyponatremia - due to hypovolemia in setting of diuresis
- urine studies support hypovolemia
- s/p 1l ns on admission
- na correcting at appropriate rate
- adjusting ivf composition and rate per na level, will d/c if na improved at 2100
- bmp at 2100 then in am
- cont fluid restriction 1500 ml/day

Hypokalemia (E87.6) - due to poor po and diuretics
- kcl added to ivf
- replace per protocol

CHF EF 30-35%
- diuretics and sglt2i on hold

Progress Note - Physician

Dental abscess (K04.7)
Facial cellulitis (L03.211)
Severe sepsis (R65.20)
Weakness (R53.1)

Thank you for consult. Will cont to follow closely.

Electronically Signed By: Piceno, Jessie MD
On: 03.06.2024 18:49 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/6/2024 17:45 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Smuckler DO,Jason M (3/6/2024 17:45 PST)
SIGN INFORMATION:	Smuckler DO,Jason M (3/6/2024 17:45 PST)

Subjective

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History
Active Problems(10)
Alcohol abuse
Anxiety
Cardiac arrest
Convulsion
Depression
Hypothyroid
MRSA
Obesity
Pneumonia
Ventilator dependence

Past Surgical History
TRACH

Social History
No social history documented.

Family History
No Family History documented.

Progress Note - Physician

Objective

Measurements (most recent)

Height (cm)	175.26 cm	Weidner RN, Robyn	03/03 23:13
Height (inches)	69.00 inch	SYSTEM	03/03 23:13
Weight	99.79 kg	Weidner RN, Robyn	03/03 23:13
Weight (lbs)	220.00 lb	SYSTEM	03/03 23:13
Body Mass Index	32.49 kg/m2	Weidner RN, Robyn	03/03 23:13

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	**H** 37.6	03/06/2024 16:36	**L** 36.3	03/05/2024 20:05	**H** 37.6	03/06/2024 16:36
Heart Rate Monitored	80	03/06/2024 16:36	61	03/06/2024 04:31	92	03/06/2024 10:00
Resp Rate	16	03/06/2024 16:36	16	03/05/2024 20:05	16	03/05/2024 20:05
SBP	125	03/06/2024 16:36	123	03/06/2024 00:03	**H** 157	03/06/2024 10:00
DBP	**H** 86	03/06/2024 16:36	**L** 58	03/06/2024 00:03	**H** 86	03/06/2024 16:36
MAP	101	03/06/2024 16:36	84	03/06/2024 00:03	101	03/06/2024 16:36
SpO2	99	03/06/2024 16:36	**L** 93	03/06/2024 04:31	99	03/06/2024 00:03
O2 Flow Rate	on 2 l/min		on 2 l/min		on 2 l/min	
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Intake and Output (current encounter)

	03/04 02:25 03/06 17:45	03/06 07:00 03/06 17:45	03/05 07:00 03/06 06:59	03/04 07:00 03/05 06:59	03/03 07:00 03/04 06:59
Intake	2996.8	360	1586.8	50	1000
Output	2825	375	1325	1125	0
Net Total	171.8	-15	261.8	-1075	1000
Stool Count	0	0	0	0	0

Physical Exam

Home Medications

buPROPion (BuPROPion (Eqv-Wellbutrin SR) 150 mg/12 hours oral tablet, extended release) 150 Milligram 1 Tabs By Mouth 2 Times a Day

Progress Note - Physician

dapagliflozin (dapagliflozin 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
escitalopram (escitalopram 10 mg oral tablet) 20 Milligram 2 Tabs By Mouth Daily
famotidine (famotidine 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
furosemide (furosemide 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
levETIRAcetam (levETIRAcetam 500 mg oral tablet) 500 Milligram 1 Tabs By Mouth 2 Times a Day
losartan (losartan 50 mg oral tablet) 50 Milligram 1 Tabs By Mouth Daily
metoprolol (Metoprolol Succinate ER 25 mg oral tablet, extended release) 25 Milligram 1 Tabs By Mouth Daily
zonisamide (Zonegran 100 mg oral capsule) 200 Milligram 2 Capsules By Mouth Daily

Inpatient Medications
Medications (16) Active
SCHEDULED: (14)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
potassium chloride 20 mEq Oral Packet (potassium chloride) 30 mEq, Oral, q3H
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq (NS w/K20 20 mEq) 20 mEq 1,000 mL, IV, 100 mL/hr
PRN: (1)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H

Results
Recent Labs
General Chemistry

Glucose Level	109 mg/dL (High)	03/06/2024 16:26
POC Glucose	82 mg/dL (Normal)	03/06/2024 16:28
Sodium	126 mmol/L (Low)	03/06/2024 16:26
Potassium	4.1 mmol/L (Normal)	03/06/2024 16:26
Chloride	94 mmol/L (Low)	03/06/2024 16:26
CO2	28 mmol/L (Normal)	03/06/2024 16:26
Anion Gap	4 mmol/L (Low)	03/06/2024 16:26
BUN	18 mg/dL (Normal)	03/06/2024 16:26
Creatinine	0.92 mg/dL (Normal)	03/06/2024 16:26
BUN/Creat Ratio	20 (Normal)	03/06/2024 16:26

Progress Note - Physician

Calcium	9.0 mg/dL (Normal)	03/06/2024 16:26
Albumin. Level	3.6 gm/dL (Normal)	03/06/2024 04:24
TP	6.3 gm/dL (Low)	03/06/2024 04:24
A/G Ratio	1.3 (Normal)	03/06/2024 04:24
T Bili	0.9 mg/dL (Normal)	03/06/2024 04:24
Alk Phos	86 units/L (Normal)	03/06/2024 04:24
AST	20 units/L (Normal)	03/06/2024 04:24
ALT	20 units/L (Normal)	03/06/2024 04:24
Mg Lvl	2.1 mg/dL (Normal)	03/06/2024 04:24
Phosphorus	3.0 mg/dL (Normal)	03/05/2024 08:04
Estimated Creatinine Clearance	62.86 mL/min ()	03/06/2024 17:05
eGFR Cr	69 mL/min/1.73m2 (N/A)	03/06/2024 16:26
eGFR Pediatric	Not Reported (N/A)	03/06/2024 16:26
MAR Glucose Result	82 ()	03/06/2024 16:33

General Hematology

WBC	3.60 x10e3/mcL (Low)	03/06/2024 04:24
RBC	2.53 x10e6/mcL (Low)	03/06/2024 04:24
Hgb	10.7 gm/dL (Normal)	03/06/2024 04:24
Hct	31.0 % (Low)	03/06/2024 04:24
MCV	122.5 Femtoliters (High)	03/06/2024 04:24
MCH	42.3 pg (High)	03/06/2024 04:24
MCHC	34.5 gm/dL (Normal)	03/06/2024 04:24
RDW-SD	62.6 Femtoliters (High)	03/06/2024 04:24
RDW-CV	13.8 % (Normal)	03/06/2024 04:24
Plt	186 x10e3/mcL (Normal)	03/06/2024 04:24
MPV	9.4 Femtoliters (Normal)	03/06/2024 04:24
Neut % Auto	70.0 % (N/A)	03/05/2024 08:04
Lymph % Auto	17.2 % (N/A)	03/05/2024 08:04
Mono % Auto	11.3 % (N/A)	03/05/2024 08:04
Eos % Auto	0.4 % (N/A)	03/05/2024 08:04
Baso % Auto	0.7 % (N/A)	03/05/2024 08:04
Immature Grans %	0.4 % (Normal)	03/05/2024 08:04
Neut # Auto	3.22 x10e3/mcL (Normal)	03/05/2024 08:04
Lymph # Auto	0.79 x10e3/mcL (Low)	03/05/2024 08:04

Progress Note - Physician

Mono # Auto	0.52 x10e3/mcL (Normal)	03/05/2024 08:04
Eos # Auto	0.02 x10e3/mcL (Normal)	03/05/2024 08:04
Baso # Auto	0.03 x10e3/mcL (Normal)	03/05/2024 08:04
Immature Grans # Auto	0.02 x10e3/mcL (Normal)	03/05/2024 08:04

Other Chemistry

Procalcitonin Lvl	0.57 ng/mL (High)	03/05/2024 08:04

Abnormal Labs

General Chemistry

Anion Gap 4 mmol/L (Low) 03/06/2024 16:26
Chloride 94 mmol/L (Low) 03/06/2024 16:26
Glucose Level 109 mg/dL (High) 03/06/2024 16:26
Sodium 126 mmol/L (Low) 03/06/2024 16:26
TP 6.3 gm/dL (Low) 03/06/2024 04:24

General Hematology

Hct 31.0 % (Low) 03/06/2024 04:24
MCH 42.3 pg (High) 03/06/2024 04:24
MCV 122.5 Femtoliters (High) 03/06/2024 04:24
RBC 2.53 x10e6/mcL (Low) 03/06/2024 04:24
RDW-SD 62.6 Femtoliters (High) 03/06/2024 04:24
WBC 3.60 x10e3/mcL (Low) 03/06/2024 04:24

Note: Critical results are displayed in red.

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**3.60 L** 03/06/2024 04:24	**4.60** 03/05/2024 08:04	**10.94 H** 03/03/2024 23:49	
Hgb	**10.7** 03/06/2024 04:24	**11.9** 03/05/2024 08:04	**12.6** 03/03/2024 23:49	
Hct	**31.0 L** 03/06/2024 04:24	**33.8** 03/05/2024 08:04	**34.0** 03/03/2024 23:49	
Baso # Auto	**0.03** 03/05/2024 08:04	**0.02** 03/03/2024 23:49		
Baso % Auto	**0.7** 03/05/2024 08:04	**0.2** 03/03/2024 23:49		
Eos # Auto	**0.02** 03/05/2024 08:04	**0.02** 03/03/2024 23:49		
Eos % Auto	**0.4** 03/05/2024 08:04	**0.2** 03/03/2024 23:49		

Progress Note - Physician

Immature Grans %	**0.4** 03/05/2024 08:04	**0.6** 03/03/2024 23:49		
Immature Grans # Auto	**0.02** 03/05/2024 08:04	**0.07** 03/03/2024 23:49		
Lymph # Auto	**0.79 L** 03/05/2024 08:04	**0.79 L** 03/03/2024 23:49		
Lymph % Auto	**17.2** 03/05/2024 08:04	**7.2** 03/03/2024 23:49		
Mono # Auto	**0.52** 03/05/2024 08:04	**0.91 H** 03/03/2024 23:49		
Mono % Auto	**11.3** 03/05/2024 08:04	**8.3** 03/03/2024 23:49		
Neut # Auto	**3.22** 03/05/2024 08:04	**9.13 H** 03/03/2024 23:49		
Neut % Auto	**70.0** 03/05/2024 08:04	**83.5** 03/03/2024 23:49		
Plt	**186** 03/06/2024 04:24	**176** 03/05/2024 08:04	**202** 03/03/2024 23:49	
Glucose Level	**109 H** 03/06/2024 16:26	**95** 03/06/2024 04:24	**97** 03/05/2024 21:11	**93** 03/05/2024 08:04
Sodium	**126 L** 03/06/2024 16:26	**126 L** 03/06/2024 04:24	**125** 03/05/2024 21:11	**125** 03/05/2024 12:13
Potassium	**4.1** 03/06/2024 16:26	**3.4 L** 03/06/2024 04:24	**3.3 L** 03/05/2024 21:11	**3.5** 03/05/2024 08:04
Chloride	**94 L** 03/06/2024 16:26	**91 L** 03/06/2024 04:24	**92 L** 03/05/2024 21:11	**86 L** 03/05/2024 08:04
CO2	**28** 03/06/2024 16:26	**27** 03/06/2024 04:24	**27** 03/05/2024 21:11	**28** 03/05/2024 08:04
Creatinine	**0.92** 03/06/2024 16:26	**1.00** 03/06/2024 04:24	**1.13 H** 03/05/2024 21:11	**1.41 H** 03/05/2024 08:04
BUN	**18** 03/06/2024 16:26	**20 H** 03/06/2024 04:24	**18** 03/05/2024 21:11	**23 H** 03/05/2024 08:04
BUN/Creat Ratio	**20** 03/06/2024 16:26	**20** 03/06/2024 04:24	**16** 03/05/2024 21:11	**16** 03/05/2024 08:04
Mg Lvl	**2.1** 03/06/2024 04:24			
Phosphorus	**3.0** 03/05/2024 08:04			
Calcium	**9.0** 03/06/2024 16:26	**9.0** 03/06/2024 04:24	**9.0** 03/05/2024 21:11	**9.2** 03/05/2024 08:04
Albumin. Level	**3.6** 03/06/2024 04:24	**3.9** 03/05/2024 08:04		
Ur Protein	**Negative** 03/04/2024 00:47			
T Bili	**0.9** 03/06/2024 04:24			

Progress Note - Physician

Alk Phos	**86** 03/06/2024 04:24			
AST	**20** 03/06/2024 04:24			
ALT	**20** 03/06/2024 04:24			
PT	**10.9** 03/03/2024 23:49			
INR	**1.0** 03/03/2024 23:49			

Blood Glucose Trend

Glucose Level: 109 mg/dL High (03/06/24 16:26:00)
Glucose Level: 95 mg/dL (03/06/24 04:24:00)
Glucose Level: 97 mg/dL (03/05/24 21:11:00)
POC Glucose: 82 mg/dL (03/06/24 16:28:00)
POC Glucose: 89 mg/dL (03/06/24 12:23:00)
POC Glucose: 84 mg/dL (03/06/24 06:27:00)
POC Glucose: 79 mg/dL (03/05/24 23:47:00)

CBC (Last Within 24hrs)
WBC: 3.6 x10e3/mcL Low (04:24)
Hgb: 10.7 gm/dL (04:24)
Hct: 31 % Low (04:24)
Plt: 186 x10e3/mcL (04:24)

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 126 mmol/L Low (16:26)
Potassium: 4.1 mmol/L (16:26)
Chloride: 94 mmol/L Low (16:26)
CO2: 28 mmol/L (16:26)
BUN: 18 mg/dL (16:26)
Creatinine: 0.92 mg/dL (16:26)
Glucose Level: 109 mg/dL High (16:26)
Calcium: 9 mg/dL (16:26)
Mg Lvl: 2.1 mg/dL (04:24)

LFT (Last Within 24hrs)
AST: 20 units/L (04:24)
ALT: 20 units/L (04:24)
Alk Phos: 86 units/L (04:24)
T Bili: 0.9 mg/dL (04:24)

Progress Note - Physician

TP: 6.3 gm/dL Low (04:24)

Cardiology Labs
Troponin TNIH: 4.33 ng/L (03/04/24 02:25:00)
Troponin TNIH: 4.14 ng/L (03/04/24 00:56:00)
Troponin TNIH: 5.61 ng/L (03/03/24 23:49:00)

Assessment/Plan
Diagnoses
AKI (acute kidney injury) (N17.9)
Dental abscess (K04.7)
Facial cellulitis (L03.211)
Hypochloremia (E87.8)
Hypokalemia (E87.6)
Hyponatremia (E87.1)
Severe sepsis (R65.20)
Weakness (R53.1)
1. Hyponatremia (E87.1)

Assessment: NAEO
Complains of weakness and trouble with ambulation
Says she has a home PCA and home health set up
On 4LNC
Sodium is 126

Physical Exam:
General: Alert, in no acute cardiopulmonary distress. Obese
Mental Status: Oriented to person, place and time. Normal affect.
Head: Normocephalic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact.
Ear, Nose and Throat: Oropharynx clear, mucous membranes moist. Trachea midline.
Neck: Supple, Full range of motion.
Respiratory: Clear to auscultation. No wheezing, rales or rhonchi.
Cardiovascular: Heart sounds normal. No thrills. Regular rate and rhythm, no murmurs, rubs or gallops.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Normal bowel sounds.
Genitourinary: No costovertebral angle tenderness.
Neurologic: right sided weakness at baseline,
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.

Assessment:
Severe Hyponatremia, improving
Hypokalemia
Klebsiella UTI
Facial cellulitis

Progress Note - Physician

Generalized weakness
Fatigued
Dehydration
AKI
History of TBI
UTI
Severe sepsis, present on admission, source UTI versus facial cellulitis
history of cardiac arrest
Hypertension
Obesity

Plan:
IMC
Remove Foley
Does not use oxygen at home, oxygen challenge once more stable
Follow urine Cx.
Follow up repeat blood culture
Diet advanced from clears to minced and moist. Consistency to be adjusted pending swallow eval.
Nephrology following, appreciate recommendations.
Neurology following appreciate recs.
Continue home Keppra, Zonegran
IVF per nephrology.
Monitor I/O closely
Urine studies
Serial Sodium checks
Monitor neuro status
PT/OT
Full code
Activity: Up ad lib.
DVT prophylaxis: SCDs
Case management consulted to confirm home health

Discharge Planning: HHC vs SNF pending clinical course, stable renal, improved sodium, Cx results.

Jason Smuckler DO
Internal Medicine
Platinum Hospitalists
Please page YouCallMD with any questions or concerns

Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Skilled Nursing Unit/Facility

Electronically Signed By: Smuckler, Jason DO
On: 03.06.2024 17:45 PST

Progress Note - Physician

DOCUMENT NAME: Progress Note-Physician
SERVICE DATE/TIME: 3/6/2024 07:05 PST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Zaugg DO,Garet J (3/6/2024 11:26 PST)
SIGN INFORMATION: Zaugg DO,Garet J (3/6/2024 11:26 PST)

Las Vegas Neurology Center Note

SUBJECTIVE:

Patient seen and examined. No overnight events noted.

Patient concerned she did not receive her zonisamide for some reason yesterday. Discussed with patient and zonisamide given this morning. She endorses improvement of facial pain. Feels slightly less fatigued. Renal function improving.

Review of systems other than mentioned above was negative upon patient interview.
I reviewed the chart, labs, orders, and all events of the last 48 hours.

OBJECTIVE:

Vital Signs (last 24 hrs)	Last Charted	Minimum	Maximum
Heart Rate	84 (MAR 06 07:23)	61 (MAR 06 04:31)	84 (MAR 06 07:23)
Resp Rate	16 (MAR 06 07:23)	16 (MAR 05 12:35)	18 (MAR 05 11:50)
SBP	130 (MAR 06 07:23)	111 (MAR 05 13:20)	**H** 136 (MAR 05 11:50)
DBP	76 (MAR 06 07:23)	**L** 58 (MAR 05 13:20)	76 (MAR 06 07:23)
SpO2	96 (MAR 06 07:23)	**L** 92 (MAR 05 12:35)	100 (MAR 05 11:50)

Medications (10) Active
Scheduled: (8)
buPROPion 150 mg/12 hours SR Tab 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab 500 mg 1 Tabs, Oral, BID
zonisamide 100 mg Cap 200 mg 2 Caps, Oral, Daily
Continuous: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq 20 mEq 1,000 mL, IV, 100 mL/hr
PRN: (1)
acetaminophen 500 mg Tab 500 mg 1 Tabs, Oral, q6H

Labs (Last four charted values)

WBC	L **3.60** (MAR 06)	4.60 (MAR 05)	H **10.94** (MAR 03)	
Hgb	10.7 (MAR 06)	11.9 (MAR 05)	12.6 (MAR 03)	
Hct	L **31.0** (MAR 06)	33.8 (MAR 05)	34.0 (MAR 03)	
Plt	186 (MAR 06)	176 (MAR 05)	202 (MAR 03)	
Na	L **126** (MAR 06)	C **125** (MAR 05)	C **125** (MAR 05)	C **123** (MAR 05)
K	L **3.4** (MAR 06)	L **3.3** (MAR 05)	3.5 (MAR 05)	C **2.5** (MAR 04)
CO2	27 (MAR 06)	27 (MAR 05)	28 (MAR 05)	28 (MAR 04)
Cl	L **91** (MAR 06)	L **92** (MAR 05)	L **86** (MAR 05)	L **89** (MAR 04)
Cr	1.00 (MAR 06)	H **1.13** (MAR 05)	H **1.41** (MAR 05)	H **1.60** (MAR 04)
BUN	H **20** (MAR 06)	18 (MAR 05)	H **23** (MAR 05)	H **22** (MAR 04)

Progress Note - Physician

Glucose Random 95 (MAR 06) 97 (MAR 05) 93 (MAR 05) 87 (MAR 04)
Mg 2.1 (MAR 06)
Phos 3.0 (MAR 05)
Ca 9.0 (MAR 06) 9.0 (MAR 05) 9.2 (MAR 05) L **7.4** (MAR 04)
PT 10.9 (MAR 03)
INR 1.0 (MAR 03)

PHYSICAL EXAM:

GEN: Tired, oriented X 3/ No Acute Distress

HEENT: Atraumatic/Normocephalic

Neck: supple, no JVD

NEURO:

Mood/Affect: Appropriate to situation

Attention/Concentration: Intact

Fund of Knowledge: Fair

Speech: Fluent

CN 2-12: Left nasolabial fold blunting due to facial cellulitis

EOMI/PERRLA

Motor Strength: 4/5 R arm, 4-/5 R leg, 4+/5 L arm, 4/5 L leg

Tone/Bulk: Intact

Sensation: Intact to light touch

Reflexes: 2/4 throughout

Finger-nose-finger: No dysmetria

Ambulation: deferred

Babinski: negative bilaterally

Imaging:

ASSESSMENT:
Generalized asthenia in setting of the below
Hyponatremia
AKI
UTI
Facial cellulitis
Epilepsy NOS

Progress Note - Physician

History of TBI with resultant mild right hemiparesis
CHF
Hypertension
Hypothyroidism
Mood disorder NOS

PLAN:
CT brain reassuring
No indication for further neurologic workup, etiology of generalized weakness is known, no new focal deficits
Follow nephrology recs
Continue home Keppra, Zonegran
PT/OT when clinically appropriate
BP control
Sepsis management per IM

Garet Zaugg, D.O.
Board-Certified Neurologist
Las Vegas Neurology Center

Electronically Signed By: Zaugg, Garet DO
On: 03.06.2024 11:26 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/5/2024 22:26 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Tiwari,Subhikshya MD (3/5/2024 22:26 PST)
SIGN INFORMATION:	Tiwari,Subhikshya MD (3/5/2024 22:26 PST)

Subjective

Chief Complaint/Reason for Consultation
Chief Complaint: Weakness the past few days, had a GLF today. No injury, no LOC, denies hitting her head, no blood thinners. LVFR noted that pt was also hypotensive on scene. Dex 115

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History
Active Problems(10)
Alcohol abuse
Anxiety
Cardiac arrest
Convulsion
Depression
Hypothyroid
MRSA
Obesity
Pneumonia
Ventilator dependence

Progress Note - Physician

Past Surgical History
TRACH

Social History
No social history documented.

Family History
No Family History documented.

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	**L** 36.3	03/05/2024 20:05	**L** 36.4	03/05/2024 03:00	**L** 36.4	03/05/2024 03:00
Heart Rate Monitored	74	03/05/2024 20:05	63	03/05/2024 02:00	79	03/05/2024 10:34
Resp Rate	16	03/05/2024 20:05	16	03/05/2024 04:00	18	03/05/2024 07:52
SBP	134	03/05/2024 20:05	103	03/05/2024 10:34	**H** 136	03/05/2024 11:50
DBP	68	03/05/2024 20:05	**L** 52	03/05/2024 02:00	77	03/05/2024 11:23
MAP	96	03/05/2024 20:05	71	03/05/2024 02:00	97	03/05/2024 11:23
SpO2	98	03/05/2024 20:05	**L** 92	03/05/2024 12:35	100	03/05/2024 03:00
O2 Flow Rate	on 2 l/min		on 2 l/min		on 2 l/min	
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Pain Scores (Last Within 24hrs)
Numeric Pain Scale: 0 = No pain (20:00)

Intake and Output (current encounter)

	03/04 02:25 03/05 22:26	03/05 07:00 03/05 22:26	03/04 07:00 03/05 06:59	03/03 07:00 03/04 06:59	03/02 07:00 03/03 06:59
Intake	1301.5	251.5	50	1000	0
Output	2100	975	1125	0	0
Net Total	-798.5	-723.5	-1075	1000	0

Progress Note - Physician

Precautions
No Precautions documented.

Glasgow Coma Scale
Eye Opening Response Glasgow: Spontaneously (03/05/24 20:00:00)
Best Verbal Response Glasgow: Oriented and converses (03/05/24 20:00:00)
Best Motor Response Glasgow: Obeys commands (03/05/24 20:00:00)
Glasgow Coma Score: 15 (03/05/24 20:00:00)

Stroke Assessments
Glasgow Coma Scale
Eye Opening Response Glasgow: Spontaneously (03/05/24 20:00:00)
Best Verbal Response Glasgow: Oriented and converses (03/05/24 20:00:00)
Best Motor Response Glasgow: Obeys commands (03/05/24 20:00:00)
Glasgow Coma Score: 15 (03/05/24 20:00:00)

Physical Exam

Home Medications
buPROPion (BuPROPion (Eqv-Wellbutrin SR) 150 mg/12 hours oral tablet, extended release) 150 Milligram 1 Tabs By Mouth 2 Times a Day
dapagliflozin (dapagliflozin 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
escitalopram (escitalopram 10 mg oral tablet) 20 Milligram 2 Tabs By Mouth Daily
famotidine (famotidine 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
furosemide (furosemide 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
levETIRAcetam (levETIRAcetam 500 mg oral tablet) 500 Milligram 1 Tabs By Mouth 2 Times a Day
losartan (losartan 50 mg oral tablet) 50 Milligram 1 Tabs By Mouth Daily
metoprolol (Metoprolol Succinate ER 25 mg oral tablet, extended release) 25 Milligram 1 Tabs By Mouth Daily
zonisamide (Zonegran 100 mg oral capsule) 200 Milligram 2 Capsules By Mouth Daily

Inpatient Medications
Medications (10) Active
SCHEDULED: (8)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H

Progress Note - Physician

Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq (NS w/K20 20 mEq) 20 mEq 1,000 mL, IV, 75 mL/hr
PRN: (1)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefepime 2 gm, 20 mL, 12.5 mL/hr, IV Piggyback, q12H Interval	2	03/05/2024 17:24	03/04/2024 02:55

Vaccinations and Immunoprophylaxis
COVID-19 Vaccine Status: Fully +1 or more booster(s) (03/05/24 14:01:00)
COVID-19 Vaccine Status: Fully (> 2 weeks since last dose) (03/04/24 00:22:00)
COVID-19 Vaccine Status: Fully +1 or more booster(s) (01/05/23 16:38:00)

Results
Abnormal Labs

General Chemistry
Chloride 92 mmol/L (Low) 03/05/2024 21:11
Creatinine 1.13 mg/dL (High) 03/05/2024 21:11
Potassium 3.3 mmol/L (Low) 03/05/2024 21:11
Sodium 125 mmol/L (Critical) 03/05/2024 21:11

General Hematology
Lymph # Auto 0.79 x10e3/mcL (Low) 03/05/2024 08:04
MCH 41.6 pg (High) 03/05/2024 08:04
MCHC 35.2 gm/dL (High) 03/05/2024 08:04
MCV 118.2 Femtoliters (High) 03/05/2024 08:04
RBC 2.86 x10e6/mcL (Low) 03/05/2024 08:04
RDW-SD 63.6 Femtoliters (High) 03/05/2024 08:04

Other Chemistry
Procalcitonin Lvl 0.57 ng/mL (High) 03/05/2024 08:04

Note: Critical results are displayed in red.

Progress Note - Physician

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**4.60** 03/05/2024 08:04	**10.94 H** 03/03/2024 23:49		
Hgb	**11.9** 03/05/2024 08:04	**12.6** 03/03/2024 23:49		
Hct	**33.8** 03/05/2024 08:04	**34.0** 03/03/2024 23:49		
Baso # Auto	**0.03** 03/05/2024 08:04	**0.02** 03/03/2024 23:49		
Baso % Auto	**0.7** 03/05/2024 08:04	**0.2** 03/03/2024 23:49		
Eos # Auto	**0.02** 03/05/2024 08:04	**0.02** 03/03/2024 23:49		
Eos % Auto	**0.4** 03/05/2024 08:04	**0.2** 03/03/2024 23:49		
Immature Grans %	**0.4** 03/05/2024 08:04	**0.6** 03/03/2024 23:49		
Immature Grans # Auto	**0.02** 03/05/2024 08:04	**0.07** 03/03/2024 23:49		
Lymph # Auto	**0.79 L** 03/05/2024 08:04	**0.79 L** 03/03/2024 23:49		
Lymph % Auto	**17.2** 03/05/2024 08:04	**7.2** 03/03/2024 23:49		
Mono # Auto	**0.52** 03/05/2024 08:04	**0.91 H** 03/03/2024 23:49		
Mono % Auto	**11.3** 03/05/2024 08:04	**8.3** 03/03/2024 23:49		
Neut # Auto	**3.22** 03/05/2024 08:04	**9.13 H** 03/03/2024 23:49		
Neut % Auto	**70.0** 03/05/2024 08:04	**83.5** 03/03/2024 23:49		
Plt	**176** 03/05/2024 08:04	**202** 03/03/2024 23:49		
Glucose Level	**97** 03/05/2024 21:11	**93** 03/05/2024 08:04	**87** 03/04/2024 12:00	**117 H** 03/03/2024 23:49
Sodium	**125** 03/05/2024 21:11	**125** 03/05/2024 12:13	**123** 03/05/2024 08:43	**122** 03/05/2024 08:04
Potassium	**3.3 L** 03/05/2024 21:11	**3.5** 03/05/2024 08:04	**2.5** 03/04/2024 12:00	**3.2 L** 03/03/2024 23:49
Chloride	**92 L** 03/05/2024 21:11	**86 L** 03/05/2024 08:04	**89 L** 03/04/2024 12:00	**76** 03/03/2024 23:49
CO2	**27** 03/05/2024 21:11	**28** 03/05/2024 08:04	**28** 03/04/2024 12:00	**31** 03/03/2024 23:49
Creatinine	**1.13 H** 03/05/2024 21:11	**1.41 H** 03/05/2024 08:04	**1.60 H** 03/04/2024 12:00	**1.85 H** 03/03/2024 23:49

Progress Note - Physician

BUN	**18** 03/05/2024 21:11	**23 H** 03/05/2024 08:04	**22 H** 03/04/2024 12:00	**22 H** 03/03/2024 23:49
BUN/Creat Ratio	**16** 03/05/2024 21:11	**16** 03/05/2024 08:04	**14** 03/04/2024 12:00	**12** 03/03/2024 23:49
Phosphorus	**3.0** 03/05/2024 08:04			
Calcium	**9.0** 03/05/2024 21:11	**9.2** 03/05/2024 08:04	**7.4 L** 03/04/2024 12:00	**9.3** 03/03/2024 23:49
Albumin. Level	**3.9** 03/05/2024 08:04			
Ur Protein	**Negative** 03/04/2024 00:47			
PT	**10.9** 03/03/2024 23:49			
INR	**1.0** 03/03/2024 23:49			

Blood Glucose Trend

Glucose Level: 97 mg/dL (03/05/24 21:11:00)
Glucose Level: 93 mg/dL (03/05/24 08:04:00)
POC Glucose: 93 mg/dL (03/05/24 17:23:00)
POC Glucose: 100 mg/dL (03/05/24 11:42:00)
POC Glucose: 96 mg/dL (03/05/24 07:50:00)

CBC (Last Within 24hrs)
WBC: 4.6 x10e3/mcL (08:04)
Hgb: 11.9 gm/dL (08:04)
Hct: 33.8 % (08:04)
Plt: 176 x10e3/mcL (08:04)

Differential (Last Within 24hrs)
- Automated -
Neut % Auto: 70 % (08:04)
Lymph % Auto: 17.2 % (08:04)
Mono % Auto: 11.3 % (08:04)
Eos % Auto: 0.4 % (08:04)
Baso % Auto: 0.7 % (08:04)
Immature Grans %: 0.4 % (08:04)
Neut # Auto: 3.22 x10e3/mcL (08:04)
Lymph # Auto: 0.79 x10e3/mcL Low (08:04)
Mono # Auto: 0.52 x10e3/mcL (08:04)
Baso # Auto: 0.03 x10e3/mcL (08:04)

Immature Grans # Auto: 0.02 x10e3/mcL (08:04)

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 125 mmol/L Critical (21:11)
Potassium: 3.3 mmol/L Low (21:11)
Chloride: 92 mmol/L Low (21:11)
CO2: 27 mmol/L (21:11)
BUN: 18 mg/dL (21:11)
Creatinine: 1.13 mg/dL High (21:11)
Glucose Level: 97 mg/dL (21:11)
Calcium: 9 mg/dL (21:11)
Phosphorus: 3 mg/dL (08:04)

Microbiology

Culture Urine | **Completed**

Source:	U CleanCatch	Body Site:	
Collected Dt/Tm:	03/04/2024 00:47	Last Updated Dt/Tm:	03/05/2024 22:25

>100,000 cfu/ml Klebsiella pneumoniae

	Klebsiella pneumoniae	
Drug	MIC interp	BMIC Dilut
Amikacin	S	<=16
Amoxicillin/Clavulanate	S	<=4/2
Ampicillin	R	>16
Ampicillin/Sulbactam	S	8/4
Aztreonam	S	<=4
Cefazolin	S	<=2
Cefepime	S	<=2
Ceftriaxone	S	<=1
Ciprofloxacin	S	<=1
Gentamicin	S	<=1
Levofloxacin	S	<=0.25
Nitrofurantoin	S	<=32
Piperacillin/Tazobactam	S	<=4
Tetracycline	S	<=4
Tobramycin	S	<=1

Progress Note - Physician

Trimethoprim/Sulfamethoxazole	S	<=2/38

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/03/2024 23:49	Last Updated Dt/Tm:	03/05/2024 08:01

No growth at 24 hours.

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/03/2024 23:49	Last Updated Dt/Tm:	03/05/2024 08:01

No growth at 24 hours.

Cardiology Labs
Troponin TNIH: 4.33 ng/L (03/04/24 02:25:00)
Troponin TNIH: 4.14 ng/L (03/04/24 00:56:00)
Troponin TNIH: 5.61 ng/L (03/03/24 23:49:00)

Assessment/Plan
Diagnoses
AKI (acute kidney injury) (N17.9)
Dental abscess (K04.7)
Facial cellulitis (L03.211)
Hypochloremia (E87.8)
Hypokalemia (E87.6)
Hyponatremia (E87.1)
Severe sepsis (R65.20)
Weakness (R53.1)
1. Hyponatremia (E87.1)

Assessment: late entry
Subjective
States she is feeling much better as compared to yesterday. Feeling less weak. Discussed labs and Rx plan.

10 pt ROS neg except as mentioned above.

Physical Exam
General: Alert, in no acute cardiopulmonary distress. Obese

Progress Note - Physician

Mental Status: Oriented to person, place and time. Normal affect.
Head: Normocephalic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact.
Ear, Nose and Throat: Oropharynx clear, mucous membranes moist. Trachea midline.
Neck: Supple, Full range of motion.
Respiratory: Clear to auscultation. No wheezing, rales or rhonchi.
Cardiovascular: Heart sounds normal. No thrills. Regular rate and rhythm, no murmurs, rubs or gallops.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Normal bowel sounds.
Genitourinary: No costovertebral angle tenderness.
Neurologic: right sided weakness at baseline,
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.

Assessment
Severe Hyponatremia, improving
Hypokalemia
Klebsiella UTI
Facial cellulitis
Generalized weakness
Fatigued
Dehydration
AKI
History of TBI
UTI
Severe sepsis, present on admission, source UTI versus facial cellulitis
history of cardiac arrest
Hypertension
Obesity

PLAN:
Downgraded from ICU to IMC
Does not use oxygen at home, oxygen challenge once more stable
Follow urine Cx.
Diet advanced from clears to minced and moist. Consistency to be adjusted pending swallow eval.
Nephrology following, appreciate recommendations.
Neurology following appreciate recs.
IVF per nephrology.
Monitor I/O closely
Urine studies
Serial Sodium checks
Monitor neuro status
PT/OT

Full code

Activity: Up ad lib.
DVT prophylaxis: SCDs

Discharge Planning: HHC vs SNF pending clinical course, stable renal, improved sodium, Cx results.

Portions of this record are transcribed using voice recognition software and may contain

Progress Note - Physician

inadvertent errors with dictation.

Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Skilled Nursing Unit/Facility

Order Date/Time	Order Action	Order Name	Order Detail
03/05/2024 21:56	Modify	Admission/Observation/Transfer	03/04/24 2:25:00 PST, Estimated LOS 2 to 3 Nights, Admit as Inpatient, Suggested Placement: Intermediate Care/Step down, Level of Care Acute Intermediate Care, Admitting: Timple DO, Julie M Attending: Smuckler DO, Jason M, Hyponatremia
03/05/2024 21:05	Order	POC Glu	Blood, Collected, RT collect, 03/05/24 17:23:00 PST
03/05/2024 15:55	Order	Mg (Magnesium Level)	Blood, Lab Collect, AM Draw collect, 03/06/24 4:00:00 PST, Daily, for 7 Days, Stop date 03/13/24 3:59:00 PDT, Print Label By Order Location
03/05/2024 15:55	Order	CMP	Blood, Lab Collect, AM Draw collect, 03/06/24 4:00:00 PST, Daily, for 7 Times, Stop date 03/13/24 3:59:00 PDT, Print Label By Order Location
03/05/2024 15:55	Order	CBC NoDiff (CBC no Diff)	Blood, Lab Collect, AM Draw collect, 03/06/24 4:00:00 PST, Daily, for 7 Times, Stop date 03/13/24 3:59:00 PDT, Print Label By Order Location
03/05/2024 15:54	Order	Nutritional Supplements	03/05/24 15:54:00 PST, Ensure Clear Berry, Breakfast/Lunch/Dinner
03/05/2024 15:53	Modify	Diet	03/04/24 9:30:00 PST, Cardiac, Consistency: 5 - Minced and Moist (Dysphagia), Fluid Restriction: 1500 mL, Consistency 0 - Thin, Dietitian to Manage: All Diet Parameters
03/05/2024 14:54	Order	ST Bedside Swallow Exam	03/05/24 14:54:00 PST, Bedside Swallow Exam, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, IV Yes, O2 Yes, 16443966655.0
03/05/2024 14:54	Order	Speech Therapy Eval and Treat (ST Eval and Treat)	03/05/24 14:54:00 PST, Bedside Swallow Exam, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, IV Yes, O2 Yes
03/05/2024 14:54	Order	Occupational Therapy Eval and Treat (OT Evaluation and Treatment)	03/05/24 14:54:00 PST, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, Yes, Yes
03/05/2024 14:54	Order	Physical Therapy Eval and Treat (PT Evaluation and Treatment)	03/05/24 14:54:00 PST, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, Yes, Yes
03/05/2024 12:21	Order	acetaminophen 500 mg Tab (Tylenol)	500 mg, 1 Tabs, Oral, q6H, PRN: Fever/Pain (1-3)
03/05/2024 12:20	Modify	Admission/Observation/Transfer	03/04/24 2:25:00 PST, Estimated LOS 2 to 3 Nights, Admit as Inpatient, Suggested Placement: Intermediate Care/Step down, Level of Care Acute Intermediate Care, Admitting: Timple DO, Julie M Attending: Tiwari, Subhikshya MD, Hyponatremia

Progress Note - Physician

Electronically Signed By: Tiwari, Subhikshya
On: 03.05.2024 22:26 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/5/2024 12:24 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Piceno MD,Jessie M (3/5/2024 12:31 PST)
SIGN INFORMATION:	Piceno MD,Jessie M (3/5/2024 12:31 PST)

NNC Nephro PN - Wolfbrandt

Patient: **WOLFBRANDT, DONNA J** **MRN: CHH7301725** **FIN: CHH0008011012203**
Age: **66 years** Sex: **Female** DOB: **3/25/1957**
Associated Diagnoses: **None**
Author: **Piceno MD, Jessie M**

Subjective

CC: Hyponatremia, AKI

Pt reports feeling better, tolerating cld, requesting advance diet
RN reports na levels

Review of Systems

Constitutional: Weakness.
Respiratory: No shortness of breath.
Cardiovascular: No chest pain.

Objective

VS/Measurements

Inpt. Vital Signs (ST)

Vital Signs (24 hrs)	Last Charted	Minimum	Maximum
Temp	36.7 (MAR 05 11:50)	**L** 36.4 (MAR 05 03:00)	37.0 (MAR 04 20:03)
Peripheral Pulse Rate	69 (MAR 05 11:50)	**L** 59 (MAR 04 17:00)	78 (MAR 05 09:09)
Resp Rate	18 (MAR 05 11:50)	16 (MAR 04 13:01)	18 (MAR 05 07:52)
SBP	**H** 136 (MAR 05 11:50)	90 (MAR 04 18:00)	**H** 136 (MAR 05 11:50)
DBP	66 (MAR 05 11:50)	**L** 50 (MAR 04 17:00)	77 (MAR 05 11:23)
SpO2	100 (MAR 05 11:50)	**L** 90 (MAR 04 16:00)	100 (MAR 04 13:01)
	on 2 L/min	on 2 L/min	on 2 L/min
	Nasal cannula	Nasal cannula	Nasal cannula

General: Alert and oriented, No acute distress.
Eye: Extraocular movements are intact, Normal conjunctiva.
HENT: Normocephalic, dry oral mucosa.
Neck: Supple, No jugular venous distention.
Respiratory: Respirations are non-labored, Symmetrical chest wall expansion.
Cardiovascular: Normal rate, Regular rhythm.
Gastrointestinal: Soft, Non-distended.
Integumentary: Warm, Intact.
Neurologic: Alert, Oriented.

Progress Note - Physician

Psychiatric: Cooperative, Appropriate mood & affect.

Health Status

Allergies:

Allergies (1) Active	**Reaction**
No Known Allergies	None Documented

Current medications: Inpatient medications (ST)
Medications (10) Active
SCHEDULED: (8)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq (NS w/K20 20 mEq) 20 mEq 1,000 mL, IV, 75 mL/hr
PRN: (1)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H

Problem list:
Active Problems (12)
Alcohol abuse
Anxiety
At risk for falls
Cardiac arrest
Convulsion
Depression
Hypothyroid
Ineffective airway clearance
MRSA
Obesity
Pneumonia
Ventilator dependence

Assessment

Interpretation of Results

Laboratory: Fishbone Labs ST
MAR 05 08:04

C 122 | L 86 | H 23 /
_____ 93

3.5 | 28 **| H 1.41 **

MAR 05 08:04

Progress Note - Physician

```
      \  11.9   /
4.60  _______  176

      /  33.8   \.
```

Plan

AKI (acute kidney injury) (N17.9)
- denies hx of ckd, prior cr of 1 on 1/20/23
- due to pre-renal from poor po in setting of diuresis
- improving with ivf per below

Critical hyponatremia - due to hypovolemia in setting of diuresis
- urine studies support hypovolemia
- s/p 1l ns on admission
- na correcting at appropriate rate
- adjusting ivf composition and rate per na level
- cont Na q4 for now
- initial Na 116 at midnight, goal of ~8 per 24 hours; **goal of ~128 at midnight**
- cont fluid restriction 1500 ml/day

Hypokalemia (E87.6) - due to poor po and diuretics
- kcl added to ivf
- replace per protocol

CHF EF 30-35%
- diuretics and sglt2i on hold

Dental abscess (K04.7)
Facial cellulitis (L03.211)
Severe sepsis (R65.20)
Weakness (R53.1)

Thank you for consult. Will cont to follow closely.

Electronically Signed By: Piceno, Jessie MD
On: 03.05.2024 12:31 PST

DOCUMENT NAME: Progress Note-Physician
SERVICE DATE/TIME: 3/5/2024 10:23 PST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Valina DO,Anisa M (3/5/2024 10:24 PST)
SIGN INFORMATION: Valina DO,Anisa M (3/5/2024 10:24 PST)

Progress Note - Physician

Subjective

No event overnight.
patient sodium is 123 this morning.
Patient resting comfortably.

Past Medical History

Active Problems(10)

Alcohol abuse
Anxiety
Cardiac arrest
Convulsion
Depression
Hypothyroid
MRSA
Obesity
Pneumonia
Ventilator dependence

Past Surgical History

TRACH

Objective

Measurements (most recent)

Height (cm)	175.26 cm	Weidner RN, Robyn	03/03 23:13
Height (inches)	69.00 inch	SYSTEM	03/03 23:13
Weight	99.79 kg	Weidner RN, Robyn	03/03 23:13
Weight (lbs)	220.00 lb	SYSTEM	03/03 23:13
Body Mass Index	32.49 kg/m2	Weidner RN, Robyn	03/03 23:13

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	36.8	03/05/2024 07:02	**L** 36.4	03/05/2024 03:00	37.0	03/04/2024 20:03
Heart Rate Monitored	76	03/05/2024 09:09	**L** 59	03/04/2024 13:01	76	03/05/2024 09:09
Resp Rate	17	03/05/2024 09:09	16	03/04/2024 11:31	18	03/05/2024 07:52
SBP	107	03/05/2024 09:09	90	03/04/2024 18:00	133	03/05/2024 08:29
DBP	**L** 57	03/05/2024 09:09	**L** 50	03/04/2024 17:00	72	03/05/2024 01:00
MAP	77	03/05/2024 09:09	67	03/04/2024 18:00	91	03/04/2024 12:00
SpO2	95	03/05/2024 09:09	**L** 90	03/04/2024 16:00	100	03/04/2024 12:00
O2 Flow Rate	on 2 l/min		on 2 l/min			
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Progress Note - Physician

Intake and Output (current encounter)

	03/04 02:25 03/05 10:23	03/05 07:00 03/05 10:23	03/04 07:00 03/05 06:59	03/03 07:00 03/04 06:59	03/02 07:00 03/03 06:59
Intake	1050	0	50	1000	0
Output	1125	0	1125	0	0
Net Total	-75	0	-1075	1000	0

Physical Exam

General: In no acute cardiopulmonary distress. obesity.
HEENT: NC, AT, PERRLA, Neck is supple. Full ROM.
Respiratory: Clear to auscultation.
Cardiovascular: S1, S2, RRR.
Gastrointestinal: Abdomen soft, non-tender, abd obesity.
Neurologic: No focal neurological deficit.
Extremities: no C/C/E. Pulses intact.

Inpatient Medications
Medications (9) Active
SCHEDULED: (8)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq (NS w/K20 20 mEq) 20 mEq 1,000 mL, IV, 75 mL/hr
PRN: (0)

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefepime 2 gm, 20 mL, 12.5 mL/hr, IV Piggyback, q12H Interval	2	03/05/2024 03:30	03/04/2024 02:55

Results
Recent Labs

Progress Note - Physician

Cardiac

Troponin TNIH	4.33 ng/L (Normal)	03/04/2024 02:25

General Chemistry

Glucose Level	93 mg/dL (Normal)	03/05/2024 08:04
POC Glucose	96 mg/dL (Normal)	03/05/2024 07:50
Sodium	123 mmol/L (Critical)	03/05/2024 08:43
Potassium	3.5 mmol/L (Normal)	03/05/2024 08:04
Chloride	86 mmol/L (Low)	03/05/2024 08:04
CO2	28 mmol/L (Normal)	03/05/2024 08:04
Anion Gap	8 mmol/L (Normal)	03/05/2024 08:04
BUN	23 mg/dL (High)	03/05/2024 08:04
Creatinine	1.41 mg/dL (High)	03/05/2024 08:04
BUN/Creat Ratio	16 (Normal)	03/05/2024 08:04
Calcium	9.2 mg/dL (Normal)	03/05/2024 08:04
Albumin. Level	3.9 gm/dL (Normal)	03/05/2024 08:04
Phosphorus	3.0 mg/dL (Normal)	03/05/2024 08:04
Estimated Creatinine Clearance	41.02 mL/min ()	03/05/2024 09:12
eGFR Cr	41 mL/min/1.73m2 (N/A)	03/05/2024 08:04
eGFR Pediatric	Not Reported (N/A)	03/05/2024 08:04
Uric Acid	6.9 mg/dL (High)	03/04/2024 05:11
Lactic Acid Lvl	1.4 mmol/L (Normal)	03/04/2024 02:25
MAR Glucose Result	96 ()	03/05/2024 07:52

General Coagulation

PT	10.9 Seconds (Normal)	03/03/2024 23:49
INR	1.0 (Normal)	03/03/2024 23:49

General Hematology

WBC	4.60 x10e3/mcL (Normal)	03/05/2024 08:04
RBC	2.86 x10e6/mcL (Low)	03/05/2024 08:04
Hgb	11.9 gm/dL (Normal)	03/05/2024 08:04
Hct	33.8 % (Normal)	03/05/2024 08:04
MCV	118.2 Femtoliters (High)	03/05/2024 08:04
MCH	41.6 pg (High)	03/05/2024 08:04
MCHC	35.2 gm/dL (High)	03/05/2024 08:04
RDW-SD	63.6 Femtoliters (High)	03/05/2024 08:04

Progress Note - Physician

RDW-CV	14.5 % (Normal)	03/05/2024 08:04
Plt	176 x10e3/mcL (Normal)	03/05/2024 08:04
MPV	9.6 Femtoliters (Normal)	03/05/2024 08:04
Neut % Auto	70.0 % (N/A)	03/05/2024 08:04
Lymph % Auto	17.2 % (N/A)	03/05/2024 08:04
Mono % Auto	11.3 % (N/A)	03/05/2024 08:04
Eos % Auto	0.4 % (N/A)	03/05/2024 08:04
Baso % Auto	0.7 % (N/A)	03/05/2024 08:04
Immature Grans %	0.4 % (Normal)	03/05/2024 08:04
Neut # Auto	3.22 x10e3/mcL (Normal)	03/05/2024 08:04
Lymph # Auto	0.79 x10e3/mcL (Low)	03/05/2024 08:04
Mono # Auto	0.52 x10e3/mcL (Normal)	03/05/2024 08:04
Eos # Auto	0.02 x10e3/mcL (Normal)	03/05/2024 08:04
Baso # Auto	0.03 x10e3/mcL (Normal)	03/05/2024 08:04
Immature Grans # Auto	0.02 x10e3/mcL (Normal)	03/05/2024 08:04
RBC Morph	Abnormal (Abnormal)	03/03/2024 23:49
Plt Morphology	Normal (Normal)	03/03/2024 23:49
Macro	2+ (Abnormal)	03/03/2024 23:49
Plt Est	Adequate (Normal)	03/03/2024 23:49

Other Chemistry

Procalcitonin Lvl	0.57 ng/mL (High)	03/05/2024 08:04

Thyroid

TSH Ultrasensitive	13.707 mc Intl units/mL (High)	03/04/2024 05:11

UA Macroscopic

Ur Color	Yellow (Normal)	03/04/2024 00:47
Ur Clarity	Cloudy (Abnormal)	03/04/2024 00:47
UR Glucose	Negative (Normal)	03/04/2024 00:47
Ur Bili	Negative (Normal)	03/04/2024 00:47
Ur Ketones	Negative (Normal)	03/04/2024 00:47
Ur Spec Grav	1.009 (Low)	03/04/2024 00:47
Ur Blood	Negative (Normal)	03/04/2024 00:47
Ur pH	5.0 (Normal)	03/04/2024 00:47

Progress Note - Physician

Ur Protein	Negative (Normal)	03/04/2024 00:47
Ur Urobilinogen	<2.0 mg/dL (Normal)	03/04/2024 00:47
Ur Nitrite	Negative (Normal)	03/04/2024 00:47
Ur Leuk Est	3+ (Abnormal)	03/04/2024 00:47
UA Culture?	Yes (N/A)	03/04/2024 00:47

UA Microscopic

UA WBC Inst	>182 /HPF (High)	03/04/2024 00:47
UA RBC Inst	5 /HPF (High)	03/04/2024 00:47
Ur Bacteria	3+ /HPF (Abnormal)	03/04/2024 00:47
UA Squam Epi Inst	3 /HPF (Normal)	03/04/2024 00:47
Ur WBC Clumps	Few /HPF (Abnormal)	03/04/2024 00:47
Ur Mucous	Rare /LPF (Abnormal)	03/04/2024 00:47
Ur Hyal Cast	6 /LPF (High)	03/04/2024 00:47

Urine Chemistry

Ur Sodium	34 mmol/L (N/A)	03/04/2024 02:18
Ur Creat	73.9 mg/dL (N/A)	03/04/2024 02:18
Ur Chloride	60 mmol/L (N/A)	03/04/2024 02:18
Ur Osmolality	274 mOsmol/kg (Normal)	03/04/2024 02:18

Blood Glucose Trend

Glucose Level: 93 mg/dL (03/05/24 08:04:00)
Glucose Level: 87 mg/dL (03/04/24 12:00:00)
POC Glucose: 96 mg/dL (03/05/24 07:50:00)

Microbiology

Culture Urine **Preliminary**

Source: U CleanCatch Body Site:
Collected Dt/Tm: 03/04/2024 00:47 Last Updated Dt/Tm: 03/05/2024 03:17

```
>100,000 cfu/ml Klebsiella pneumoniae
Susceptibility to follow.
```

Culture Blood ARD **Preliminary**

Source: Blood Body Site:
Collected Dt/Tm: 03/03/2024 23:49 Last Updated Dt/Tm: 03/05/2024 08:01

Progress Note - Physician

No growth at 24 hours.

Culture Blood ARD		**Preliminary**	
Source:	Blood	Body Site:	
Collected Dt/Tm:	03/03/2024 23:49	Last Updated Dt/Tm:	03/05/2024 08:01

No growth at 24 hours.

Assessment/Plan
Donna Wolfbarndt
DOB: 03/25/1957

Assessment:
Hyponatremia
UTI
Acute kidney injury
Facial cellulitis
Severe sepsis, present on admission, source urinary versus skin soft tissue
history of traumatic brain injury, she has right sided weakness and balance issues, cardiac arrest, and hypertension

Hyponatremia (E87.1):
Admit to ICU, klebsialla
Every 4 hours BMP, follow result
Nephrology consult obtained, NNC
Close neurological monitoring
Seizure precautions
Avoid overcorrection, goal sodium correction 6-8 in 24 hours

AKI (acute kidney injury) (N17.9):
Nephrology following
Urinary catheter
Strict I's/O
Avoid nephrotoxic agents as able
Urine electrolytes pending

Severe sepsis (R65.20): Cefepime for empiric antibiotic coverage
Dose for renal function
Blood culture sent, follow result
Urine culture sent, follow result patient with left facial cellulitis and UA positive for UTI
No further IV fluid boluses to avoid overcorrection in sodium or further drop in sodium given patient with hyponatremia
Sepsis reassessed

Progress Note - Physician

VTE Prophylaxis: Subcutaneous heparin
» **VTE Prophylaxis Assessment:** VTE Prophylaxis Ordered

At this time patient remains critical, at risk of decompensation or death
37 minutes of cumulative critical care time were utilized, excluding procedures performed and discussion with family. This time includes patient history, exam, review of laboratory results and relevant imaging, and complex medical decision-making.
Above note was generated with the voice recognition software used by Cerner EMR. Efforts have been made to ensure accuracy of all notes, however, occasional wording errors and/or typos may persist.

Electronically Signed By: Valina, Anisa DO
On: 03.05.2024 10:24 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/5/2024 05:32 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Zaugg DO,Garet J (3/5/2024 08:12 PST)
SIGN INFORMATION:	Zaugg DO,Garet J (3/5/2024 08:12 PST)

Las Vegas Neurology Center Note

SUBJECTIVE:

Patient seen and examined. No overnight events noted.

Patient pleasant this morning, states she feels tired and fatigued. Nephrology managing AKI and hyponatremia. Patient states she feels about the same today, no new complaints. Patient remains ICU status. No seizures noted, continuing Keppra.

Review of systems other than mentioned above was negative upon patient interview.
I reviewed the chart, labs, orders, and all events of the last 48 hours.

OBJECTIVE:

Vital Signs (last 24 hrs)	Minimum	Maximum	Last Charted
Heart Rate	66 (MAR 05 07:52)	**L** 57 (MAR 04 09:00)	68 (MAR 05 00:03)
Resp Rate	18 (MAR 05 07:52)	16 (MAR 04 09:00)	18 (MAR 04 10:00)
SBP	126 (MAR 05 07:52)	90 (MAR 04 18:00)	132 (MAR 05 04:00)
DBP	**L** 58 (MAR 05 07:52)	**L** 50 (MAR 04 17:00)	72 (MAR 05 01:00)
SpO2	99 (MAR 05 07:52)	**L** 90 (MAR 04 16:00)	100 (MAR 04 10:00)

Medications (9) Active
Scheduled: (8)
buPROPion 150 mg/12 hours SR Tab 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab 20 mg 1 Tabs, Oral, Daily

Progress Note - Physician

heparin 5000 units/mL 1 mL Inj 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab 500 mg 1 Tabs, Oral, BID
zonisamide 100 mg Cap 200 mg 2 Caps, Oral, Daily
Continuous: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq 20 mEq 1,000 mL, IV, 50 mL/hr
PRN: (0)
Labs (Last four charted values)
WBC H **10.94** (MAR 03)
Hgb 12.6 (MAR 03)
Hct 34.0 (MAR 03)
Plt 202 (MAR 03)
Na C **125** (MAR 05) C **124** (MAR 05) C **122** (MAR 04) C **119** (MAR 04)
K C **2.5** (MAR 04) L **3.2** (MAR 03)
CO2 28 (MAR 04) 31 (MAR 03)
Cl L **89** (MAR 04) C **76** (MAR 03)
Cr H **1.60** (MAR 04) H **1.85** (MAR 03)
BUN H **22** (MAR 04) H **22** (MAR 03)
Glucose Random 87 (MAR 04) H **117** (MAR 03)
Ca L **7.4** (MAR 04) 9.3 (MAR 03)
PT 10.9 (MAR 03)
INR 1.0 (MAR 03)

PHYSICAL EXAM:

GEN: Tired, oriented X 3/ No Acute Distress

HEENT: Atraumatic/Normocephalic

Neck: supple, no JVD

NEURO:

Mood/Affect: Appropriate to situation

Attention/Concentration: Intact

Fund of Knowledge: Fair

Speech: Fluent

CN 2-12: Left nasolabial fold blunting due to facial cellulitis

EOMI/PERRLA

Motor Strength: 4-/5 R arm, 3+/5 R leg, 4/5 L arm, 4-/5 L leg

Tone/Bulk: Intact

Sensation: Intact to light touch

Reflexes: 2/4 throughout

Progress Note - Physician

Finger-nose-finger: No dysmetria

Ambulation: deferred

Babinski: negative bilaterally

Imaging:

ASSESSMENT:
Generalized asthenia in setting of the below
Hyponatremia
AKI
UTI
Facial cellulitis
History of TBI with resultant mild right hemiparesis
CHF
Hypertension
Hypothyroidism
Mood disorder NOS

PLAN:
CT brain reassuring
No indication for further neurologic workup, etiology of generalized weakness is known, no new focal deficits
Follow nephrology recs
Recommend avoiding cefepime given propensity for CNS toxicity, lowering of seizure threshold in setting of renal insufficiency
PT/OT when clinically appropriate
BP control
Sepsis management per ICU

Garet Zaugg, D.O.
Board-Certified Neurologist
Las Vegas Neurology Center

Electronically Signed By: Zaugg, Garet DO
On: 03.05.2024 08:12 PST

CHH- Centennial Hills Hospital

6900 N. Durango Dr.
Las Vegas, NV 89149-4409

(702) 835-9700

Patient Information

Patient Name: WOLFBRANDT, DONNA J
Home Address: 3501 SHADY TIMBER STREET
LAS VEGAS, NV 891292850
Home Phone: 7025759934
Employer Name:
Employer Phone:

Sex: Female
DOB: 03/25/1957
Age: 66 Years
Religion: Lutheran
SSN: XXX-XX-5943
Race: White

Guarantor Information

Guarantor Name: WOLFBRANDT, DONNA J
Patient's Reltn:
Billing Address: 3501 SHADY TIMBER STREET
LAS VEGAS, NV 891292850
Billing Phone: 7025759934
Employer Name:
Employer Phone:

Sex: Female
DOB: 03/25/1957
Age: 66 Years
Religion: Lutheran
SSN: XXX-XX-5943
Marital Status: Divorced

Contact Information

Emergency Contact
Contact Name: PEGGY REED
Patient's Reltn: Child
Sex:
Home Phone: 7025955345

Next of Kin
Contact Name: PEGGY REED
Patient's Reltn: Child
Sex:
Home Phone: 7025955345

Primary Insurance

Subscriber Name: WOLFBRANDT, DONNA J
Patient's Reltn:
Sex: Female
DOB: 03/25/1957
Age: 66 Years
Employer Name:
Employer Phone:
Financial Class: Medicare Acute Care
Group Name:

Insurance Name: MEDICARE PART A INVISION
Claim Address:

Insurance Phone:
Policy Number: 1PD7RV6FE54
Group Number:
Authorization Number:
Authorization Phone:
Authorization Contact:

Secondary Insurance

Subscriber Name: WOLFBRANDT, DONNA J
Patient's Reltn:
Sex: Female
DOB: 03/25/1957
Age: 66 Years
Employer Name:
Employer Phone:
Financial Class: Medicaid
Group Name:

Insurance Name: MEDICAID 2ND MCARE INVISION
Claim Address:

Insurance Phone:
Policy Number: 00001321767
Group Number:
Authorization Number:
Authorization Phone:
Authorization Contact:

Encounter Information

Reg Dt/Tm: 03/04/2024 02:25
Est Dt of Arrival: 03/05/2024 23:30
Inpt Adm Dt/Tm: 03/04/2024 02:25
Disch Dt/Tm:
Observation Dt/Tm:
VIP Indicator:
Admit Reason: HYPONATREMIA

Patient Type: Inpatient
Medical Service: IMC-Intermediate Ca
Location: CHH 4S
Room/Bed: 402 / 01
Isolation:
Disease Alert:

Admit Type: Emergency
Admit Source: Emergency Room
Advance Directive:
Reg Clerk: Contributor_system, CHH_I
Admit Physician: Timple DO, Julie M
Attend Physician: Smuckler DO, Jason
PCP: Family Forte

WOLFBRANDT, DONNA J Female / 66 Years

MRN: CHH7301725

FIN: CHH0008011012203

Printed By: USER, XR DEFAULT on 03/07/2024 15:08
Registration last updated by: Contributor_system, CHH_INVISION on 03/05/2024 23:30

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279

Admit: 3/4/2024
Disch: Disch Time:
FIN: CHH0008011012203
Readmit Risk: 32 - LOW 03/05/24 21:45

DOB/Sex: 3/25/1957 / Female

Attending: Smuckler DO,Jason M

Facesheets

DOCUMENT NAME:	Facesheets
SERVICE DATE/TIME:	3/4/2024 00:00 PST
RESULT STATUS:	Unauth
PERFORM INFORMATION:	
SIGN INFORMATION:	

FACE SHEET REGISTRATION FORM_20240304.pdf
Please click on link to see image.

Attachment(s): 3/4/2024 00:00 PST FACE SHEET REGISTRATION FORM_20240304.pdf

Centennial-Wolfbrandt, Donna J-Enc #8011012203-IPT-ICU- FACE SHEET REGISTRATION FORM - 3/4/2024 - 1 pg

MED REC# **7301725**

PT#	PATIENT NAME						
8011012203	WOLFBRANDT, DONNA J						
ADDRESS: 3501 SHADY TIMBER STREET UNIT 1015							COUNTY: 001
CITY: LAS VEGAS	STATE: NV	ZIP: 891292850	PHONE: (702)575-9934	PATIENT LANGUAGE: ENGLISH			
AGE: 66Y	D.O.B.: 03/25/1957	SEX: F	RACE: 1	MS: D	ETHNICITY:	PREVIOUS NAME: ,	RELIGION: LUT	INFANT AGE:
PT EMPLOYER: DISABLED,	TELEPHONE:	EXT:	OCCUPATION: DISABLED				
EMPLOYER ADDRESS:							
CITY:	STATE:	ZIP:	RETIREMENT DATE: 12/01/19	DISASTER TAG ID:			

Contact Information

NEAREST RELATIVE NAME: REED, PEGGY			
RLTN: N	ADDRESS: 2005 RUTH DR		
CITY: LAS VEGAS	STATE: NV	ZIP: 891231111	PHONE: (702)595-5345	EXT.
EMERGENCY CONTACT NAME: REED, PEGGY			
RLTN: N	ADDRESS: 2005 RUTH DR		
CITY: LAS VEGAS	STATE: NV	ZIP: 891081111	PHONE: (702)595-5345	EXT.

Guarantor Information

GUARANTOR NAME: WOLFBRANDT, DONNA J				RLTN: S
ADDRESS: 3501 SHADY TIMBER STREET UNI				PHONE: (702)575-9934
CITY: LAS VEGAS	STATE: NV	ZIP: 891292850	OCCUPATION: DISABLED	
GUARANTOR EMPLOYER: DISABLED			PHONE: (702)486-920	
ADDRESS:				
CITY: LAS VEGAS	STATE: NV	ZIP:		

Insurance Information

PC: M

INSURANCE NAME 1: MEDICARE PART A	PLAN: A00	PHONE: (800)633-4227	POLICY#: 1PD7RV6FE54	GROUP#	AUTH#	AUTHORIZED BY	
MAILING ADDRESS: ATTN NOVITAS SOLUTIO P O BOX 3385			CITY: MECHANISCBU	STATE: PA	ZIP: 17055		
SUBSCRIBER NAME: WOLFBRANDT, DONNA J	MAIL TO NAME: MEDICARE,	INS SEX: F	D.O.B.: 19570325	GNM: DISABLED			
COB: 1	PAYOR ID: A00	UR PHONE: (800)525-2395	EXT	INS VERIFIED	SUBSCRIBER RLTN: S	NOC REF NUMBER	NOC REF BY
INSURANCE NAME 2: MEDICAID 2ND MCRE	PLAN: D74	PHONE: (877)638-3472	POLICY#: 00001321767	GROUP#	AUTH#	AUTHORIZED BY	
MAILING ADDRESS: ATTN HP ENTERPRISE P O BOX 30042			CITY: RENO	STATE: NV	ZIP: 895203042		
SUBSCRIBER NAME: WOLFBRANDT, DONNA J	MAIL TO NAME: MEDICAID 2ND MCRE,	INS SEX: F	D.O.B.: 19570325	GNM: NEVADA			
COB: 2	PAYOR ID: D74	UR PHONE: (800)525-2395	EXT	INS VERIFIED	SUBSCRIBER RLTN: S	NOC REF NUMBER	NOC REF BY
INSURANCE NAME 3	PLAN	PHONE	POLICY#	GROUP#	AUTH#	AUTHORIZED BY	
MAILING ADDRESS			CITY	STATE	ZIP		
SUBSCRIBER NAME	MAIL TO NAME	INS SEX	D.O.B.	GNM			
COB	PAYOR ID	UR PHONE	EXT	INS VERIFIED	SUBSCRIBER RLTN	NOC REF NUMBER	NOC REF BY

Admitting Information

ADM DATE: 03/04/2024	TIME: 0225	SRC: EO	PT TYPE: D	PRI: X	ACCIDENT INFO.: H 03/03/2024	VOLUNTARY ADM	ORGAN DNR	ISOL	PT STS: IA	
ATTENDING PHYSICIAN: SMUCKLER JASON	ATTENDING #: 933267	REF SRC	ADM BY: FIERRF	PREV ADM DATE	PREVIOUS FACILITY					
ADM DIAGNOSIS: HYPONATREMIA	HOSP SVC: ICU	DIAGNOSTIC CODE	SURGERY DATE: //	BLDLESS: N	N/S: 4S	RM/BED: 402 01	Ethnicity			
ADMITTING PHYSICIAN: TIMPLE JULIE	ADMITTING #: 928721	DISCHARGE DATE	TIME	DISCHARGE DISP	PREVIOUS DISCHARGE DATE					
PROCEDURE	CLINICAL COMMENT	MODE OF ARRIVAL / ACCOMPANIED BY: AMB AMBULANCE								
REFERRING PHYSICIAN: NO REFERRING	REFERRING #: 999870	EMAIL ADDRESS								
REL OF INFO: Y	ADV DIR	LIVING WILL	LOCATION OF WILL / DIRECTIVE	FAMILY DOCTOR: FORTE FAMILY PRACTICE	FAMILY DOCTOR #: 200618	DIV: C				

Centennial Hills Hospital
6900 North Durango Drive
Las Vegas, NV 89149

EL0012

MEDICAL RECORD

UHS-9002
Rev. 10/2021

Patient Identification

8011012203-7301725

WOLFBRANDT, DONNA J
DOB: 03/25/1957 66Y SX: F ICU
MRN: 7301725 ADM/REG DT:03/04/2024
Centennial Hills Hospital

Page 1 of 1

Progress Note - Physician

DOCUMENT NAME: Progress Note-Physician
SERVICE DATE/TIME: 3/7/2024 13:29 PST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Piceno MD,Jessie M (3/7/2024 13:32 PST)
SIGN INFORMATION: Piceno MD,Jessie M (3/7/2024 13:32 PST)

NNC Nephro PN - Wolfbrandt

Patient: **WOLFBRANDT, DONNA J** **MRN: CHH7301725** **FIN: CHH0008011012203**
Age: **66 years** Sex: **Female** DOB: **3/25/1957**
Associated Diagnoses: **None**
Author: **Piceno MD, Jessie M**

Subjective

CC: Hyponatremia, AKI

Pt reports feeling better but with sob
RN reports na level remains low

Review of Systems

Constitutional: Weakness.
Respiratory: No shortness of breath.
Cardiovascular: No chest pain.

Objective

VS/Measurements

Inpt. Vital Signs (ST)

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	36.7	(MAR 07 11:51)	36.8	(MAR 06 14:00)	**H** 37.6	(MAR 06 16:36)
Heart Rate Monitored	92	(MAR 07 11:51)	76	(MAR 06 14:00)	92	(MAR 07 11:51)
Resp Rate	17	(MAR 07 11:51)	16	(MAR 06 14:00)	17	(MAR 06 18:00)
SBP	**H** 159	(MAR 07 11:51)	125	(MAR 06 16:36)	**H** 162	(MAR 07 07:45)
DBP	79	(MAR 07 11:51)	66	(MAR 07 00:42)	**H** 86	(MAR 06 16:36)
SpO2	97	(MAR 07 11:51)	96	(MAR 06 14:00)	99	(MAR 06 16:36)
	on 1 L/min		on 2 L/min		on 2 L/min	
	Nasal cannula		Nasal cannula		Nasal cannula	

General: Alert and oriented, No acute distress.
Eye: Extraocular movements are intact, Normal conjunctiva.
HENT: Normocephalic, dry oral mucosa.
Neck: Supple, No jugular venous distention.
Respiratory: Respirations are non-labored, Symmetrical chest wall expansion.
Cardiovascular: Normal rate, Regular rhythm.
Gastrointestinal: Soft, Non-distended.
Integumentary: Warm, Intact.
Neurologic: Alert, Oriented.
Psychiatric: Cooperative, Appropriate mood & affect.

Health Status

Allergies:

Progress Note - Physician

Allergies (1) Active	**Reaction**
No Known Allergies	None Documented

Current medications: Inpatient medications (ST)
Medications (14) Active
SCHEDULED: (13)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (0)
PRN: (1)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H

Problem list:
Active Problems (13)
Alcohol abuse
Anxiety
At risk for falls
At risk of pressure sore
Cardiac arrest
Convulsion
Depression
Hypothyroid
Ineffective airway clearance
MRSA
Obesity
Pneumonia
Ventilator dependence

Assessment
Interpretation of Results
Laboratory: Fishbone Labs ST
MAR 07 03:15

L 128 | **L 97** | 16 **/**
_____ 89

4.3 | 25 | 0.92 ****

MAR 07 03:15

Progress Note - Physician

```
        \  10.8    /
L 3.42  _______  205

        /  L 31.5  \.
```

Plan

AKI (acute kidney injury) (N17.9)
- denies hx of ckd, prior cr of 1 on 1/20/23
- due to pre-renal from poor po in setting of diuresis
- improved with ivf per below

Critical hyponatremia - due to hypovolemia in setting of diuresis
- urine studies support hypovolemia
- s/p 1l ns on admission
- na correcting at appropriate rate
- adjusting ivf composition but now discontinued
- tolvaptan 7.5 mg po x 1 as now with early hypervolemia

Hypokalemia (E87.6) - due to poor po and diuretics
- kcl added to ivf, improved
- replace per protocol

CHF EF 30-35%
- diuretics and sglt2i on hold

Dental abscess (K04.7)
Facial cellulitis (L03.211)
Severe sepsis (R65.20)
Weakness (R53.1)

Thank you for consult. Will cont to follow closely.

Electronically Signed By: Piceno, Jessie MD
On: 03.07.2024 13:32 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/7/2024 07:20 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Zaugg DO,Garet J (3/7/2024 08:34 PST)
SIGN INFORMATION:	Zaugg DO,Garet J (3/7/2024 08:34 PST)

Las Vegas Neurology Center Note

Progress Note - Physician

SUBJECTIVE:

Patient seen and examined. No overnight events noted.

Patient in good spirits this morning. Denies facial pain. Denies fevers or chills. Feels as though she has a little more energy. Remains seizure-free, on home AEDs.

Review of systems other than mentioned above was negative upon patient interview.
I reviewed the chart, labs, orders, and all events of the last 48 hours.

OBJECTIVE:

Vital Signs (last 24 hrs)	**Last Charted**	**Minimum**	**Maximum**
Heart Rate	78 (MAR 07 04:35)	76 (MAR 06 14:00)	94 (MAR 06 10:00)
Resp Rate	16 (MAR 07 04:35)	16 (MAR 06 10:00)	17 (MAR 06 18:00)
SBP	132 (MAR 07 04:35)	125 (MAR 06 16:36)	**H** 157 (MAR 06 10:00)
DBP	73 (MAR 07 04:35)	66 (MAR 07 00:42)	**H** 86 (MAR 06 16:36)
SpO2	96 (MAR 07 04:35)	96 (MAR 06 14:00)	99 (MAR 06 16:36)

Medications (14) Active
Scheduled: (13)
buPROPion 150 mg/12 hours SR Tab 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab 500 mg 1 Tabs, Oral, BID
Protocol Protocol, N/A, q72H Interval
Protocol Protocol, N/A, q72H Interval
Protocol Protocol, N/A, q72H Interval
Protocol Protocol, N/A, q72H Interval
Protocol Protocol, N/A, q72H Interval
zonisamide 100 mg Cap 200 mg 2 Caps, Oral, Daily
Continuous: (0)
PRN: (1)
acetaminophen 500 mg Tab 500 mg 1 Tabs, Oral, q6H

Labs (Last four charted values)

WBC	L **3.42** (MAR 07)	L **3.60** (MAR 06)	4.60 (MAR 05)	H **10.94** (MAR 03)
Hgb	10.8 (MAR 07)	10.7 (MAR 06)	11.9 (MAR 05)	12.6 (MAR 03)
Hct	L **31.5** (MAR 07)	L **31.0** (MAR 06)	33.8 (MAR 05)	34.0 (MAR 03)
Plt	205 (MAR 07)	186 (MAR 06)	176 (MAR 05)	202 (MAR 03)
Na	L **128** (MAR 07)	C **125** (MAR 06)	L **126** (MAR 06)	L **126** (MAR 06)
K	4.3 (MAR 07)	4.8 (MAR 06)	4.1 (MAR 06)	L **3.4** (MAR 06)
CO2	25 (MAR 07)	26 (MAR 06)	28 (MAR 06)	27 (MAR 06)
Cl	L **97** (MAR 07)	L **97** (MAR 06)	L **94** (MAR 06)	L **91** (MAR 06)
Cr	0.92 (MAR 07)	0.93 (MAR 06)	0.92 (MAR 06)	1.00 (MAR 06)
BUN	16 (MAR 07)	16 (MAR 06)	18 (MAR 06)	H **20** (MAR 06)
Glucose Random	89 (MAR 07)	93 (MAR 06)	H **109** (MAR 06)	95 (MAR 06)
Mg	2.0 (MAR 07)	2.1 (MAR 06)		
Phos	3.0 (MAR 05)			
Ca	9.2 (MAR 07)	9.2 (MAR 06)	9.0 (MAR 06)	9.0 (MAR 06)
PT	10.9 (MAR 03)			
INR	1.0 (MAR 03)			

Progress Note - Physician

PHYSICAL EXAM:

GEN: Tired, oriented X 3/ No Acute Distress

HEENT: Atraumatic/Normocephalic

Neck: supple, no JVD

NEURO:

Mood/Affect: Appropriate to situation

Attention/Concentration: Intact

Fund of Knowledge: Fair

Speech: Fluent

CN 2-12: Left nasolabial fold blunting due to facial cellulitis

EOMI/PERRLA

Motor Strength: 4/5 R arm, 4-/5 R leg, 4+/5 L arm, 4/5 L leg

Tone/Bulk: Intact

Sensation: Intact to light touch

Reflexes: 2/4 throughout

Finger-nose-finger: No dysmetria

Ambulation: deferred

Babinski: negative bilaterally

Imaging:

ASSESSMENT:
Generalized asthenia in setting of the below, improving
Hyponatremia
AKI
UTI
Facial cellulitis
Epilepsy NOS
History of TBI with resultant mild right hemiparesis
CHF
Hypertension
Hypothyroidism
Mood disorder NOS

Progress Note - Physician

PLAN:
CT brain reassuring
No indication for further neurologic workup, etiology of generalized weakness is known, no new focal deficits
Continue home Keppra, Zonegran
PT/OT when clinically appropriate
BP control
Follow nephrology recs
Sepsis management per IM

Garet Zaugg, D.O.
Board-Certified Neurologist
Las Vegas Neurology Center

Electronically Signed By: Zaugg, Garet DO
On: 03.07.2024 08:34 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/6/2024 18:47 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Piceno MD,Jessie M (3/6/2024 18:49 PST)
SIGN INFORMATION:	Piceno MD,Jessie M (3/6/2024 18:49 PST)

NNC Nephro PN - Wolfbrandt

Patient: **WOLFBRANDT, DONNA J** **MRN: CHH7301725** **FIN: CHH0008011012203**
Age: **66 years** Sex: **Female** DOB: **3/25/1957**
Associated Diagnoses: **None**
Author: **Piceno MD, Jessie M**

Subjective
CC: Hyponatremia, AKI

Pt reports feeling better, diet being advanced
RN reports na levels

Review of Systems
Constitutional: Weakness.
Respiratory: No shortness of breath.
Cardiovascular: No chest pain.

Objective
VS/Measurements
Inpt. Vital Signs (ST)

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	**H** 37.6	(MAR 06 16:36)			**L** 36.3	(MAR 05 20:05) **H** 37.6 (MAR 06 16:36)
Heart Rate Monitored	80	(MAR 06 16:36)	61	(MAR 06 04:31)	92	(MAR 06 10:00)
Resp Rate	16	(MAR 06 16:36)	16	(MAR 05 20:05)	16	(MAR 05 20:05)
SBP	125	(MAR 06 16:36)	123	(MAR 06 00:03)	**H** 157	(MAR 06 10:00)
DBP	**H** 86	(MAR 06 16:36)	**L** 58	(MAR 06 00:03)	**H** 86	(MAR 06 16:36)

Progress Note - Physician

SpO2	99 (MAR 06 16:36) on 2 L/min Nasal cannula	**L** 93 (MAR 06 04:31) on 2 L/min Nasal cannula	99 (MAR 06 00:03) on 2 L/min Nasal cannula

General: Alert and oriented, No acute distress.
Eye: Extraocular movements are intact, Normal conjunctiva.
HENT: Normocephalic, dry oral mucosa.
Neck: Supple, No jugular venous distention.
Respiratory: Respirations are non-labored, Symmetrical chest wall expansion.
Cardiovascular: Normal rate, Regular rhythm.
Gastrointestinal: Soft, Non-distended.
Integumentary: Warm, Intact.
Neurologic: Alert, Oriented.
Psychiatric: Cooperative, Appropriate mood & affect.

Health Status

Allergies:

Allergies (1) Active	**Reaction**
No Known Allergies	None Documented

Current medications: Inpatient medications (ST)
Medications (15) Active
SCHEDULED: (13)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq (NS w/K20 20 mEq) 20 mEq 1,000 mL, IV, 100 mL/hr
PRN: (1)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H

Problem list:
Active Problems (13)
Alcohol abuse
Anxiety
At risk for falls
At risk of pressure sore
Cardiac arrest
Convulsion
Depression

Progress Note - Physician

Hypothyroid
Ineffective airway clearance
MRSA
Obesity
Pneumonia
Ventilator dependence

Assessment

Interpretation of Results

Laboratory: Fishbone Labs ST
MAR 06 16:26

L 126 | **L 94** | 18 **/**
_____ **H 109**

4.1 | 28 | 0.92 ****

MAR 06 04:24

**** 10.7 **/**
L 3.60 _____ 186

/ **L 31.0** ****.

Plan

AKI (acute kidney injury) (N17.9)
- denies hx of ckd, prior cr of 1 on 1/20/23
- due to pre-renal from poor po in setting of diuresis
- improving with ivf per below

Critical hyponatremia - due to hypovolemia in setting of diuresis
- urine studies support hypovolemia
- s/p 1l ns on admission
- na correcting at appropriate rate
- adjusting ivf composition and rate per na level, will d/c if na improved at 2100
- bmp at 2100 then in am
- cont fluid restriction 1500 ml/day

Hypokalemia (E87.6) - due to poor po and diuretics
- kcl added to ivf
- replace per protocol

CHF EF 30-35%
- diuretics and sglt2i on hold

Progress Note - Physician

Dental abscess (K04.7)
Facial cellulitis (L03.211)
Severe sepsis (R65.20)
Weakness (R53.1)

Thank you for consult. Will cont to follow closely.

Electronically Signed By: Piceno, Jessie MD
On: 03.06.2024 18:49 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/6/2024 17:45 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Smuckler DO,Jason M (3/6/2024 17:45 PST)
SIGN INFORMATION:	Smuckler DO,Jason M (3/6/2024 17:45 PST)

Subjective

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History
Active Problems(10)
Alcohol abuse
Anxiety
Cardiac arrest
Convulsion
Depression
Hypothyroid
MRSA
Obesity
Pneumonia
Ventilator dependence

Past Surgical History
TRACH

Social History
No social history documented.

Family History
No Family History documented.

Progress Note - Physician

Objective

Measurements (most recent)

Height (cm)	175.26 cm	Weidner RN, Robyn	03/03 23:13
Height (inches)	69.00 inch	SYSTEM	03/03 23:13
Weight	99.79 kg	Weidner RN, Robyn	03/03 23:13
Weight (lbs)	220.00 lb	SYSTEM	03/03 23:13
Body Mass Index	32.49 kg/m2	Weidner RN, Robyn	03/03 23:13

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	**H** 37.6	03/06/2024 16:36	**L** 36.3	03/05/2024 20:05	**H** 37.6	03/06/2024 16:36
Heart Rate Monitored	80	03/06/2024 16:36	61	03/06/2024 04:31	92	03/06/2024 10:00
Resp Rate	16	03/06/2024 16:36	16	03/05/2024 20:05	16	03/05/2024 20:05
SBP	125	03/06/2024 16:36	123	03/06/2024 00:03	**H** 157	03/06/2024 10:00
DBP	**H** 86	03/06/2024 16:36	**L** 58	03/06/2024 00:03	**H** 86	03/06/2024 16:36
MAP	101	03/06/2024 16:36	84	03/06/2024 00:03	101	03/06/2024 16:36
SpO2	99	03/06/2024 16:36	**L** 93	03/06/2024 04:31	99	03/06/2024 00:03
O2 Flow Rate	on 2 l/min		on 2 l/min		on 2 l/min	
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Intake and Output (current encounter)

	03/04 02:25 03/06 17:45	03/06 07:00 03/06 17:45	03/05 07:00 03/06 06:59	03/04 07:00 03/05 06:59	03/03 07:00 03/04 06:59
Intake	2996.8	360	1586.8	50	1000
Output	2825	375	1325	1125	0
Net Total	171.8	-15	261.8	-1075	1000
Stool Count	0	0	0	0	0

Physical Exam

Home Medications

buPROPion (BuPROPion (Eqv-Wellbutrin SR) 150 mg/12 hours oral tablet, extended release) 150 Milligram 1 Tabs By Mouth 2 Times a Day

Progress Note - Physician

dapagliflozin (dapagliflozin 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
escitalopram (escitalopram 10 mg oral tablet) 20 Milligram 2 Tabs By Mouth Daily
famotidine (famotidine 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
furosemide (furosemide 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
levETIRAcetam (levETIRAcetam 500 mg oral tablet) 500 Milligram 1 Tabs By Mouth 2 Times a Day
losartan (losartan 50 mg oral tablet) 50 Milligram 1 Tabs By Mouth Daily
metoprolol (Metoprolol Succinate ER 25 mg oral tablet, extended release) 25 Milligram 1 Tabs By Mouth Daily
zonisamide (Zonegran 100 mg oral capsule) 200 Milligram 2 Capsules By Mouth Daily

Inpatient Medications
Medications (16) Active
SCHEDULED: (14)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
potassium chloride 20 mEq Oral Packet (potassium chloride) 30 mEq, Oral, q3H
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq (NS w/K20 20 mEq) 20 mEq 1,000 mL, IV, 100 mL/hr
PRN: (1)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H

Results
Recent Labs
General Chemistry

Glucose Level	109 mg/dL (High)	03/06/2024 16:26
POC Glucose	82 mg/dL (Normal)	03/06/2024 16:28
Sodium	126 mmol/L (Low)	03/06/2024 16:26
Potassium	4.1 mmol/L (Normal)	03/06/2024 16:26
Chloride	94 mmol/L (Low)	03/06/2024 16:26
CO2	28 mmol/L (Normal)	03/06/2024 16:26
Anion Gap	4 mmol/L (Low)	03/06/2024 16:26
BUN	18 mg/dL (Normal)	03/06/2024 16:26
Creatinine	0.92 mg/dL (Normal)	03/06/2024 16:26
BUN/Creat Ratio	20 (Normal)	03/06/2024 16:26

Progress Note - Physician

Calcium	9.0 mg/dL (Normal)	03/06/2024 16:26
Albumin. Level	3.6 gm/dL (Normal)	03/06/2024 04:24
TP	6.3 gm/dL (Low)	03/06/2024 04:24
A/G Ratio	1.3 (Normal)	03/06/2024 04:24
T Bili	0.9 mg/dL (Normal)	03/06/2024 04:24
Alk Phos	86 units/L (Normal)	03/06/2024 04:24
AST	20 units/L (Normal)	03/06/2024 04:24
ALT	20 units/L (Normal)	03/06/2024 04:24
Mg Lvl	2.1 mg/dL (Normal)	03/06/2024 04:24
Phosphorus	3.0 mg/dL (Normal)	03/05/2024 08:04
Estimated Creatinine Clearance	62.86 mL/min ()	03/06/2024 17:05
eGFR Cr	69 mL/min/1.73m2 (N/A)	03/06/2024 16:26
eGFR Pediatric	Not Reported (N/A)	03/06/2024 16:26
MAR Glucose Result	82 ()	03/06/2024 16:33

General Hematology

WBC	3.60 x10e3/mcL (Low)	03/06/2024 04:24
RBC	2.53 x10e6/mcL (Low)	03/06/2024 04:24
Hgb	10.7 gm/dL (Normal)	03/06/2024 04:24
Hct	31.0 % (Low)	03/06/2024 04:24
MCV	122.5 Femtoliters (High)	03/06/2024 04:24
MCH	42.3 pg (High)	03/06/2024 04:24
MCHC	34.5 gm/dL (Normal)	03/06/2024 04:24
RDW-SD	62.6 Femtoliters (High)	03/06/2024 04:24
RDW-CV	13.8 % (Normal)	03/06/2024 04:24
Plt	186 x10e3/mcL (Normal)	03/06/2024 04:24
MPV	9.4 Femtoliters (Normal)	03/06/2024 04:24
Neut % Auto	70.0 % (N/A)	03/05/2024 08:04
Lymph % Auto	17.2 % (N/A)	03/05/2024 08:04
Mono % Auto	11.3 % (N/A)	03/05/2024 08:04
Eos % Auto	0.4 % (N/A)	03/05/2024 08:04
Baso % Auto	0.7 % (N/A)	03/05/2024 08:04
Immature Grans %	0.4 % (Normal)	03/05/2024 08:04
Neut # Auto	3.22 x10e3/mcL (Normal)	03/05/2024 08:04
Lymph # Auto	0.79 x10e3/mcL (Low)	03/05/2024 08:04

Progress Note - Physician

Mono # Auto	0.52 x10e3/mcL (Normal)	03/05/2024 08:04
Eos # Auto	0.02 x10e3/mcL (Normal)	03/05/2024 08:04
Baso # Auto	0.03 x10e3/mcL (Normal)	03/05/2024 08:04
Immature Grans # Auto	0.02 x10e3/mcL (Normal)	03/05/2024 08:04

Other Chemistry

Procalcitonin Lvl	0.57 ng/mL (High)	03/05/2024 08:04

Abnormal Labs

General Chemistry

Anion Gap 4 mmol/L (Low) 03/06/2024 16:26
Chloride 94 mmol/L (Low) 03/06/2024 16:26
Glucose Level 109 mg/dL (High) 03/06/2024 16:26
Sodium 126 mmol/L (Low) 03/06/2024 16:26
TP 6.3 gm/dL (Low) 03/06/2024 04:24

General Hematology

Hct 31.0 % (Low) 03/06/2024 04:24
MCH 42.3 pg (High) 03/06/2024 04:24
MCV 122.5 Femtoliters (High) 03/06/2024 04:24
RBC 2.53 x10e6/mcL (Low) 03/06/2024 04:24
RDW-SD 62.6 Femtoliters (High) 03/06/2024 04:24
WBC 3.60 x10e3/mcL (Low) 03/06/2024 04:24

Note: Critical results are displayed in red.

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**3.60 L** 03/06/2024 04:24	**4.60** 03/05/2024 08:04	**10.94 H** 03/03/2024 23:49	
Hgb	**10.7** 03/06/2024 04:24	**11.9** 03/05/2024 08:04	**12.6** 03/03/2024 23:49	
Hct	**31.0 L** 03/06/2024 04:24	**33.8** 03/05/2024 08:04	**34.0** 03/03/2024 23:49	
Baso # Auto	**0.03** 03/05/2024 08:04	**0.02** 03/03/2024 23:49		
Baso % Auto	**0.7** 03/05/2024 08:04	**0.2** 03/03/2024 23:49		
Eos # Auto	**0.02** 03/05/2024 08:04	**0.02** 03/03/2024 23:49		
Eos % Auto	**0.4** 03/05/2024 08:04	**0.2** 03/03/2024 23:49		

Progress Note - Physician

Immature Grans %	**0.4** 03/05/2024 08:04	**0.6** 03/03/2024 23:49		
Immature Grans # Auto	**0.02** 03/05/2024 08:04	**0.07** 03/03/2024 23:49		
Lymph # Auto	**0.79 L** 03/05/2024 08:04	**0.79 L** 03/03/2024 23:49		
Lymph % Auto	**17.2** 03/05/2024 08:04	**7.2** 03/03/2024 23:49		
Mono # Auto	**0.52** 03/05/2024 08:04	**0.91 H** 03/03/2024 23:49		
Mono % Auto	**11.3** 03/05/2024 08:04	**8.3** 03/03/2024 23:49		
Neut # Auto	**3.22** 03/05/2024 08:04	**9.13 H** 03/03/2024 23:49		
Neut % Auto	**70.0** 03/05/2024 08:04	**83.5** 03/03/2024 23:49		
Plt	**186** 03/06/2024 04:24	**176** 03/05/2024 08:04	**202** 03/03/2024 23:49	
Glucose Level	**109 H** 03/06/2024 16:26	**95** 03/06/2024 04:24	**97** 03/05/2024 21:11	**93** 03/05/2024 08:04
Sodium	**126 L** 03/06/2024 16:26	**126 L** 03/06/2024 04:24	**125** 03/05/2024 21:11	**125** 03/05/2024 12:13
Potassium	**4.1** 03/06/2024 16:26	**3.4 L** 03/06/2024 04:24	**3.3 L** 03/05/2024 21:11	**3.5** 03/05/2024 08:04
Chloride	**94 L** 03/06/2024 16:26	**91 L** 03/06/2024 04:24	**92 L** 03/05/2024 21:11	**86 L** 03/05/2024 08:04
CO2	**28** 03/06/2024 16:26	**27** 03/06/2024 04:24	**27** 03/05/2024 21:11	**28** 03/05/2024 08:04
Creatinine	**0.92** 03/06/2024 16:26	**1.00** 03/06/2024 04:24	**1.13 H** 03/05/2024 21:11	**1.41 H** 03/05/2024 08:04
BUN	**18** 03/06/2024 16:26	**20 H** 03/06/2024 04:24	**18** 03/05/2024 21:11	**23 H** 03/05/2024 08:04
BUN/Creat Ratio	**20** 03/06/2024 16:26	**20** 03/06/2024 04:24	**16** 03/05/2024 21:11	**16** 03/05/2024 08:04
Mg Lvl	**2.1** 03/06/2024 04:24			
Phosphorus	**3.0** 03/05/2024 08:04			
Calcium	**9.0** 03/06/2024 16:26	**9.0** 03/06/2024 04:24	**9.0** 03/05/2024 21:11	**9.2** 03/05/2024 08:04
Albumin. Level	**3.6** 03/06/2024 04:24	**3.9** 03/05/2024 08:04		
Ur Protein	**Negative** 03/04/2024 00:47			
T Bili	**0.9** 03/06/2024 04:24			

Progress Note - Physician

Alk Phos	**86** 03/06/2024 04:24			
AST	**20** 03/06/2024 04:24			
ALT	**20** 03/06/2024 04:24			
PT	**10.9** 03/03/2024 23:49			
INR	**1.0** 03/03/2024 23:49			

Blood Glucose Trend

Glucose Level: 109 mg/dL High (03/06/24 16:26:00)
Glucose Level: 95 mg/dL (03/06/24 04:24:00)
Glucose Level: 97 mg/dL (03/05/24 21:11:00)
POC Glucose: 82 mg/dL (03/06/24 16:28:00)
POC Glucose: 89 mg/dL (03/06/24 12:23:00)
POC Glucose: 84 mg/dL (03/06/24 06:27:00)
POC Glucose: 79 mg/dL (03/05/24 23:47:00)

CBC (Last Within 24hrs)
WBC: 3.6 x10e3/mcL Low (04:24)
Hgb: 10.7 gm/dL (04:24)
Hct: 31 % Low (04:24)
Plt: 186 x10e3/mcL (04:24)

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 126 mmol/L Low (16:26)
Potassium: 4.1 mmol/L (16:26)
Chloride: 94 mmol/L Low (16:26)
CO2: 28 mmol/L (16:26)
BUN: 18 mg/dL (16:26)
Creatinine: 0.92 mg/dL (16:26)
Glucose Level: 109 mg/dL High (16:26)
Calcium: 9 mg/dL (16:26)
Mg Lvl: 2.1 mg/dL (04:24)

LFT (Last Within 24hrs)
AST: 20 units/L (04:24)
ALT: 20 units/L (04:24)
Alk Phos: 86 units/L (04:24)
T Bili: 0.9 mg/dL (04:24)

Progress Note - Physician

TP: 6.3 gm/dL Low (04:24)

Cardiology Labs
Troponin TNIH: 4.33 ng/L (03/04/24 02:25:00)
Troponin TNIH: 4.14 ng/L (03/04/24 00:56:00)
Troponin TNIH: 5.61 ng/L (03/03/24 23:49:00)

Assessment/Plan
Diagnoses
AKI (acute kidney injury) (N17.9)
Dental abscess (K04.7)
Facial cellulitis (L03.211)
Hypochloremia (E87.8)
Hypokalemia (E87.6)
Hyponatremia (E87.1)
Severe sepsis (R65.20)
Weakness (R53.1)
1. Hyponatremia (E87.1)

Assessment: NAEO
Complains of weakness and trouble with ambulation
Says she has a home PCA and home health set up
On 4LNC
Sodium is 126

Physical Exam:
General: Alert, in no acute cardiopulmonary distress. Obese
Mental Status: Oriented to person, place and time. Normal affect.
Head: Normocephalic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact.
Ear, Nose and Throat: Oropharynx clear, mucous membranes moist. Trachea midline.
Neck: Supple, Full range of motion.
Respiratory: Clear to auscultation. No wheezing, rales or rhonchi.
Cardiovascular: Heart sounds normal. No thrills. Regular rate and rhythm, no murmurs, rubs or gallops.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Normal bowel sounds.
Genitourinary: No costovertebral angle tenderness.
Neurologic: right sided weakness at baseline,
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.

Assessment:
Severe Hyponatremia, improving
Hypokalemia
Klebsiella UTI
Facial cellulitis

Progress Note - Physician

Generalized weakness
Fatigued
Dehydration
AKI
History of TBI
UTI
Severe sepsis, present on admission, source UTI versus facial cellulitis
history of cardiac arrest
Hypertension
Obesity

Plan:
IMC
Remove Foley
Does not use oxygen at home, oxygen challenge once more stable
Follow urine Cx.
Follow up repeat blood culture
Diet advanced from clears to minced and moist. Consistency to be adjusted pending swallow eval.
Nephrology following, appreciate recommendations.
Neurology following appreciate recs.
Continue home Keppra, Zonegran
IVF per nephrology.
Monitor I/O closely
Urine studies
Serial Sodium checks
Monitor neuro status
PT/OT
Full code
Activity: Up ad lib.
DVT prophylaxis: SCDs
Case management consulted to confirm home health

Discharge Planning: HHC vs SNF pending clinical course, stable renal, improved sodium, Cx results.

Jason Smuckler DO
Internal Medicine
Platinum Hospitalists
Please page YouCallMD with any questions or concerns

Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Skilled Nursing Unit/Facility

Electronically Signed By: Smuckler, Jason DO
On: 03.06.2024 17:45 PST

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Progress Note - Physician

DOCUMENT NAME: Progress Note-Physician
SERVICE DATE/TIME: 3/6/2024 07:05 PST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Zaugg DO,Garet J (3/6/2024 11:26 PST)
SIGN INFORMATION: Zaugg DO,Garet J (3/6/2024 11:26 PST)

Las Vegas Neurology Center Note

SUBJECTIVE:

Patient seen and examined. No overnight events noted.

Patient concerned she did not receive her zonisamide for some reason yesterday. Discussed with patient and zonisamide given this morning. She endorses improvement of facial pain. Feels slightly less fatigued. Renal function improving.

Review of systems other than mentioned above was negative upon patient interview.
I reviewed the chart, labs, orders, and all events of the last 48 hours.

OBJECTIVE:

Vital Signs (last 24 hrs)	Last Charted	Minimum	Maximum
Heart Rate	84 (MAR 06 07:23)	61 (MAR 06 04:31)	84 (MAR 06 07:23)
Resp Rate	16 (MAR 06 07:23)	16 (MAR 05 12:35)	18 (MAR 05 11:50)
SBP	130 (MAR 06 07:23)	111 (MAR 05 13:20)	**H** 136 (MAR 05 11:50)
DBP	76 (MAR 06 07:23)	**L** 58 (MAR 05 13:20)	76 (MAR 06 07:23)
SpO2	96 (MAR 06 07:23)	**L** 92 (MAR 05 12:35)	100 (MAR 05 11:50)

Medications (10) Active
Scheduled: (8)
buPROPion 150 mg/12 hours SR Tab 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab 500 mg 1 Tabs, Oral, BID
zonisamide 100 mg Cap 200 mg 2 Caps, Oral, Daily
Continuous: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq 20 mEq 1,000 mL, IV, 100 mL/hr
PRN: (1)
acetaminophen 500 mg Tab 500 mg 1 Tabs, Oral, q6H

Labs (Last four charted values)

WBC	L **3.60** (MAR 06)	4.60 (MAR 05)	H **10.94** (MAR 03)	
Hgb	10.7 (MAR 06)	11.9 (MAR 05)	12.6 (MAR 03)	
Hct	L **31.0** (MAR 06)	33.8 (MAR 05)	34.0 (MAR 03)	
Plt	186 (MAR 06)	176 (MAR 05)	202 (MAR 03)	
Na	L **126** (MAR 06)	C **125** (MAR 05)	C **125** (MAR 05)	C **123** (MAR 05)
K	L **3.4** (MAR 06)	L **3.3** (MAR 05)	3.5 (MAR 05)	C **2.5** (MAR 04)
CO2	27 (MAR 06)	27 (MAR 05)	28 (MAR 05)	28 (MAR 04)
Cl	L **91** (MAR 06)	L **92** (MAR 05)	L **86** (MAR 05)	L **89** (MAR 04)
Cr	1.00 (MAR 06)	H **1.13** (MAR 05)	H **1.41** (MAR 05)	H **1.60** (MAR 04)
BUN	H **20** (MAR 06)	18 (MAR 05)	H **23** (MAR 05)	H **22** (MAR 04)

Progress Note - Physician

Glucose Random 95 (MAR 06) 97 (MAR 05) 93 (MAR 05) 87 (MAR 04)
Mg 2.1 (MAR 06)
Phos 3.0 (MAR 05)
Ca 9.0 (MAR 06) 9.0 (MAR 05) 9.2 (MAR 05) L **7.4** (MAR 04)
PT 10.9 (MAR 03)
INR 1.0 (MAR 03)

PHYSICAL EXAM:

GEN: Tired, oriented X 3/ No Acute Distress

HEENT: Atraumatic/Normocephalic

Neck: supple, no JVD

NEURO:

Mood/Affect: Appropriate to situation

Attention/Concentration: Intact

Fund of Knowledge: Fair

Speech: Fluent

CN 2-12: Left nasolabial fold blunting due to facial cellulitis

EOMI/PERRLA

Motor Strength: 4/5 R arm, 4-/5 R leg, 4+/5 L arm, 4/5 L leg

Tone/Bulk: Intact

Sensation: Intact to light touch

Reflexes: 2/4 throughout

Finger-nose-finger: No dysmetria

Ambulation: deferred

Babinski: negative bilaterally

Imaging:

ASSESSMENT:
Generalized asthenia in setting of the below
Hyponatremia
AKI
UTI
Facial cellulitis
Epilepsy NOS

Progress Note - Physician

History of TBI with resultant mild right hemiparesis
CHF
Hypertension
Hypothyroidism
Mood disorder NOS

PLAN:
CT brain reassuring
No indication for further neurologic workup, etiology of generalized weakness is known, no new focal deficits
Follow nephrology recs
Continue home Keppra, Zonegran
PT/OT when clinically appropriate
BP control
Sepsis management per IM

Garet Zaugg, D.O.
Board-Certified Neurologist
Las Vegas Neurology Center

Electronically Signed By: Zaugg, Garet DO
On: 03.06.2024 11:26 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/5/2024 22:26 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Tiwari,Subhikshya MD (3/5/2024 22:26 PST)
SIGN INFORMATION:	Tiwari,Subhikshya MD (3/5/2024 22:26 PST)

Subjective

Chief Complaint/Reason for Consultation
Chief Complaint: Weakness the past few days, had a GLF today. No injury, no LOC, denies hitting her head, no blood thinners. LVFR noted that pt was also hypotensive on scene. Dex 115

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History
Active Problems(10)
Alcohol abuse
Anxiety
Cardiac arrest
Convulsion
Depression
Hypothyroid
MRSA
Obesity
Pneumonia
Ventilator dependence

Progress Note - Physician

Past Surgical History
TRACH

Social History
No social history documented.

Family History
No Family History documented.

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	**L** 36.3	03/05/2024 20:05	**L** 36.4	03/05/2024 03:00	**L** 36.4	03/05/2024 03:00
Heart Rate Monitored	74	03/05/2024 20:05	63	03/05/2024 02:00	79	03/05/2024 10:34
Resp Rate	16	03/05/2024 20:05	16	03/05/2024 04:00	18	03/05/2024 07:52
SBP	134	03/05/2024 20:05	103	03/05/2024 10:34	**H** 136	03/05/2024 11:50
DBP	68	03/05/2024 20:05	**L** 52	03/05/2024 02:00	77	03/05/2024 11:23
MAP	96	03/05/2024 20:05	71	03/05/2024 02:00	97	03/05/2024 11:23
SpO2	98	03/05/2024 20:05	**L** 92	03/05/2024 12:35	100	03/05/2024 03:00
O2 Flow Rate	on 2 l/min		on 2 l/min		on 2 l/min	
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Pain Scores (Last Within 24hrs)
Numeric Pain Scale: 0 = No pain (20:00)

Intake and Output (current encounter)

	03/04 02:25 03/05 22:26	03/05 07:00 03/05 22:26	03/04 07:00 03/05 06:59	03/03 07:00 03/04 06:59	03/02 07:00 03/03 06:59
Intake	1301.5	251.5	50	1000	0
Output	2100	975	1125	0	0
Net Total	-798.5	-723.5	-1075	1000	0

Progress Note - Physician

Precautions
No Precautions documented.

Glasgow Coma Scale
Eye Opening Response Glasgow: Spontaneously (03/05/24 20:00:00)
Best Verbal Response Glasgow: Oriented and converses (03/05/24 20:00:00)
Best Motor Response Glasgow: Obeys commands (03/05/24 20:00:00)
Glasgow Coma Score: 15 (03/05/24 20:00:00)

Stroke Assessments
Glasgow Coma Scale
Eye Opening Response Glasgow: Spontaneously (03/05/24 20:00:00)
Best Verbal Response Glasgow: Oriented and converses (03/05/24 20:00:00)
Best Motor Response Glasgow: Obeys commands (03/05/24 20:00:00)
Glasgow Coma Score: 15 (03/05/24 20:00:00)

Physical Exam

Home Medications
buPROPion (BuPROPion (Eqv-Wellbutrin SR) 150 mg/12 hours oral tablet, extended release) 150 Milligram 1 Tabs By Mouth 2 Times a Day
dapagliflozin (dapagliflozin 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
escitalopram (escitalopram 10 mg oral tablet) 20 Milligram 2 Tabs By Mouth Daily
famotidine (famotidine 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
furosemide (furosemide 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
levETIRAcetam (levETIRAcetam 500 mg oral tablet) 500 Milligram 1 Tabs By Mouth 2 Times a Day
losartan (losartan 50 mg oral tablet) 50 Milligram 1 Tabs By Mouth Daily
metoprolol (Metoprolol Succinate ER 25 mg oral tablet, extended release) 25 Milligram 1 Tabs By Mouth Daily
zonisamide (Zonegran 100 mg oral capsule) 200 Milligram 2 Capsules By Mouth Daily

Inpatient Medications
Medications (10) Active
SCHEDULED: (8)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H

Progress Note - Physician

Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq (NS w/K20 20 mEq) 20 mEq 1,000 mL, IV, 75 mL/hr
PRN: (1)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefepime 2 gm, 20 mL, 12.5 mL/hr, IV Piggyback, q12H Interval	2	03/05/2024 17:24	03/04/2024 02:55

Vaccinations and Immunoprophylaxis
COVID-19 Vaccine Status: Fully +1 or more booster(s) (03/05/24 14:01:00)
COVID-19 Vaccine Status: Fully (> 2 weeks since last dose) (03/04/24 00:22:00)
COVID-19 Vaccine Status: Fully +1 or more booster(s) (01/05/23 16:38:00)

Results
Abnormal Labs

General Chemistry
Chloride 92 mmol/L (Low) 03/05/2024 21:11
Creatinine 1.13 mg/dL (High) 03/05/2024 21:11
Potassium 3.3 mmol/L (Low) 03/05/2024 21:11
Sodium 125 mmol/L (Critical) 03/05/2024 21:11

General Hematology
Lymph # Auto 0.79 x10e3/mcL (Low) 03/05/2024 08:04
MCH 41.6 pg (High) 03/05/2024 08:04
MCHC 35.2 gm/dL (High) 03/05/2024 08:04
MCV 118.2 Femtoliters (High) 03/05/2024 08:04
RBC 2.86 x10e6/mcL (Low) 03/05/2024 08:04
RDW-SD 63.6 Femtoliters (High) 03/05/2024 08:04

Other Chemistry
Procalcitonin Lvl 0.57 ng/mL (High) 03/05/2024 08:04

Note: Critical results are displayed in red.

Progress Note - Physician

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**4.60** 03/05/2024 08:04	**10.94 H** 03/03/2024 23:49		
Hgb	**11.9** 03/05/2024 08:04	**12.6** 03/03/2024 23:49		
Hct	**33.8** 03/05/2024 08:04	**34.0** 03/03/2024 23:49		
Baso # Auto	**0.03** 03/05/2024 08:04	**0.02** 03/03/2024 23:49		
Baso % Auto	**0.7** 03/05/2024 08:04	**0.2** 03/03/2024 23:49		
Eos # Auto	**0.02** 03/05/2024 08:04	**0.02** 03/03/2024 23:49		
Eos % Auto	**0.4** 03/05/2024 08:04	**0.2** 03/03/2024 23:49		
Immature Grans %	**0.4** 03/05/2024 08:04	**0.6** 03/03/2024 23:49		
Immature Grans # Auto	**0.02** 03/05/2024 08:04	**0.07** 03/03/2024 23:49		
Lymph # Auto	**0.79 L** 03/05/2024 08:04	**0.79 L** 03/03/2024 23:49		
Lymph % Auto	**17.2** 03/05/2024 08:04	**7.2** 03/03/2024 23:49		
Mono # Auto	**0.52** 03/05/2024 08:04	**0.91 H** 03/03/2024 23:49		
Mono % Auto	**11.3** 03/05/2024 08:04	**8.3** 03/03/2024 23:49		
Neut # Auto	**3.22** 03/05/2024 08:04	**9.13 H** 03/03/2024 23:49		
Neut % Auto	**70.0** 03/05/2024 08:04	**83.5** 03/03/2024 23:49		
Plt	**176** 03/05/2024 08:04	**202** 03/03/2024 23:49		
Glucose Level	**97** 03/05/2024 21:11	**93** 03/05/2024 08:04	**87** 03/04/2024 12:00	**117 H** 03/03/2024 23:49
Sodium	**125** 03/05/2024 21:11	**125** 03/05/2024 12:13	**123** 03/05/2024 08:43	**122** 03/05/2024 08:04
Potassium	**3.3 L** 03/05/2024 21:11	**3.5** 03/05/2024 08:04	**2.5** 03/04/2024 12:00	**3.2 L** 03/03/2024 23:49
Chloride	**92 L** 03/05/2024 21:11	**86 L** 03/05/2024 08:04	**89 L** 03/04/2024 12:00	**76** 03/03/2024 23:49
CO2	**27** 03/05/2024 21:11	**28** 03/05/2024 08:04	**28** 03/04/2024 12:00	**31** 03/03/2024 23:49
Creatinine	**1.13 H** 03/05/2024 21:11	**1.41 H** 03/05/2024 08:04	**1.60 H** 03/04/2024 12:00	**1.85 H** 03/03/2024 23:49

Progress Note - Physician

BUN	**18** 03/05/2024 21:11	**23 H** 03/05/2024 08:04	**22 H** 03/04/2024 12:00	**22 H** 03/03/2024 23:49
BUN/Creat Ratio	**16** 03/05/2024 21:11	**16** 03/05/2024 08:04	**14** 03/04/2024 12:00	**12** 03/03/2024 23:49
Phosphorus	**3.0** 03/05/2024 08:04			
Calcium	**9.0** 03/05/2024 21:11	**9.2** 03/05/2024 08:04	**7.4 L** 03/04/2024 12:00	**9.3** 03/03/2024 23:49
Albumin. Level	**3.9** 03/05/2024 08:04			
Ur Protein	**Negative** 03/04/2024 00:47			
PT	**10.9** 03/03/2024 23:49			
INR	**1.0** 03/03/2024 23:49			

Blood Glucose Trend

Glucose Level: 97 mg/dL (03/05/24 21:11:00)
Glucose Level: 93 mg/dL (03/05/24 08:04:00)
POC Glucose: 93 mg/dL (03/05/24 17:23:00)
POC Glucose: 100 mg/dL (03/05/24 11:42:00)
POC Glucose: 96 mg/dL (03/05/24 07:50:00)

CBC (Last Within 24hrs)
WBC: 4.6 x10e3/mcL (08:04)
Hgb: 11.9 gm/dL (08:04)
Hct: 33.8 % (08:04)
Plt: 176 x10e3/mcL (08:04)

Differential (Last Within 24hrs)
- Automated -
Neut % Auto: 70 % (08:04)
Lymph % Auto: 17.2 % (08:04)
Mono % Auto: 11.3 % (08:04)
Eos % Auto: 0.4 % (08:04)
Baso % Auto: 0.7 % (08:04)
Immature Grans %: 0.4 % (08:04)
Neut # Auto: 3.22 x10e3/mcL (08:04)
Lymph # Auto: 0.79 x10e3/mcL Low (08:04)
Mono # Auto: 0.52 x10e3/mcL (08:04)
Baso # Auto: 0.03 x10e3/mcL (08:04)

Immature Grans # Auto: 0.02 x10e3/mcL (08:04)

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 125 mmol/L Critical (21:11)
Potassium: 3.3 mmol/L Low (21:11)
Chloride: 92 mmol/L Low (21:11)
CO2: 27 mmol/L (21:11)
BUN: 18 mg/dL (21:11)
Creatinine: 1.13 mg/dL High (21:11)
Glucose Level: 97 mg/dL (21:11)
Calcium: 9 mg/dL (21:11)
Phosphorus: 3 mg/dL (08:04)

Microbiology

Culture Urine **Completed**

Source:	U CleanCatch	Body Site:	
Collected Dt/Tm:	03/04/2024 00:47	Last Updated Dt/Tm:	03/05/2024 22:25

>100,000 cfu/ml Klebsiella pneumoniae

	Klebsiella pneumoniae	
Drug	MIC interp	BMIC Dilut
Amikacin	S	<=16
Amoxicillin/Clavulanate	S	<=4/2
Ampicillin	R	>16
Ampicillin/Sulbactam	S	8/4
Aztreonam	S	<=4
Cefazolin	S	<=2
Cefepime	S	<=2
Ceftriaxone	S	<=1
Ciprofloxacin	S	<=1
Gentamicin	S	<=1
Levofloxacin	S	<=0.25
Nitrofurantoin	S	<=32
Piperacillin/Tazobactam	S	<=4
Tetracycline	S	<=4
Tobramycin	S	<=1

Progress Note - Physician

Trimethoprim/Sulfamethoxazole	S	<=2/38

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/03/2024 23:49	Last Updated Dt/Tm:	03/05/2024 08:01

No growth at 24 hours.

Culture Blood ARD **Preliminary**

Source:	Blood	Body Site:	
Collected Dt/Tm:	03/03/2024 23:49	Last Updated Dt/Tm:	03/05/2024 08:01

No growth at 24 hours.

Cardiology Labs
Troponin TNIH: 4.33 ng/L (03/04/24 02:25:00)
Troponin TNIH: 4.14 ng/L (03/04/24 00:56:00)
Troponin TNIH: 5.61 ng/L (03/03/24 23:49:00)

Assessment/Plan
Diagnoses
AKI (acute kidney injury) (N17.9)
Dental abscess (K04.7)
Facial cellulitis (L03.211)
Hypochloremia (E87.8)
Hypokalemia (E87.6)
Hyponatremia (E87.1)
Severe sepsis (R65.20)
Weakness (R53.1)
1. Hyponatremia (E87.1)

Assessment: late entry
Subjective
States she is feeling much better as compared to yesterday. Feeling less weak. Discussed labs and Rx plan.

10 pt ROS neg except as mentioned above.

Physical Exam
General: Alert, in no acute cardiopulmonary distress. Obese

Progress Note - Physician

Mental Status: Oriented to person, place and time. Normal affect.
Head: Normocephalic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact.
Ear, Nose and Throat: Oropharynx clear, mucous membranes moist. Trachea midline.
Neck: Supple, Full range of motion.
Respiratory: Clear to auscultation. No wheezing, rales or rhonchi.
Cardiovascular: Heart sounds normal. No thrills. Regular rate and rhythm, no murmurs, rubs or gallops.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Normal bowel sounds.
Genitourinary: No costovertebral angle tenderness.
Neurologic: right sided weakness at baseline,
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.

Assessment
Severe Hyponatremia, improving
Hypokalemia
Klebsiella UTI
Facial cellulitis
Generalized weakness
Fatigued
Dehydration
AKI
History of TBI
UTI
Severe sepsis, present on admission, source UTI versus facial cellulitis
history of cardiac arrest
Hypertension
Obesity

PLAN:
Downgraded from ICU to IMC
Does not use oxygen at home, oxygen challenge once more stable
Follow urine Cx.
Diet advanced from clears to minced and moist. Consistency to be adjusted pending swallow eval.
Nephrology following, appreciate recommendations.
Neurology following appreciate recs.
IVF per nephrology.
Monitor I/O closely
Urine studies
Serial Sodium checks
Monitor neuro status
PT/OT

Full code

Activity: Up ad lib.
DVT prophylaxis: SCDs

Discharge Planning: HHC vs SNF pending clinical course, stable renal, improved sodium, Cx results.

Portions of this record are transcribed using voice recognition software and may contain

Progress Note - Physician

inadvertent errors with dictation.

Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Skilled Nursing Unit/Facility

Order Date/Time	Order Action	Order Name	Order Detail
03/05/2024 21:56	Modify	Admission/Observation/Transfer	03/04/24 2:25:00 PST, Estimated LOS 2 to 3 Nights, Admit as Inpatient, Suggested Placement: Intermediate Care/Step down, Level of Care Acute Intermediate Care, Admitting: Timple DO, Julie M Attending: Smuckler DO, Jason M, Hyponatremia
03/05/2024 21:05	Order	POC Glu	Blood, Collected, RT collect, 03/05/24 17:23:00 PST
03/05/2024 15:55	Order	Mg (Magnesium Level)	Blood, Lab Collect, AM Draw collect, 03/06/24 4:00:00 PST, Daily, for 7 Days, Stop date 03/13/24 3:59:00 PDT, Print Label By Order Location
03/05/2024 15:55	Order	CMP	Blood, Lab Collect, AM Draw collect, 03/06/24 4:00:00 PST, Daily, for 7 Times, Stop date 03/13/24 3:59:00 PDT, Print Label By Order Location
03/05/2024 15:55	Order	CBC NoDiff (CBC no Diff)	Blood, Lab Collect, AM Draw collect, 03/06/24 4:00:00 PST, Daily, for 7 Times, Stop date 03/13/24 3:59:00 PDT, Print Label By Order Location
03/05/2024 15:54	Order	Nutritional Supplements	03/05/24 15:54:00 PST, Ensure Clear Berry, Breakfast/Lunch/Dinner
03/05/2024 15:53	Modify	Diet	03/04/24 9:30:00 PST, Cardiac, Consistency: 5 - Minced and Moist (Dysphagia), Fluid Restriction: 1500 mL, Consistency 0 - Thin, Dietitian to Manage: All Diet Parameters
03/05/2024 14:54	Order	ST Bedside Swallow Exam	03/05/24 14:54:00 PST, Bedside Swallow Exam, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, IV Yes, O2 Yes, 16443966655.0
03/05/2024 14:54	Order	Speech Therapy Eval and Treat (ST Eval and Treat)	03/05/24 14:54:00 PST, Bedside Swallow Exam, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, IV Yes, O2 Yes
03/05/2024 14:54	Order	Occupational Therapy Eval and Treat (OT Evaluation and Treatment)	03/05/24 14:54:00 PST, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, Yes, Yes
03/05/2024 14:54	Order	Physical Therapy Eval and Treat (PT Evaluation and Treatment)	03/05/24 14:54:00 PST, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, Yes, Yes
03/05/2024 12:21	Order	acetaminophen 500 mg Tab (Tylenol)	500 mg, 1 Tabs, Oral, q6H, PRN: Fever/Pain (1-3)
03/05/2024 12:20	Modify	Admission/Observation/Transfer	03/04/24 2:25:00 PST, Estimated LOS 2 to 3 Nights, Admit as Inpatient, Suggested Placement: Intermediate Care/Step down, Level of Care Acute Intermediate Care, Admitting: Timple DO, Julie M Attending: Tiwari, Subhikshya MD, Hyponatremia

Progress Note - Physician

Electronically Signed By: Tiwari, Subhikshya
On: 03.05.2024 22:26 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/5/2024 12:24 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Piceno MD,Jessie M (3/5/2024 12:31 PST)
SIGN INFORMATION:	Piceno MD,Jessie M (3/5/2024 12:31 PST)

NNC Nephro PN - Wolfbrandt

Patient: **WOLFBRANDT, DONNA J** **MRN: CHH7301725** **FIN: CHH0008011012203**
Age: **66 years** Sex: **Female** DOB: **3/25/1957**
Associated Diagnoses: **None**
Author: **Piceno MD, Jessie M**

Subjective

CC: Hyponatremia, AKI

Pt reports feeling better, tolerating cld, requesting advance diet
RN reports na levels

Review of Systems

Constitutional: Weakness.
Respiratory: No shortness of breath.
Cardiovascular: No chest pain.

Objective

VS/Measurements

Inpt. Vital Signs (ST)

Vital Signs (24 hrs)	Last Charted	Minimum	Maximum
Temp	36.7 (MAR 05 11:50)	**L** 36.4 (MAR 05 03:00)	37.0 (MAR 04 20:03)
Peripheral Pulse Rate	69 (MAR 05 11:50)	**L** 59 (MAR 04 17:00)	78 (MAR 05 09:09)
Resp Rate	18 (MAR 05 11:50)	16 (MAR 04 13:01)	18 (MAR 05 07:52)
SBP	**H** 136 (MAR 05 11:50)	90 (MAR 04 18:00)	**H** 136 (MAR 05 11:50)
DBP	66 (MAR 05 11:50)	**L** 50 (MAR 04 17:00)	77 (MAR 05 11:23)
SpO2	100 (MAR 05 11:50)	**L** 90 (MAR 04 16:00)	100 (MAR 04 13:01)
	on 2 L/min	on 2 L/min	on 2 L/min
	Nasal cannula	Nasal cannula	Nasal cannula

General: Alert and oriented, No acute distress.
Eye: Extraocular movements are intact, Normal conjunctiva.
HENT: Normocephalic, dry oral mucosa.
Neck: Supple, No jugular venous distention.
Respiratory: Respirations are non-labored, Symmetrical chest wall expansion.
Cardiovascular: Normal rate, Regular rhythm.
Gastrointestinal: Soft, Non-distended.
Integumentary: Warm, Intact.
Neurologic: Alert, Oriented.

Progress Note - Physician

Psychiatric: Cooperative, Appropriate mood & affect.

Health Status

Allergies:

Allergies (1) Active	**Reaction**
No Known Allergies	None Documented

Current medications: Inpatient medications (ST)
Medications (10) Active
SCHEDULED: (8)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq (NS w/K20 20 mEq) 20 mEq 1,000 mL, IV, 75 mL/hr
PRN: (1)
acetaminophen 500 mg Tab (Tylenol) 500 mg 1 Tabs, Oral, q6H

Problem list:
Active Problems (12)
Alcohol abuse
Anxiety
At risk for falls
Cardiac arrest
Convulsion
Depression
Hypothyroid
Ineffective airway clearance
MRSA
Obesity
Pneumonia
Ventilator dependence

Assessment

Interpretation of Results

Laboratory: Fishbone Labs ST
MAR 05 08:04

C 122 | L 86 | H 23 /
_____ 93

3.5 | 28 **| H 1.41 **

MAR 05 08:04

Progress Note - Physician

```
      \  11.9   /
4.60  _______  176

      /  33.8   \.
```

Plan

AKI (acute kidney injury) (N17.9)
- denies hx of ckd, prior cr of 1 on 1/20/23
- due to pre-renal from poor po in setting of diuresis
- improving with ivf per below

Critical hyponatremia - due to hypovolemia in setting of diuresis
- urine studies support hypovolemia
- s/p 1l ns on admission
- na correcting at appropriate rate
- adjusting ivf composition and rate per na level
- cont Na q4 for now
- initial Na 116 at midnight, goal of ~8 per 24 hours; **goal of ~128 at midnight**
- cont fluid restriction 1500 ml/day

Hypokalemia (E87.6) - due to poor po and diuretics
- kcl added to ivf
- replace per protocol

CHF EF 30-35%
- diuretics and sglt2i on hold

Dental abscess (K04.7)
Facial cellulitis (L03.211)
Severe sepsis (R65.20)
Weakness (R53.1)

Thank you for consult. Will cont to follow closely.

Electronically Signed By: Piceno, Jessie MD
On: 03.05.2024 12:31 PST

DOCUMENT NAME: Progress Note-Physician
SERVICE DATE/TIME: 3/5/2024 10:23 PST
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Valina DO,Anisa M (3/5/2024 10:24 PST)
SIGN INFORMATION: Valina DO,Anisa M (3/5/2024 10:24 PST)

Progress Note - Physician

Subjective

No event overnight.
patient sodium is 123 this morning.
Patient resting comfortably.

Past Medical History

Active Problems(10)

Alcohol abuse
Anxiety
Cardiac arrest
Convulsion
Depression
Hypothyroid
MRSA
Obesity
Pneumonia
Ventilator dependence

Past Surgical History

TRACH

Objective

Measurements (most recent)

Height (cm)	175.26 cm	Weidner RN, Robyn	03/03 23:13
Height (inches)	69.00 inch	SYSTEM	03/03 23:13
Weight	99.79 kg	Weidner RN, Robyn	03/03 23:13
Weight (lbs)	220.00 lb	SYSTEM	03/03 23:13
Body Mass Index	32.49 kg/m2	Weidner RN, Robyn	03/03 23:13

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	36.8	03/05/2024 07:02	**L** 36.4	03/05/2024 03:00	37.0	03/04/2024 20:03
Heart Rate Monitored	76	03/05/2024 09:09	**L** 59	03/04/2024 13:01	76	03/05/2024 09:09
Resp Rate	17	03/05/2024 09:09	16	03/04/2024 11:31	18	03/05/2024 07:52
SBP	107	03/05/2024 09:09	90	03/04/2024 18:00	133	03/05/2024 08:29
DBP	**L** 57	03/05/2024 09:09	**L** 50	03/04/2024 17:00	72	03/05/2024 01:00
MAP	77	03/05/2024 09:09	67	03/04/2024 18:00	91	03/04/2024 12:00
SpO2	95	03/05/2024 09:09	**L** 90	03/04/2024 16:00	100	03/04/2024 12:00
O2 Flow Rate	on 2 l/min		on 2 l/min			
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Progress Note - Physician

Intake and Output (current encounter)

	03/04 02:25 03/05 10:23	03/05 07:00 03/05 10:23	03/04 07:00 03/05 06:59	03/03 07:00 03/04 06:59	03/02 07:00 03/03 06:59
Intake	1050	0	50	1000	0
Output	1125	0	1125	0	0
Net Total	-75	0	-1075	1000	0

Physical Exam

General: In no acute cardiopulmonary distress. obesity.
HEENT: NC, AT, PERRLA, Neck is supple. Full ROM.
Respiratory: Clear to auscultation.
Cardiovascular: S1, S2, RRR.
Gastrointestinal: Abdomen soft, non-tender, abd obesity.
Neurologic: No focal neurological deficit.
Extremities: no C/C/E. Pulses intact.

Inpatient Medications
Medications (9) Active
SCHEDULED: (8)
buPROPion 150 mg/12 hours SR Tab (buPROPion SR 12 hour extended release) 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab (famotidine) 20 mg 1 Tabs, Oral, Daily
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe (insulin aspart NovoLog correctional scale Low) 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab (levETIRAcetam) 500 mg 1 Tabs, Oral, BID
zonisamide 100 mg Cap (Zonegran) 200 mg 2 Caps, Oral, Daily
CONTINUOUS: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq (NS w/K20 20 mEq) 20 mEq 1,000 mL, IV, 75 mL/hr
PRN: (0)

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefepime 2 gm, 20 mL, 12.5 mL/hr, IV Piggyback, q12H Interval	2	03/05/2024 03:30	03/04/2024 02:55

Results
Recent Labs

Progress Note - Physician

Cardiac

Troponin TNIH	4.33 ng/L (Normal)	03/04/2024 02:25

General Chemistry

Glucose Level	93 mg/dL (Normal)	03/05/2024 08:04
POC Glucose	96 mg/dL (Normal)	03/05/2024 07:50
Sodium	123 mmol/L (Critical)	03/05/2024 08:43
Potassium	3.5 mmol/L (Normal)	03/05/2024 08:04
Chloride	86 mmol/L (Low)	03/05/2024 08:04
CO2	28 mmol/L (Normal)	03/05/2024 08:04
Anion Gap	8 mmol/L (Normal)	03/05/2024 08:04
BUN	23 mg/dL (High)	03/05/2024 08:04
Creatinine	1.41 mg/dL (High)	03/05/2024 08:04
BUN/Creat Ratio	16 (Normal)	03/05/2024 08:04
Calcium	9.2 mg/dL (Normal)	03/05/2024 08:04
Albumin. Level	3.9 gm/dL (Normal)	03/05/2024 08:04
Phosphorus	3.0 mg/dL (Normal)	03/05/2024 08:04
Estimated Creatinine Clearance	41.02 mL/min ()	03/05/2024 09:12
eGFR Cr	41 mL/min/1.73m2 (N/A)	03/05/2024 08:04
eGFR Pediatric	Not Reported (N/A)	03/05/2024 08:04
Uric Acid	6.9 mg/dL (High)	03/04/2024 05:11
Lactic Acid Lvl	1.4 mmol/L (Normal)	03/04/2024 02:25
MAR Glucose Result	96 ()	03/05/2024 07:52

General Coagulation

PT	10.9 Seconds (Normal)	03/03/2024 23:49
INR	1.0 (Normal)	03/03/2024 23:49

General Hematology

WBC	4.60 x10e3/mcL (Normal)	03/05/2024 08:04
RBC	2.86 x10e6/mcL (Low)	03/05/2024 08:04
Hgb	11.9 gm/dL (Normal)	03/05/2024 08:04
Hct	33.8 % (Normal)	03/05/2024 08:04
MCV	118.2 Femtoliters (High)	03/05/2024 08:04
MCH	41.6 pg (High)	03/05/2024 08:04
MCHC	35.2 gm/dL (High)	03/05/2024 08:04
RDW-SD	63.6 Femtoliters (High)	03/05/2024 08:04

Progress Note - Physician

RDW-CV	14.5 % (Normal)	03/05/2024 08:04
Plt	176 x10e3/mcL (Normal)	03/05/2024 08:04
MPV	9.6 Femtoliters (Normal)	03/05/2024 08:04
Neut % Auto	70.0 % (N/A)	03/05/2024 08:04
Lymph % Auto	17.2 % (N/A)	03/05/2024 08:04
Mono % Auto	11.3 % (N/A)	03/05/2024 08:04
Eos % Auto	0.4 % (N/A)	03/05/2024 08:04
Baso % Auto	0.7 % (N/A)	03/05/2024 08:04
Immature Grans %	0.4 % (Normal)	03/05/2024 08:04
Neut # Auto	3.22 x10e3/mcL (Normal)	03/05/2024 08:04
Lymph # Auto	0.79 x10e3/mcL (Low)	03/05/2024 08:04
Mono # Auto	0.52 x10e3/mcL (Normal)	03/05/2024 08:04
Eos # Auto	0.02 x10e3/mcL (Normal)	03/05/2024 08:04
Baso # Auto	0.03 x10e3/mcL (Normal)	03/05/2024 08:04
Immature Grans # Auto	0.02 x10e3/mcL (Normal)	03/05/2024 08:04
RBC Morph	Abnormal (Abnormal)	03/03/2024 23:49
Plt Morphology	Normal (Normal)	03/03/2024 23:49
Macro	2+ (Abnormal)	03/03/2024 23:49
Plt Est	Adequate (Normal)	03/03/2024 23:49

Other Chemistry

Procalcitonin Lvl	0.57 ng/mL (High)	03/05/2024 08:04

Thyroid

TSH Ultrasensitive	13.707 mc Intl units/mL (High)	03/04/2024 05:11

UA Macroscopic

Ur Color	Yellow (Normal)	03/04/2024 00:47
Ur Clarity	Cloudy (Abnormal)	03/04/2024 00:47
UR Glucose	Negative (Normal)	03/04/2024 00:47
Ur Bili	Negative (Normal)	03/04/2024 00:47
Ur Ketones	Negative (Normal)	03/04/2024 00:47
Ur Spec Grav	1.009 (Low)	03/04/2024 00:47
Ur Blood	Negative (Normal)	03/04/2024 00:47
Ur pH	5.0 (Normal)	03/04/2024 00:47

Progress Note - Physician

Ur Protein	Negative (Normal)	03/04/2024 00:47
Ur Urobilinogen	<2.0 mg/dL (Normal)	03/04/2024 00:47
Ur Nitrite	Negative (Normal)	03/04/2024 00:47
Ur Leuk Est	3+ (Abnormal)	03/04/2024 00:47
UA Culture?	Yes (N/A)	03/04/2024 00:47

UA Microscopic

UA WBC Inst	>182 /HPF (High)	03/04/2024 00:47
UA RBC Inst	5 /HPF (High)	03/04/2024 00:47
Ur Bacteria	3+ /HPF (Abnormal)	03/04/2024 00:47
UA Squam Epi Inst	3 /HPF (Normal)	03/04/2024 00:47
Ur WBC Clumps	Few /HPF (Abnormal)	03/04/2024 00:47
Ur Mucous	Rare /LPF (Abnormal)	03/04/2024 00:47
Ur Hyal Cast	6 /LPF (High)	03/04/2024 00:47

Urine Chemistry

Ur Sodium	34 mmol/L (N/A)	03/04/2024 02:18
Ur Creat	73.9 mg/dL (N/A)	03/04/2024 02:18
Ur Chloride	60 mmol/L (N/A)	03/04/2024 02:18
Ur Osmolality	274 mOsmol/kg (Normal)	03/04/2024 02:18

Blood Glucose Trend

Glucose Level: 93 mg/dL (03/05/24 08:04:00)
Glucose Level: 87 mg/dL (03/04/24 12:00:00)
POC Glucose: 96 mg/dL (03/05/24 07:50:00)

Microbiology

Culture Urine **Preliminary**

Source: U CleanCatch Body Site:
Collected Dt/Tm: 03/04/2024 00:47 Last Updated Dt/Tm: 03/05/2024 03:17

```
>100,000 cfu/ml Klebsiella pneumoniae
Susceptibility to follow.
```

Culture Blood ARD **Preliminary**

Source: Blood Body Site:
Collected Dt/Tm: 03/03/2024 23:49 Last Updated Dt/Tm: 03/05/2024 08:01

Progress Note - Physician

No growth at 24 hours.

Culture Blood ARD		**Preliminary**	
Source:	Blood	Body Site:	
Collected Dt/Tm:	03/03/2024 23:49	Last Updated Dt/Tm:	03/05/2024 08:01

No growth at 24 hours.

Assessment/Plan
Donna Wolfbarndt
DOB: 03/25/1957

Assessment:
Hyponatremia
UTI
Acute kidney injury
Facial cellulitis
Severe sepsis, present on admission, source urinary versus skin soft tissue
history of traumatic brain injury, she has right sided weakness and balance issues, cardiac arrest, and hypertension

Hyponatremia (E87.1):
Admit to ICU, klebsialla
Every 4 hours BMP, follow result
Nephrology consult obtained, NNC
Close neurological monitoring
Seizure precautions
Avoid overcorrection, goal sodium correction 6-8 in 24 hours

AKI (acute kidney injury) (N17.9):
Nephrology following
Urinary catheter
Strict I's/O
Avoid nephrotoxic agents as able
Urine electrolytes pending

Severe sepsis (R65.20): Cefepime for empiric antibiotic coverage
Dose for renal function
Blood culture sent, follow result
Urine culture sent, follow result patient with left facial cellulitis and UA positive for UTI
No further IV fluid boluses to avoid overcorrection in sodium or further drop in sodium given patient with hyponatremia
Sepsis reassessed

Progress Note - Physician

VTE Prophylaxis: Subcutaneous heparin
» **VTE Prophylaxis Assessment:** VTE Prophylaxis Ordered

At this time patient remains critical, at risk of decompensation or death
37 minutes of cumulative critical care time were utilized, excluding procedures performed and discussion with family. This time includes patient history, exam, review of laboratory results and relevant imaging, and complex medical decision-making.
Above note was generated with the voice recognition software used by Cerner EMR. Efforts have been made to ensure accuracy of all notes, however, occasional wording errors and/or typos may persist.

Electronically Signed By: Valina, Anisa DO
On: 03.05.2024 10:24 PST

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	3/5/2024 05:32 PST
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Zaugg DO,Garet J (3/5/2024 08:12 PST)
SIGN INFORMATION:	Zaugg DO,Garet J (3/5/2024 08:12 PST)

Las Vegas Neurology Center Note

SUBJECTIVE:

Patient seen and examined. No overnight events noted.

Patient pleasant this morning, states she feels tired and fatigued. Nephrology managing AKI and hyponatremia. Patient states she feels about the same today, no new complaints. Patient remains ICU status. No seizures noted, continuing Keppra.

Review of systems other than mentioned above was negative upon patient interview.
I reviewed the chart, labs, orders, and all events of the last 48 hours.

OBJECTIVE:

Vital Signs (last 24 hrs)	Last Charted	Minimum	Maximum
Heart Rate	66 (MAR 05 07:52)	**L** 57 (MAR 04 09:00)	68 (MAR 05 00:03)
Resp Rate	18 (MAR 05 07:52)	16 (MAR 04 09:00)	18 (MAR 04 10:00)
SBP	126 (MAR 05 07:52)	90 (MAR 04 18:00)	132 (MAR 05 04:00)
DBP	**L** 58 (MAR 05 07:52)	**L** 50 (MAR 04 17:00)	72 (MAR 05 01:00)
SpO2	99 (MAR 05 07:52)	**L** 90 (MAR 04 16:00)	100 (MAR 04 10:00)

Medications (9) Active
Scheduled: (8)
buPROPion 150 mg/12 hours SR Tab 150 mg 1 Tabs, Oral, BID
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
famotidine 20 mg Tab 20 mg 1 Tabs, Oral, Daily

Progress Note - Physician

heparin 5000 units/mL 1 mL Inj 5,000 units 1 mL, SubCutaneous, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin aspart NovoLOG 1 unit/ 0.01mL syringe 2-8 units, SubCutaneous, Before Meals and HS
levETIRAcetam 500 mg Tab 500 mg 1 Tabs, Oral, BID
zonisamide 100 mg Cap 200 mg 2 Caps, Oral, Daily
Continuous: (1)
Sodium Chloride 0.9% with KCl 20 mEq/l 20 mEq 20 mEq 1,000 mL, IV, 50 mL/hr
PRN: (0)
Labs (Last four charted values)

WBC	H **10.94** (MAR 03)			
Hgb	12.6 (MAR 03)			
Hct	34.0 (MAR 03)			
Plt	202 (MAR 03)			
Na	C **125** (MAR 05)	C **124** (MAR 05)	C **122** (MAR 04)	C **119** (MAR 04)
K	C **2.5** (MAR 04)	L **3.2** (MAR 03)		
CO2	28 (MAR 04)	31 (MAR 03)		
Cl	L **89** (MAR 04)	C **76** (MAR 03)		
Cr	H **1.60** (MAR 04)	H **1.85** (MAR 03)		
BUN	H **22** (MAR 04)	H **22** (MAR 03)		
Glucose Random	87 (MAR 04)	H **117** (MAR 03)		
Ca	L **7.4** (MAR 04)	9.3 (MAR 03)		
PT	10.9 (MAR 03)			
INR	1.0 (MAR 03)			

PHYSICAL EXAM:

GEN: Tired, oriented X 3/ No Acute Distress

HEENT: Atraumatic/Normocephalic

Neck: supple, no JVD

NEURO:

Mood/Affect: Appropriate to situation

Attention/Concentration: Intact

Fund of Knowledge: Fair

Speech: Fluent

CN 2-12: Left nasolabial fold blunting due to facial cellulitis

EOMI/PERRLA

Motor Strength: 4-/5 R arm, 3+/5 R leg, 4/5 L arm, 4-/5 L leg

Tone/Bulk: Intact

Sensation: Intact to light touch

Reflexes: 2/4 throughout

Progress Note - Physician

Finger-nose-finger: No dysmetria

Ambulation: deferred

Babinski: negative bilaterally

Imaging:

ASSESSMENT:
Generalized asthenia in setting of the below
Hyponatremia
AKI
UTI
Facial cellulitis
History of TBI with resultant mild right hemiparesis
CHF
Hypertension
Hypothyroidism
Mood disorder NOS

PLAN:
CT brain reassuring
No indication for further neurologic workup, etiology of generalized weakness is known, no new focal deficits
Follow nephrology recs
Recommend avoiding cefepime given propensity for CNS toxicity, lowering of seizure threshold in setting of renal insufficiency
PT/OT when clinically appropriate
BP control
Sepsis management per ICU

Garet Zaugg, D.O.
Board-Certified Neurologist
Las Vegas Neurology Center

Electronically Signed By: Zaugg, Garet DO
On: 03.05.2024 08:12 PST

Orders

Order: **Consult to Case Management (Case Management Consult)**			
Order Date/Time: 3/5/2024 10:21 PST			
Signed Date/Time: 3/5/2024 10:21 PST			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Consults	Activity Type: Case Management Consults
End-state Date/Time: 3/5/2024 10:21 PST		End-state Reason:	
Ordering Physician: Smuckler DO,Jason M		Consulting Physician:	
Entered By: Valina DO,Anisa M on 3/5/2024 10:21 PST			
Order Details: 3/5/24 10:21:00 AM PST, Routine, Reason for Consult: D/C to Home w Home Health, For PT services. Says she has a home PCA and home health set up?			
Order Comment:			

Orders - Admission/Discharge (Curaspan)

Order: **Placement Change**			
Order Date/Time: 3/5/2024 10:25 PST			
Signed Date/Time: 3/5/2024 10:25 PST			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/5/2024 10:25 PST		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/5/2024 10:25 PST			
Order Details: 3/5/24 10:25:59 AM PST, Suggested Placement: Intermediate Care/Step down			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			
Action Type: Order	Action Date/Time: 3/5/2024 10:25 PST	Action Personnel: SYSTEM	
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert	
Order Details: 03/05/24 10:25:59 PST, Suggested Placement: Intermediate Care/Step down			
Review Information: Nurse Review: Electronically Signed, Fishman RN,Lindsey on 3/5/2024 14:48 PST Doctor Cosign: Not Required			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Orders - Admission/Discharge (Curaspan)

Order: **Change Attending Physician**			
Order Date/Time: 3/5/2024 10:25 PST			
Signed Date/Time: 3/5/2024 10:25 PST			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/5/2024 10:35 PST		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/5/2024 10:25 PST			
Order Details: 3/5/24 10:25:59 AM PST, Admitting: Timple DO, Julie M, Attending: Timple DO, Julie M			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/5/2024 10:35 PST	Action Personnel: Hernandez,Stephanie
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/05/24 10:25:59 PST, Admitting: Timple DO, Julie M, Attending: Timple DO, Julie M		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/5/2024 10:25 PST	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/05/24 10:25:59 PST, Admitting: Timple DO, Julie M, Attending: Timple DO, Julie M		
Review Information: Nurse Review: Not Reviewed Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Patient: WOLFBRANDT, DONNA J
MRN: CHH7301725; SHM4680279
DOB/Sex: 3/25/1957 / Female
Attending: Smuckler DO,Jason M

Admit: 3/4/2024
Disch:
FIN: CHH0008011012203

Orders - Admission/Discharge (Curaspan)

Order: **Change Attending Physician**			
Order Date/Time: 3/5/2024 12:20 PST			
Signed Date/Time: 3/5/2024 12:20 PST			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/5/2024 12:26 PST		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/5/2024 12:20 PST			
Order Details: 3/5/24 12:20:58 PM PST, Admitting: Timple DO, Julie M, Attending: Tiwari, Subhikshya MD			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/5/2024 12:26 PST	Action Personnel: Hart,Demi
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/05/24 12:20:58 PST, Admitting: Timple DO, Julie M, Attending: Tiwari, Subhikshya MD		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/5/2024 12:20 PST	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/05/24 12:20:58 PST, Admitting: Timple DO, Julie M, Attending: Tiwari, Subhikshya MD		
Review Information: Nurse Review: Not Reviewed Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Orders - Admission/Discharge (Curaspan)

Order: **Change Attending Physician**			
Order Date/Time: 3/5/2024 21:56 PST			
Signed Date/Time: 3/5/2024 21:56 PST			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/5/2024 22:05 PST		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/5/2024 21:56 PST			
Order Details: 3/5/24 9:56:29 PM PST, Admitting: Timple DO, Julie M, Attending: Smuckler DO, Jason M			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/5/2024 22:05 PST	Action Personnel: Adams,Ivelisse
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/05/24 21:56:29 PST, Admitting: Timple DO, Julie M, Attending: Smuckler DO, Jason M		
Review Information: Doctor Cosign: Not Required		
Order Comment:		
Action Type: Order	Action Date/Time: 3/5/2024 21:56 PST	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/05/24 21:56:29 PST, Admitting: Timple DO, Julie M, Attending: Smuckler DO, Jason M		
Review Information: Nurse Review: Electronically Signed, Cypher RN,Tiffany on 3/5/2024 22:02 PST Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Physical Therapy Forms

DOCUMENT TYPE:	PT Daily Documentation - Text
SERVICE DATE/TIME:	3/7/2024 11:00 PST
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Qiu PTA,Carrie (3/7/2024 11:39 PST)
SIGNED INFORMATION:	Qiu PTA,Carrie (3/7/2024 11:39 PST)

PT Daily Documentation Entered On: 3/7/2024 11:39 PST
Performed On: 3/7/2024 11:00 PST by Qiu PTA, Carrie

Review/Treatments Provided
PT Goals : PT Short Term Goals

No qualifying data available
PT Plan : Treatment Frequency: Other: 4-5x/week Performed By: Aikels PT, MPT, Dawn 03/06/2024
Treatment Duration: 1-2 weeks Performed By: Aikels PT, MPT, Dawn 03/06/2024

Physical Therapy Forms

Planned Treatments: Balance training, Basic activities of daily living, Bed mobility training, Equipment training, Gait training, Manual therapy, Neuromuscular reeducation, Patient education, Therapeutic exercises, Transfer training Performed By: Aikels PT, MPT, Dawn 03/06/2024
Physical Therapy Orders : No active Physical Therapy Orders.
Pain Symptoms : No
Suspected Stroke Patient : No
Post-Acute Discharge Therapy Recommendation PT : Continued intermittent therapy to address functional deficits
PT Therapeutic Exercise : Yes

Qiu PTA, Carrie - 3/7/2024 11:35 PST

Mobility
Mobility Grid
Roll Left : Rehab Minimal assistance
Roll Right : Rehab Minimal assistance
Supine to Sit : Rehab Minimal assistance
Sit to Supine : Rehab Minimal assistance
Transfer Sit to Stand : Rehab Minimal assistance
Transfer Stand to Sit : Rehab Minimal assistance

Qiu PTA, Carrie - 3/7/2024 11:35 PST

Ambulation Level : Min A
Device : Front-wheel walker (FWW)
Ambulation Distance : 5 ft

Qiu PTA, Carrie - 3/7/2024 11:35 PST

Therapeutic Exercise
Therapeutic Exercise Grid

	Exercise 1
Exercise :	Lower extremity strengthening
Position :	Unsupported sit
Repetition/Time :	10x1
Completed :	Yes
Comment :	AP, GS, LAQ
	Qiu PTA, Carrie - 3/7/2024 11:35 PST

Education
Teaching Method : Demonstration, Explanation
Barriers to Learning : None evident

Qiu PTA, Carrie - 3/7/2024 11:35 PST

Physical Therapy Education Grid
Ambulation with Roller Walker : Verbalizes understanding, Demonstrates, Needs further teaching
Bed Mobility : Verbalizes understanding, Demonstrates, Needs further teaching
Bed Positioning : Verbalizes understanding, Demonstrates, Needs further teaching
Sit to Stand Transfers : Needs further teaching, Demonstrates, Verbalizes understanding
Exercise Program : Verbalizes understanding, Demonstrates, Needs further teaching

Qiu PTA, Carrie - 3/7/2024 11:35 PST

Physical Therapy Forms

Additional Information
Additional Information PT : RN consulted and cleared pt. for PT tx. Pt. presents semi supine in bed w/ no c/o pain and agreeable to tx. Pt. on 1.5L NCO2 throughout session. Bed mobility training semi supine<>sit EOB min A w/ no dizziness w/ position change. Transfer training sit<>stand FWW min A w/ no LOB or dizziness w/ upright position. Gait training 5'x1 FWW min A forward, pt. demos step to gait pattern w/ significantly decreased stride length and cadence, pt. demos SOB and labored breathing, no decrease in O2 sat, pt. limited by decreased endurnace for further OOB tx at this time. Assisted pt. BTB and instructed w/ BLE ther ex's as charted above. Pt. left semi supine in bed w/ bed alarm on, call light in reach, positioned for comfort & all needs met.

Barriers to DC: fall risk

Total in time: 40 min

supervised by Krista Rapozo, PT, DPT.

Qiu PTA, Carrie - 3/7/2024 11:39 PST

Environmental Safety Implemented : Adequate room lighting, Bed in low position, Call device within reach, Personal items within reach

Qiu PTA, Carrie - 3/7/2024 11:35 PST

PTA Timed Modalities 2024
PTA Timed Min Total : 40
PTA Timed Units Total : 3
PTA THERAPEUTIC ACT 15MIN : 15
ACTIVITY THERAPEUTIC CQ : 1
PTA EXERCISE THERAPEUTIC 15MIN : 10
EXERCISE THERAPEUTIC CQ : 1
PTA GAIT TRAINING 15MIN : 15
GAIT TRAINING CQ : 1

Qiu PTA, Carrie - 3/7/2024 11:35 PST

DOCUMENT TYPE:	PT Examination - Text
SERVICE DATE/TIME:	3/6/2024 08:30 PST
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Aikels PT,MPT,Dawn (3/6/2024 13:53 PST)
SIGNED INFORMATION:	Aikels PT,MPT,Dawn (3/6/2024 13:53 PST)

PT Evaluation/Reevaluation Entered On: 3/6/2024 13:32 PST
Performed On: 3/6/2024 8:30 PST by Aikels PT, MPT, Dawn

General Info
PT Assessment Type : Initial Evaluation
Physical Therapy Orders : PT Evaluation and Treatment - 03/05/24 14:54:00 PST, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, Yes, Yes
Precautions RTF : Active Orders:
» Oxygen Therapy, 03/03/24 23:27:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater

than 90%, Ordered
» Oxygen Therapy, 03/03/24 23:23:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 94%, Ordered

Orientation Assessment Rehab : Oriented x 4
Affect/Behavior Rehab : Appropriate, Cooperative
Basic Command Following : Intact
Safety/Judgment : Intact
Pain Symptoms : No
Suspected Stroke Patient : No

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Home Environment
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Fishman RN, Lindsey 03/05/2024
Living Situation : Home with family care
Lives With : Child(ren)
Lives In : Apartment

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Home Environment II
Living Environment : Home Environment
Living Situation: Home with family care Performed By: Fishman RN, Lindsey 03/05/2024
Home Equipment Rehab : 4-Wheel walker, Manual wheelchair

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Prior Functional Status Grid
ADL : Assist needed
Mobility : Independent
Instrumental ADL : Dependent
Cognitive-Communication Skills : Independent

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

LE Range/Strength
LE Overall Range of Motion Grid
Left Lower Extremity Passive Range : Within functional limits
Left Lower Extremity Active Range : Within functional limits
Right Lower Extremity Passive Range : Within functional limits
Right Lower Extremity Active Range : Within functional limits

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Left Lower Extremity Strength Grid
Hip Flexion : 3
Hip Extension : 3
Hip Abduction : 3
Hip Adduction : 3
Hip External Rotation : 3
Hip Internal Rotation : 3
Knee Flexion : 3+
Knee Extension : 3+
Ankle Dorsiflexion : 3+
Ankle Plantarflexion : 3+
Ankle Eversion : 3
Ankle Inversion : 3

Physical Therapy Forms

Great Toe Extension : 3

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Right Lower Extremity Strength Grid
Hip Flexion : 3
Hip Extension : 3
Hip Abduction : 3
Hip Adduction : 3
Hip External Rotation : 3
Hip Internal Rotation : 3
Knee Flexion : 3+
Knee Extension : 3+
Ankle Dorsiflexion : 3+
Ankle Plantarflexion : 3+
Ankle Eversion : 3
Ankle Inversion : 3
Great Toe Extension : 3

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Balance
Kansas University Balance Scale : Yes
Sitting Balance Score : 4
Standing Balance Score : 1+

Aikels PT, MPT, Dawn - 3/6/2024 13:29 PST

Mobility
Mobility Grid
Roll Left : Rehab Minimal assistance
Roll Right : Rehab Minimal assistance
Supine to Sit : Rehab Minimal assistance
Sit to Supine : Rehab Minimal assistance
Transfer Sit to Stand : Rehab Minimal assistance
Transfer Stand to Sit : Rehab Minimal assistance

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Ambulation Level : Does not occur

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Assessment
PT Impairments or Limitations : Ambulation deficits, Balance deficits, Bed mobility deficits, Endurance deficits, Equipment training, Strength deficits, Transfer deficits, Transition deficits
Barriers to Safe Discharge PT : Medical diagnosis

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Education
Teaching Method : Demonstration, Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Physical Therapy Education Grid
Bed Mobility : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Bed Positioning : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision

Physical Therapy Forms

Sit to Stand Transfers : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Physical Therapy Plan of Care : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Long Term Goals
PT Patient,Caregiver Goal : return home safely

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

PT Long Term Goals Rehab

	Long Term Goal 1	Long Term Goal 2	Long Term Goal 3
Goal :	Ind bed mob	MI transfers using LRAD	MI ambulation x 150' using LRAD
Status :	Initial	Initial	Initial
Timeframe To Complete :	1-2 weeks	1-2 weeks	1-2 weeks
Comments :	LTG=STG		
	Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST	Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST	Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Plan
PT Anticipated Treatments, Needs : Balance training, Basic activities of daily living, Bed mobility training, Equipment training, Gait training, Manual therapy, Neuromuscular reeducation, Patient education, Therapeutic exercises, Transfer training
PT Frequency Rehab : Other: 4-5x/week
PT Duration Rehab : 1-2 weeks
PT Intensity Per Day : 30-60 minutes
Rehabilitation Potential : Good
PT Plan/Goals Established w Pt/Caregiver : Yes
Post-Acute Discharge Therapy Recommendation PT : Continued intermittent therapy to address functional deficits

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Additional Information
Additional Information PT : This pt is a 66 yo female who presented to CHH with the primary c/o waekness. Ground level fall experienced and EMS contacted. She was found to be hypotensive on the seen. CT reveals wht matter changes and the pt was assessed as having AKI, facial cellulitis and UTI

PMH: cardiac arrest, TBI, R hemi, HTN, CHF, depression
PLOF: the pt lives with son Down's syndrome son in a 1st floor apt and was MI for ambulation, household distances using 4WW. She has a PCA ~4 hours/day to assist with G/H and higher level household ADLs/IADLs

Treatment recap: After nsg authorized IE and tx, the pt is seen in a semisupine position and ageed to services. She performs mobility per grid when HOB is elevated and with railing requiring min A for sup>sit transfers to EOB. No visible LOB noted at EOB. STS is with min A using FWW. While standing, increased

Physical Therapy Forms

global tremors noted which increase with standing time. She declined prog gt training d/t uncontrollable tremors but was able to take a few side steps toward HOB. She returns BTB with all needs met and CLIR. Intermittent verbal cues required for sequencing and safety during transitional movements including proper hand placement. All vitals monitored and found to be WFL. Nsg notified of the pts function. She was found appropriate for continued skilled intervention to promote improved function and reduce fall risk
Px: fall risk, foley cath
Barriers to DC: fall risk

Treatment time: 40 minutes

Aikels PT, MPT, Dawn - 3/6/2024 13:53 PST

Environmental Safety Implemented : Adequate room lighting, Bed in low position, Call device within reach, Personal items within reach

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

PT Non Timed Modalities 2024
PT Eval Moderate Complexity Total Number Min : 15
PT Total Non Timed : 15

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

PT Timed Modalities 2024
PT Timed Min Total : 25
PT Timed Unit Total : 2
PT THERAPEUTIC ACT 15MIN : 25
Activity Therapeutic Num of Units : 2

Aikels PT, MPT, Dawn - 3/6/2024 13:33 PST

Occupational Therapy Forms

DOCUMENT TYPE:	OT Daily Documentation - Text
SERVICE DATE/TIME:	3/7/2024 12:32 PST
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Nguyen OT,Anthony (3/7/2024 14:41 PST)
SIGNED INFORMATION:	Nguyen OT,Anthony (3/7/2024 14:41 PST)

OT Daily Documentation Entered On: 3/7/2024 14:52 PST
Performed On: 3/7/2024 12:32 PST by Nguyen OT, Anthony

Review/Treatments Provided
OT Goals : OT Short Term Goals

No qualifying data available
OT Plan : Treatment Frequency: 5-7 times per week Performed By: Nguyen OT, Anthony 03/06/2024
Treatment Duration: 1-2 weeks Performed By: Nguyen OT, Anthony 03/06/2024
Planned Treatments: Balance training, Basic Activities of Daily Living, Equipment training, Home program, Mobility training, Pain management, Patient education, Safety education, Therapeutic exercises Performed By: Nguyen OT, Anthony 03/06/2024
Occupational Therapy Orders : No active Occupational Therapy orders.

Occupational Therapy Forms

Pain Symptoms : No
Suspected Stroke Patient : No
Post-Acute Discharge Therapy Recommendation OT : Continued intermittent therapy to address functional deficits
OT Therapeutic Activity,Mobility,Balance : Yes

Nguyen OT, Anthony - 3/7/2024 14:41 PST

Therapeutic Activities
Reassess Activities of Daily Living : Yes

Nguyen OT, Anthony - 3/7/2024 14:41 PST

Basic ADL
Upper Extremity Dressing Indep Measure
Grooming : Min A
UE Dressing : Min A
Toileting : Total Assistance

Nguyen OT, Anthony - 3/7/2024 14:41 PST

Mobility
Mobility Grid
Supine to Sit : Rehab Minimal assistance
Sit to Supine : Rehab Minimal assistance
Transfer Sit to Stand : Rehab Minimal assistance
Transfer Stand to Sit : Rehab Minimal assistance

Nguyen OT, Anthony - 3/7/2024 14:41 PST

Ambulation Level OT : Min A
Device utilized OT : Front-wheel walker (FWW)

Nguyen OT, Anthony - 3/7/2024 14:41 PST

Additional Information
Additional Information OT : Room 402

Pt cleared by NSG for OT tx. Pt waited to recieve her neuro medication prior to her tx. Pt min A for supine to EOB. Pt min A for sit to stand with FWW. Pt ambulated ~5ft until she demo increased tremors. Pt had difficulty stepping back to EOB due to tremor and weakness. Bed released to bring next to patient to sit back down. Pt demos increased fatigue and expressed frustrations on how debilitated she is now. Therapeutic listening provided with patient at EOB. Pt declined using FWW to side step to HOB. Pt able to scoot to HOB with CGA. Pt groomed self at EOB with min A. Pt returned semisupine in bed with min A. Pt will cont to benefit from skilled OT for improved functional independence. Time spent: 38 min

DC recommendation: intermittent therapy
Dx: weakness, hyponatremia
Barriers: weakness, poor mobility and self care
Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach

Nguyen OT, Anthony - 3/7/2024 14:41 PST

OT Timed Modalities 2024
OT Timed Min Total : 38
OT Timed Units Total : 3
OT THERAPEUTIC ACT 15MIN : 38
OT THERAPEUTIC ACT Num of Units : 3

Nguyen OT, Anthony - 3/7/2024 14:41 PST

Occupational Therapy Forms

DOCUMENT TYPE:	OT Evaluation - Text
SERVICE DATE/TIME:	3/6/2024 09:01 PST
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Nguyen OT,Anthony (3/6/2024 14:28 PST)
SIGNED INFORMATION:	Nguyen OT,Anthony (3/6/2024 14:28 PST)

OT Evaluation/Reevaluation Entered On: 3/6/2024 14:42 PST
Performed On: 3/6/2024 9:01 PST by Nguyen OT, Anthony

General Information
OT Assessment Type : Initial Evaluation
Occupational Therapy Orders : OT Evaluation and Treatment - 03/05/24 14:54:00 PST, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, Yes, Yes
Precautions RTF : Active Orders:
» Oxygen Therapy, 03/03/24 23:27:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered
» Oxygen Therapy, 03/03/24 23:23:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 94%, Ordered

Orientation Assessment Rehab : Oriented x 4
Affect/Behavior Rehab : Appropriate, Calm, Cooperative
Basic Command Following : Intact
Safety/Judgment : Intact
Pain Symptoms : No
Suspected Stroke Patient : No

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Home Environment
Living Environment : Home Environment
*ADL: Assist needed Performed By: Aikels PT, MPT, Dawn 03/06/2024
*Cognitive-Communication Skills: Independent Performed By: Aikels PT, MPT, Dawn 03/06/2024
*Instrumental ADL: Dependent Performed By: Aikels PT, MPT, Dawn 03/06/2024
*Mobility: Independent Performed By: Aikels PT, MPT, Dawn 03/06/2024
Devices/Equipment at Home: 4-Wheel walker, Manual wheelchair Performed By: Aikels PT, MPT, Dawn 03/06/2024
Lives In: Apartment Performed By: Aikels PT, MPT, Dawn 03/06/2024
Lives With: Child(ren) Performed By: Aikels PT, MPT, Dawn 03/06/2024
Living Situation: Home with family care Performed By: Aikels PT, MPT, Dawn 03/06/2024
Living Situation : Home with family care
Lives With : Child(ren)
Lives In : Apartment
Patient's Responsibilities Rehab : Personal ADL

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Home Environment II

Occupational Therapy Forms

Living Environment : Home Environment
Living Situation: Home with family care Performed By: Fishman RN, Lindsey 03/05/2024
Home Equipment Rehab : 4-Wheel walker, Manual wheelchair
Nguyen OT, Anthony - 3/6/2024 14:28 PST

ADL : Assist needed
Mobility : Independent
Instrumental ADL : Dependent
Cognitive-Communication Skills : Independent
Nguyen OT, Anthony - 3/6/2024 14:28 PST

Basic ADL
Activities of Daily Living Grid
UE Dressing : Min A
Toileting : Total Assistance
Nguyen OT, Anthony - 3/6/2024 14:28 PST

Balance
General Balance : Standing
General Balance Assessment : Fair
Nguyen OT, Anthony - 3/6/2024 14:28 PST

UE Strength/ROM
Upper Extremity Overall ROM Grid
Left Upper Extremity Passive Range : Within functional limits
Left Upper Extremity Active Range : Within functional limits
Right Upper Extremity Passive Range : Within functional limits
Right Upper Extremity Active Range : Within functional limits
Nguyen OT, Anthony - 3/6/2024 14:28 PST

Mobility
Mobility Grid
Supine to Sit : Rehab Contact guard assistance
Sit to Supine : Rehab Contact guard assistance
Transfer Sit to Stand : Rehab Contact guard assistance
Transfer Stand to Sit : Rehab Contact guard assistance
Nguyen OT, Anthony - 3/6/2024 14:28 PST

Ambulation Level OT : Min A
Device utilized OT : Front-wheel walker (FWW)
Nguyen OT, Anthony - 3/6/2024 14:28 PST

OT Assessment
Impairments/Limitations : Balance deficits, Basic activity of daily living deficits, Endurance deficits, Equipment training, Mobility deficits, Strength deficits
Barriers to Safe Discharge : Medical diagnosis
Nguyen OT, Anthony - 3/6/2024 14:28 PST

Education
Teaching Method : Explanation
Learner(s) Present : Self
Barriers to Learning : None evident
Nguyen OT, Anthony - 3/6/2024 14:28 PST

Occupational Therapy Education Grid
Activity of Daily Living Training : Needs further teaching, Needs practice/supervision
Bed to Chair Transfers : Needs practice/supervision, Needs further teaching
Sit to Stand Transfers : Needs further teaching, Needs practice/supervision

Occupational Therapy Forms

Exercise Program : Needs further teaching, Needs practice/supervision
Home Safety : Needs practice/supervision, Needs further teaching
Plan of Care : Verbalizes understanding
Safety Awareness : Verbalizes understanding

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Long Term Goals
OT Patient/Caregiver Goal : Pt unable to state

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Long Term Goal 5

	Long Term Goal 1	Long Term Goal 2	Long Term Goal 3	Long Term Goal 4
Goal :	Pt to be mod I with transfers	Pt to be mod I with toileting	Pt to groom self in front of sink mod I	Pt to ambulate household distances mod I
Status :	Initial assessment	Initial assessment	Initial assessment	Initial assessment
Timeframe To Complete :	2 weeks	2 weeks	2 weeks	2 weeks
	Nguyen OT, Anthony - 3/6/2024 14:28 PST	Nguyen OT, Anthony - 3/6/2024 14:28 PST	Nguyen OT, Anthony - 3/6/2024 14:28 PST	Nguyen OT, Anthony - 3/6/2024 14:28 PST

Plan
OT Anticipated Treatments, Needs : Balance training, Basic Activities of Daily Living, Equipment training, Home program, Mobility training, Pain management, Patient education, Safety education, Therapeutic exercises
OT Frequency Rehab : 5-7 times per week
OT Duration Rehab : 1-2 weeks
OT Intensity Per Day : Other: as tolerated
Rehabilitation Potential : Excellent
OT Tx Plan/Goals Established w Patient : Yes
OT Evaluation Time : 15 Minutes
OT Evaluation Charge : Yes
Post-Acute Discharge Therapy Recommendation OT : Continued intermittent therapy to address functional deficits

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Additional Information
Additional Information OT : Room 402

Pt cleared by NSG for OT IE. Pt in bed and agreeable to OT IE. Pt is a 66 y/o female admitted for generalized weakness, hyponatremia, AKI, UTI, facial cellulitis. Pt has hx of TBI and mood disorder. Pt lives in 1st floor apt with son where she is mod I with ADL's except bathing and mod I with mobility at home. Pt has a self pay caregiver that comes to assist with bathing at home. Pt CGA for supine to EOB. Pt sat up EOB with SPV for safety. Pt donned clean gown with min A at EOB. Pt CGA for sit to stand with FWW. Pt had bowel accident during stand requiring total A for pericare. Pt side step to HOB with min A. Pt returned semisupine in bed with CGA. Pt educated on role of OT, POC, and recommendations. Pt demos impaired mobility, strength, balance, and self care. Pt will benefit from skilled OT for improved functional

Occupational Therapy Forms

independence. Time spent: 38 min

DC recommendation: intermittent therapy
Dx: generalized weakness, hyponatremia, AKI, UTI, facial cellulitis
PMH: TBI, mood disorder, CHF, HTN, hypothyroidism
Barriers: impaired mobility, strength, balance, and self care
Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach

Nguyen OT, Anthony - 3/6/2024 14:28 PST

OT Non Timed Modalities 2024
Eval Moderate Complexity Total Number Min : 15
OT Total NonTimed Mod : 15

Nguyen OT, Anthony - 3/6/2024 14:28 PST

OT Timed Modalities 2024
OT Timed Min Total : 23
OT Timed Units Total : 2
OT THERAPEUTIC ACT 15MIN : 23
OT THERAPEUTIC ACT Num of Units : 2

Nguyen OT, Anthony - 3/6/2024 14:28 PST

Speech Therapy

DOCUMENT TYPE:	Clinical Swallowing - Text
SERVICE DATE/TIME:	3/6/2024 12:17 PST
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Hernandez SLP,Gardenia (3/6/2024 16:12 PST)
SIGNED INFORMATION:	Hernandez SLP,Gardenia (3/6/2024 16:12 PST)

ST Bedside Swallow Eval Entered On: 3/6/2024 15:55 PST
Performed On: 3/6/2024 12:17 PST by Hernandez SLP, Gardenia

General Information
Speech Orders : ST Bedside Swallow Exam - 03/05/24 14:54:00 PST, Bedside Swallow Exam, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, IV Yes, O2 Yes, 16443966655.0
ST Eval and Treat - 03/05/24 14:54:00 PST, Bedside Swallow Exam, Routine, Stop date 03/05/24 14:54:00 PST, Stretcher, IV Yes, O2 Yes
Precautions RTF : Active Orders:
» Oxygen Therapy, 03/03/24 23:27:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered
» Oxygen Therapy, 03/03/24 23:23:00 PST, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 94%, Ordered

History of Swallow Studies : No qualifying data available

Hernandez SLP, Gardenia - 3/6/2024 15:55 PST

Pre-Assessment

Speech Therapy

Oxygen Therapy : Nasal cannula
Respirations : Other: WFLs
Tracheostomy Information : Oxygen Flow Rate: 2 L/min Performed by: Cruz, Nathan 03/06/24 12:00:00
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Oral-Facial Exam
Facial Movement : Symmetric resting/crying
Dentition : Own teeth
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Oral Structure Grid
Labial Structure : Within functional limits
Mandible : Within functional limits
Lingual Structure : Within functional limits
Hard Palate : Within functional limits
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Oral Function Grid
Labial : Within functional limits
Mandible : Within functional limits
Lingual : Within functional limits
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Volitional Cough : Within functional limits
Secretion Management : Adequate
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Swallowing Exam
Patient Position : Upright 90 degrees
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Liquids Swallowing Exam Adult Grid

	Ice Chips
Comment :	please see additional info
	Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Assessment/Recommendations
Clinical Swallowing Impression Grid
Within Functional Limits : Solids, Liquids
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Diet Consistency : Regular
Liquid Viscosity : Thin
Medications : Other: as tolerated – whole w/ liquids or floated in puree
Supervision : Intermittent
Positioning : Upright 90 degrees
Clinical Swallowing Recommendations : Intervention not indicated
Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Education
Learner(s) Present for Session : Demonstration, Explanation
Learner(s) Present : Self

Speech Therapy

Barriers to Learning : None evident

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Document learning evaluation for Responsible Learner(s)

Swallowing Strategies : Verbalizes understanding, Demonstrates

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Long Term Goals

SLP Patient, Caregiver Goals : N/A

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

Additional Information

Additional Information SLP : RM: 402
Minutes: 20

Per RN okay to proceed with swallow evaluation as ordered per MD.

66 y/o female admitted for: generalized asthenia, hyponatremia, AKI, UTI, facial cellulitis, severe sepsis, and dehydration. PMHx positive for: epilepsy NOS, TBI with resultant mild right hemiparesis, CHF, HTN, hypothyroidism, mood disorder NOS, and cardiac arrest per medical record. CXR results from 3/3/24 and head CT results from 3/4/24 were negative for any acute findings. CT Maxillofacial w/o contrast from 3/4/24 noted. Pt on 2L O2 via nasal cannula and on a minced & moist texture/thin liquid diet per MD. Pt was AA&Ox3-4. Impressions as follows.

Pt's vocal quality was strong and clear. Volitional cough was strong and dry. Oral motor structure and function assessment was WFLs. Hyo-laryngeal elevation was palpable. PO trials of thin liquids, purees, regular solids, and mixed consistencies were assessed. Anterior oral containment, mastication, oral manipulation, and A-P transfer were WFLs. Complete oral clearance occurred. No overt clinical s/sx concerning for aspiration were demonstrated. Pt denied any globus sensation.

Overall, at this time pt presents with oropharyngeal swallow that is WFLs. Recommend a regular texture/thin liquid diet. No need for skilled SLP intervention to address swallow. SLP to sign off. Please re-consult SLP service should further concerns arise.

CONCLUSIONS:
OROPHARYNGEAL SWALLOW IS WFLS. NO OVERT CLINICAL S/SX CONCERNING FOR ASPIRATION.

RECOMMENDATIONS:
1. Diet: regular texture/thin liquids
2. Medications: as tolerated - whole w/ liquids or floated in puree
3. Standard aspiration precautions, up at 90 degrees

Environmental Safety Implemented : Bed in low position, Call device within reach

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

ST Non Timed Modalities 2024

ST Eval Oral/Swallow Func Minutes : 20
ST Total Non Timed : 20

Hernandez SLP, Gardenia - 3/6/2024 16:12 PST

History and Physical

DOCUMENT NAME: History & Physical
SERVICE DATE/TIME: 3/4/2024 06:06 PST
RESULT STATUS: Unauth
PERFORM INFORMATION: Cruz-moreno APRN,Naomi S (3/4/2024 06:07 PST)
SIGN INFORMATION: Cruz-moreno APRN,Naomi S (3/4/2024 06:07 PST)

Chief Complaint/Reason for Consultation
Weakness the past few days, had a GLF today. No injury, no LOC, denies hitting her head, no blood thinners. LVFR noted that pt was also hypotensive on scene. Dex 115

History of Present Illness
The patient is a 66-year-old female admitted today from home after she sustained a fall. She tells me she has been having generalized weakness. She fell when she was trying to transfer from her bed to her wheelchair. In the emergency department she was found to have sodium 116. Nephrology has been consulted. She is given 1 L IV fluid bolus. UA also positive for UTI.

She has past medical history of traumatic brain injury, she has right sided weakness and balance issues, cardiac arrest, and hypertension.

Initial labs show white blood cell count 10.94, sodium 116, potassium 3.2, BUN 22, creatinine 1.85, and lactic acid 2.6. UA shows 3+ leukocyte esterase and greater than 182 white blood cells.
CT head without contrast shows no acute intracranial disease.
CT maxillofacial shows left facial fat stranding.

Patient seen and examined in the emergency department. She is currently awake, alert, appears in no acute distress. She is able to move all 4 extremities against gravity, however does have chronic right-sided weakness. She will be admitted intensive care unit for treatment of hyponatremia.

Review of Systems
Constitutional: No fever, chills
HEENT: No headache
Cardiac: No chest pain, palpitations
Pulmonary: No shortness of breath or cough
GI: No nausea, vomiting or abdominal pain.
GU: no pain or burning with urination
Skin: No itching or rash
Neurological: No new focal weakness; no dizziness.

Objective

Measurements (most recent)

Height (cm)	175.26 cm	Weidner RN, Robyn	03/03 23:13
Height (inches)	69.00 inch	SYSTEM	03/03 23:13
Weight	99.79 kg	Weidner RN, Robyn	03/03 23:13
Weight (lbs)	220.00 lb	SYSTEM	03/03 23:13
Body Mass Index	32.49 kg/m2	Weidner RN, Robyn	03/03 23:13

Histories
Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History
Active Problems(10)
Alcohol abuse
Anxiety
Cardiac arrest
Convulsion
Depression
Hypothyroid
MRSA
Obesity
Pneumonia
Ventilator dependence

Past Surgical History
TRACH

Social History
No social history documented.

Family History
No Family History documented.

Medications
Home Medications
alendronate (alendronate 70 mg oral tablet)
70 Milligram 1 Tabs By Mouth Every Tuesday
bisacodyl (bisacodyl 5 mg oral delayed release

History and Physical

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	36.8	03/03/2024 23:12	36.8	03/03/2024 23:12	36.8	03/03/2024 23:12
Heart Rate Monitored	**L** 58	03/04/2024 03:00	**L** 58	03/04/2024 03:00	65	03/03/2024 23:39
Resp Rate	20	03/04/2024 03:00	18	03/04/2024 02:00	20	03/04/2024 01:14
SBP	102	03/04/2024 03:00	94	03/03/2024 23:12	114	03/04/2024 02:00
DBP	**L** 56	03/04/2024 03:00	**L** 52	03/03/2024 23:12	61	03/03/2024 23:39
MAP	71	03/04/2024 03:00	71	03/04/2024 03:00	82	03/04/2024 02:00
SpO2	100	03/04/2024 03:00	94	03/03/2024 23:12	100	03/04/2024 01:14
O2 Flow Rate	on 2 l/min				on 2 l/min	
O2 Therapy	Nasal cannula		Room air		Nasal cannula	

Intake and Output (current encounter)

	03/04 02:25 03/04 06:06	03/04 07:00 03/04 06:06	03/03 07:00 03/04 06:59	03/02 07:00 03/03 06:59	03/01 07:00 03/02 06:59
Intake	1000	0	1000	0	0
Output	0	0	0	0	0
Net Total	1000	0	1000	0	0

Glasgow Coma Scale

Eye Opening Response Glasgow: Spontaneously (03/04/24 01:15:00)
Best Verbal Response Glasgow: Oriented and converses (03/04/24 01:15:00)
Best Motor Response Glasgow: Obeys commands (03/04/24 01:15:00)
Glasgow Coma Score: 15 (03/04/24 01:15:00)

Physical Exam

General: Alert, in no acute cardiopulmonary distress.
Mental Status: Oriented to person, place and time. Normal affect.
Head: Normocephalic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact.
Ear, Nose and Throat: Oropharynx clear, mucous membranes moist. Trachea midline.
Neck: Supple, Full range of motion.
Respiratory: Clear to auscultation. No wheezing, rales or rhonchi.

tablet)
5 Milligram 1 Tabs By Mouth Daily as needed as needed for constipation
calcium carbonate (calcium carbonate 500 mg (200 mg elemental calcium) oral tablet, chewable)
1,000 Milligram 2 Tabs Chewed Daily
carvedilol (carvedilol 3.125 mg oral tablet)
3.125 Milligram 1 Tabs By Mouth 2 Times a Day With Meals for 30 Days
cholecalciferol (Vitamin D3 1000 intl units oral capsule) 1,000 International units 1 Capsules By Mouth Daily
dapagliflozin (dapagliflozin 10 mg oral tablet)
10 Milligram 1 Tabs By Mouth Daily for 30 Days
escitalopram (escitalopram 10 mg oral tablet)
20 Milligram 2 Tabs By Mouth Daily
famotidine (famotidine 20 mg oral tablet)
20 Milligram 1 Tabs By Mouth 2 Times a Day
furosemide (Lasix 40 mg oral tablet)
40 Milligram 1 Tabs By Mouth 2 Times a Day
levETIRAcetam (Keppra 500 mg oral tablet)
500 Milligram 1 Tabs By Mouth 2 Times a Day
levothyroxine (levothyroxine 100 mcg (0.1 mg) oral tablet)
100 Microgram 1 Tabs By Mouth Daily
lisinopril (lisinopril 40 mg oral tablet)
40 Milligram 1 Tabs By Mouth Daily
multivitamin 1 Tabs By Mouth Daily

History and Physical

Cardiovascular: S1,S2. Regular rate and rhythm, no murmurs, rubs or gallops.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Normal bowel sounds.
Genitourinary: No costovertebral angle tenderness.
Neurologic: Bilteral upper extremities 4/5 motor. Right LE 3/5 motor, LLE 4/5 motor.
Skin: No rashes or lesions.
Musculoskeletal: No gross deformities.
Extremities: No edema noted. Pulses palpable x 4.

Assessment/Plan
Patient Donna Wolfbarndt
DOB: 03/25/1957

Assessment:
Hyponatremia
UTI
Acute kidney injury
Facial cellulitis
Severe sepsis, present on admission, source urinary versus skin soft tissue
history of traumatic brain injury, she has right sided weakness and balance issues, cardiac arrest, and hypertension

Hyponatremia (E87.1): Admit to ICU
Send stat TSH, serum osmolality, urine osmolality, urine sodium and urine chloride, follow result
Every 4 hours BMP, follow result
Nephrology consult obtained, NNC
Close neurological monitoring
Seizure precautions
Avoid overcorrection, goal sodium correction 6-8 in 24 hours

AKI (acute kidney injury) (N17.9): Nephrology following
Urinary catheter
Strict I's/O
Avoid nephrotoxic agents as able
Urine electrolytes pending

Severe sepsis (R65.20): Cefepime for empiric antibiotic coverage
Dose for renal function
Blood culture sent, follow result
Urine culture sent, follow result patient with left facial cellulitis and UA positive for UTI
Lactic acid 1.4
She was given 1 L IV fluid bolus
No further IV fluid boluses to avoid overcorrection in sodium or further drop in sodium given patient with hyponatremia
Sepsis reassessed

rivaroxaban (Xarelto 10 mg oral tablet)
10 Milligram 1 Tabs By Mouth Daily
spironolactone (spironolactone 25 mg oral tablet)
25 Milligram 1 Tabs By Mouth Daily for 30 Days
zonisamide (Zonegran 100 mg oral capsule)
200 Milligram 2 Capsules By Mouth Daily

Inpatient Medications
Medications (2) Active
SCHEDULED: (2)
cefepime 2 gm 20 mL, IV Piggyback, q12H Interval
heparin 5000 units/mL 1 mL Inj (heparin) 5,000 units 1 mL, SubCutaneous, q12H
CONTINUOUS: (0)
PRN: (0)

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefepime 2 gm, 20 mL, 12.5 mL/hr, IV Piggyback, q12H Interval	1	03/04/2024 02:55	03/04/2024 02:55

Results
Image
CT Head or Brain w/o Contrast 03/04/2024 00:04 by Puckett, Aliitasi
IMPRESSION: 1. No acute intracranial process 2. Mild nonspecific white matter changes likely due to chronic microvascular ischemia
CT Maxillofacial w/o Contrast 03/04/2024 00:04 by Puckett, Aliitasi
FINDINGS: The paranasal sinuses and mastoid air cells are

History and Physical

VTE Prophylaxis: Subcutaneous heparin
» **VTE Prophylaxis Assessment:** VTE Prophylaxis Ordered

Discussed with RN
70 minutes spent evaluating patient

Order Date/Time	Order Action	Order Name	Order Detail
03/04/2024 06:06	Order	heparin 5000 units/mL 1 mL Inj (heparin)	5,000 units, 1 mL, SubCutaneous, q12H
03/04/2024 02:43	Modify	cefepime	2 gm, 20 mL, 12.5 mL/hr, IV Piggyback, q12H Interval
03/04/2024 02:42	Order	TSH Ultra (TSH Ultrasensitive)	Blood, Nurse collect, Stat collect, 03/04/24 2:42:00 PST, Stop date 03/04/24 2:42:00 PST
03/04/2024 02:42	Order	U Cl (Urine Chloride Random)	Urine, Nurse collect, Routine collect, 03/04/24 2:42:00 PST, Stop date 03/04/24 2:42:00 PST, Print Label By Order Location
03/04/2024 02:41	Order	cefepime	2 gm, 20 mL, IV Piggyback, q12H Interval

clear. Left facial subcutaneous fat stranding. No acute osseous change. IMPRESSION: Left facial subcutaneous fat stranding. Correlate for cellulitis or hematoma. Negative for fracture.

Patient Information Form

Hospital Name: Centennial Hills Hospital Medical Center **Phone:** (702) 835-9700

Hospital Address: 6900 N DURANGO DR LAS VEGAS, NV 89149

Patient Demographics

Patient Name: WOLFBRANDT, DONNA J.
Medical Rec #: 7301725 **SSN:**
Marital Status: D **Gender:** F
Date of Birth: 03-25-1957 **Age:** 66
Religion: LUT
Episode ID: 8011012203
Height:
Weight:

Admit and Length of Stay Information

Unit: 4S
Admit Type: EMERGENCY **Room:** 402
Admission Date: 03-04-2024 **Bed:** 01
Est. Discharge Date: 03-10-2024 **ALC Date:**
Pt. Functional Status Prior to Admission:
PCP: FORTE FAMILY PRACTICE,
Attending Physician: SMUCKLER JASON,
Phone Number:

Diagnosis Information:

Rugs: **Primary:** Hypo-osmolality and hyponatremia **Secondary:** Hypo-osmolality and hyponatremia

Discharge Notes:

Notes:

Mode of Transportation:
Will patient receive radiation or dialysis off-site?
☐ Yes ☐ No

Payer Source:
Schedule of Treatments:

Patient Address

Living Arrangement: Select One
Facility Name:
Street: 3501 SHADY TIMBER STREI
City: LAS VEGAS
State/Zip: NV 891292850
Home Phone: (702)575-9934
Work Phone:

Next of Kin

First Name/MI: PEGGY
Last Name: REED
Street: 2005 RUTH DR
City: LAS VEGAS
State/Zip: NV 891081111
Home Phone: (702)595-5345
Work Phone:
Relation: N
☐ Emerg. Contact ☐ POA

Emergency Contact:

First Name/MI:
Last Name:
Street:
City:
State/Zip:
Home Phone:
Work Phone:
Relation:
☐ Emerg. Contact ☐ POA

Payer Information:

Ins. Group ID#:
Primary Payer: MEDICARE **Member ID#:** 1PD7RV6FE54
Contact person at Ins. Co. (First/MI/Last): **Phone:**
Patient has met 3 consecutive, acute level of care days during this admission & may be eligible for the Medicare Extended Care Benefit. ☐ Yes ☐ No ☐ N/A ☐ Unknown

Secondary Payer: MEDICAID 2ND MCRE **Member ID#:** 00001321767 **Phone #:**
Other Payer: **Member ID#:** **Phone #:**

Income if known: ☐ Private Funds ☐ SSA ☐ SSI ☐ Pension ☐ VA ☐ Other

Patient Medicaid Eligible? ☐ Yes ☐ No **If Yes, submitted by our financial office?** ☐ Yes ☐ No
Contact person in financial office (First/MI/Last):

No Fault: Must have claim number, name of insurance company, name of insured, telephone number, and policy number. This information is needed even if no fault is exhausted. No faults must have secondary insurance information.

Case Contact

First/MI/Last: Diana Thomas **Phone** 7023000688 **Date:** 03-07-2024